Janssen Research & Development *

Clinical Protocol

A Phase 3 Randomized Study Comparing Talquetamab in Combination with Pomalidomide (Tal-P), Talquetamab in Combination with Teclistamab (Tal-Tec), and Investigator's Choice of Either Elotuzumab, Pomalidomide, and Dexamethasone (EPd) or Pomalidomide, Bortezomib, and Dexamethasone (PVd) in Participants with Relapsed or Refractory Myeloma who Have Received 1 to 4 Prior Lines of Therapy Including an Anti-CD38 Antibody and Lenalidomide

A Study Comparing Talquetamab Plus Pomalidomide, Talquetamab Plus Teclistamab, and Elotuzumab, Pomalidomide, and Dexamethasone or Pomalidomide, Bortezomib, and Dexamethasone in Participants with Relapsed or Refractory Myeloma who Have Received an Anti-CD38 Antibody and Lenalidomide

MonumenTAL-6

Protocol 64407564MMY3009; Phase 3

Version: AMENDMENT 2

JNJ-64407564 and JNJ-64007957 (talquetamab and teclistamab)

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Studies conducted at sites in the United States (US) will be conducted under US Food & Drug Administration Investigational New Drug (IND) regulations (21 CFR Part 312). Studies conducted at sites in the European Economic Area (EEA) will be conducted under Regulation [EU] No 536/2014.

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GCP Compliance: This study will be conducted in compliance with GCP, and applicable regulatory requirements.

Confidentiality Statement

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PROTOCOL AMENDMENT SUMMARY OF CHANGES TABLE

DOCUMENT HISTORY		
Document	**Country/Territory Affected**	**Date**
Amendment 2	All	21 March 2024
Amendment 1	All	14 July 2023
Original Protocol	All	26 June 2023

Amendment 2 (21 March 2024)

Overall Rationale for the Amendment: The protocol has been modified in response to Health Authority feedback.

The changes made to the clinical protocol 64407564MMY3009 as part of Protocol Amendment 2 are listed below, including the rationale of each change and a list of all applicable sections. Changes made in previous protocol amendments are listed in Section 10.21, Appendix 21: Protocol Amendment History.

Section Number and Name	Description of Change	Brief Rationale
Title page	Added CTR requirement per Regulation [EU] No 536/2014.	Revised per Health Authority feedback.
1.2. Schema (Figures 2 and 3)	Figures 2 and 3 were added to reflect dosing for each study arm.	Text revised per Health Authority feedback.
1.2. Schema (Figures 2 and 3) 1.3. Schedule of Activities (Tables 4 and 5); 2.1.6. Rationale for Combining Talquetamab and Teclistamab; 4.3.1. Treatment Dose and Schedule for Tal-Tec; 6.6.1.2.1.1. Talquetamab and Teclistamab Dose Skips/Delays (Table 24 and Table 25)	The study procedures, assessments and dosing schedules for talquetamab was updated to specify that participants with response of confirmed PR or better will switch to Q4W dosing from Cycle 7 onward.	Text revised per Health Authority feedback.
4.3.1. Treatment Dose and Schedule for Tal-Tec	Added justification to the proposed finite duration of 26 cycles for talquetamab and teclistamab.	Text revised per Health Authority feedback.
2.2.5. Talquetamab/Teclistamab Combination Studies; 4.3.1. Treatment Dose and Schedule for Tal-Tec	Updated clinical data from Study RedirecTT-1 with longer follow-up for justification of dose of the talquetamab and teclistamab combination.	Text revised per Health Authority feedback.
1.2. Schema	References to SmPC for the EPd and PVd regimens added.	For consistency with other sections in the protocol.
1.3. Schedule of Activities (Table 2)	Window for randomization (eligibility and C1D1 dose) updated to specify business or calendar days.	To align with teclistamab program level protocol language.
1.3. Schedule of Activities (Tables 4 and 5)	Split dosing schedule and pretreatment medications for Arm A (Table 4) and Arm B (Table 5).	For improved readability
1.3. Schedule of Activities (Tables 3, 6, 7, 9, 10, and 12); 8.2.2. Bone Marrow Examination (Table 34)	Reduced immunogenicity and biomarker collections	To reduce participant burden.
	In Table 34, footnote ‘d’ updated to remove biomarker assessments.	To improve clarity.
1.3. Schedule of Activities (Tables 6, 9, and 12)	In Table 6, footnote for predose sample collection for PK samples revised to allow collection between -24 to 0 hour.	To allow greater flexibility in sample collection.

Section Number and Name	Description of Change	Brief Rationale
	In Tables 9 and 12, footnote for predose sample collection for biomarker samples added to allow collection between -4 to 0 hour.	
1.3. Schedule of Activities (Table 6)	Footnote f added for circulating proteins (serum).	To improve clarity
1.3.2. Schedule of Activities (Table 3)	Added TLS-related laboratory assessments at Cycle 2.	Revised per Health Authority feedback.
1.3.4. Schedule of Activities for Arm C (PVd) (Table 10)	Updated to include pre-treatment chest imaging prior to administration of bortezomib.	Revised per Health Authority feedback.
1.3. Schedule of Activities (Tables 3, 7, and 10)	Added MUGA or TTE to the schedule of assessments for patients who might be exposed to bortezomib or pomalidomide. Added instructions for monitoring of cardiac toxicity.	Revised per Health Authority feedback.
1.3. Schedule of Activities (Tables 3, 7, and 10); 8.3.1. Physical Examinations	Updated to require physical examination once per cycle.	Revised per Health Authority feedback.
1.3.2. Schedule of Activities for Arm A (Tal-P) and Arm B (Tal-Tec) (Table 3); 8.3.8. Neurologic Examination	SoA tables updated to indicate neurological examinations during treatment period 'as clinically indicated'. In Section 8.3.8., reference to Table 3 added.	For consistency across protocol sections.
1.3. Schedule of Activities (Tables 3, 7, and 10); 8.3.5.3. Ongoing HBV Testing and HCV Testing; 8.3.6. HIV Testing	Added window of ±4 weeks for PCR testing for HBV, HCV, and HIV.	To allow greater flexibility in testing and reduce patient burden. To align with teclistamab program level protocol language.
1.3. Schedule of Activities (Tables 3, 7, and 10)	Hematology and chemistry assessments updated to add that from C1D8, assessment may be performed within 48 hours before dosing.	To align with talquetamab program level protocol language.
1.3. Schedule of Activities (Tables 3, 7, and 10); 8.2. Efficacy Assessments	Update timing of disease evaluations in the case of cycle delays to ensure they are done every 28 days (±7 days).	To align with talquetamab program level protocol language.
1.3.2. Schedule of Activities for Arm A (Tal-P) and Arm B (Tal-Tec) (Table 4)	Deleted reference to the pomalidomide product information in footnote "e".	Revised per Health Authority feedback.
1.3.2. Schedule of Activities for Arm A (Tal-P) and Arm B (Tal-Tec) (Table 4)	Added note for preteatement medications for participants in Q4W dosing schedule.	To align with talquetamab program level protocol language.
1.3.3. Schedule of Activities for Arm C (EPd) (Table 8)	Ranitidine replaced by locally available H2 blocker and footnote added.	To provide alternative options after ranitidine was withdrawn from market in US.
1.3.3. Schedule of Activities for Arm C (EPd) (Table 8); 6.6.2.4. Dexamethasone Dose Modifications (Table 31)	Dexamethasone PO dose for participants >75 years of age updated to 8 mg. In Table 31, same change was made for dose level 0, and dose level -1 was modified accordingly.	To ensure consistency with the label.
2.1.1. Multiple Myeloma	Deleted repetitive information.	To improve clarity
2.3.1.1. Risks Associated with Talquetamab and Teclistamab	Updated management of infections to include mitigation strategies for PML.	Revised per Health Authority feedback.
	Table 14 title revised to delete 'potential'.	For clarity.

Section Number and Name	Description of Change	Brief Rationale
(Table 14); 6.1.4.4.2. Infection Management	Updated management of skin and nail changes to include use of emollients.	To align with teclistamab program level protocol language.
2.2.5. Talquetamab/Teclistamab Combination Studies	Revised text to reflect current study status for ongoing studies. Added reference to talquetamab IB.	To update current study status for ongoing talquetamab studies.
2.3.1.3. Risks Associated with PVd (Pomalidomide and Bortezomib)	Deleted repetitive information and replaced by cross-reference to Section 2.3.1.2	To improve clarity.
4.2.1. Study-specific Ethical Design Considerations; 5.1. Inclusion criteria (Criterion 13); 8.4. Adverse Events, Serious Adverse Events, and Other Safety Reporting; 10.2.3. Appendix 2: Informed Consent Process	The term legally acceptable representative has been replaced with legally designated representative. In Section 4.2.1, text revised to include information about legally designated representative.	To align with ICF v12 and clinical protocol template.
5.1. Inclusion Criteria (Criterion 15)	Criterion 15 updated to include (new text in bold): 'Participants who have received only one prior line of myeloma therapy must be considered lenalidomide refractory **or intolerant to treatment'**.	To improve clarity and align with patient and provider input.
5.2. Exclusion Criteria (Criterion 1)	Added criterion that participants who had not received prior treatment of both PI and lenalidomide will be excluded from the EPd treatment arm (Arm C); added numbering for Criterion 1.c.ii.	Revised per Health Authority feedback.
5.2. Exclusion Criteria (Criterion 3)	Unstable angina added to cardiac conditions.	To align with teclistamab program.
5.3. Lifestyle Considerations (Criterion 8)	To specify timing requirements when participants should be restricted from driving or operating heavy machinery.	To align with talquetamab program level protocol language.
5.4. Screen Failures	Updated IWRS text	To meet company requirements
6.1. Study Treatment Administered (Table 16)	Text added to IMP designation table specifying whether or not the IMP is used in accordance with Marketing Authorization. Updated table to reflect conditional EMA approval granted for talquetamab.	To align with clinical protocol template, revised per Health Authority feedback.
	In Table 16, order of study treatment administration updated to include: **'at least 15 minutes apart (it is suggested to administer talquetamab first)'**.	To improve clarity
	Current/Former Name(s) or Alias(es) updated to include 'TALVEY'.	To include talquetamab brand name for completeness.
6.1. Study Treatment Administered (Table 17)	Dose Formulation for dexamethasone updated to delete 'solution for infusion'	To improve clarity on route of administration of dexamethasone as study treatment.
	Dosing Instructions and Route of Administration for dexamethasone updated to delete IV administration for dexamethasone.	
	Order of treatment administration updated to indicate dexamethasone will be administered **3 to 24** hours prior to elotuzumab.	For consistency across protocol sections.

Section Number and Name	Description of Change	Brief Rationale
6.1.2. Pretreatment Medications	Updated time window in which dexamethasone should not be administered as pretreatment medication for talquetamab to 'after **C1D15**'.	For consistency across protocol sections.
6.1.2.1. Event-driven Required Pretreatment Medications (Arm A and Arm B Only)	Updated information about dexamethasone maximal dose as pretreatment medication.	For consistency across protocol sections.
6.1.3.1. Safety Monitoring for Talquetamab SC and Teclistamab SC Dosing Through the First Treatment Dose; 10.16. Appendix 16: Considerations for Outpatient Talquetamab and Teclistamab Administration Through the First Treatment Dose	Update requirements for outpatient dosing during Cycle 1 to specify that participant is required to stay within 30 minutes of transportation to the site and remain in the company of a competent adult at all times from after Step-up Dose 1 until 48 hours after administration of study treatment dose.	To align guidance for outpatient dosing with the new guidelines for close observation through study treatment dose.
	Updated Cycle 1 monitoring to require at least 2 hours observation after administration of talquetamab and teclistamab in the outpatient setting.	Revised per Health Authority feedback
6.1.3.1.1. Required Hospitalization for Talquetamab SC and Teclistamab SC Dosing Following Specified Adverse Events (Table 19)	Revised CRS hospitalization criteria for subsequent talquetamab administration.	To align with other bispecific clinical study protocols.
6.1.4.3. Hypogammaglobulinemia (Participants Receiving Talquetamab and Teclistamab); 6.9.1. Permitted Therapies; 11. References	Added literature reference added for Ig replacement. Added text to specify procedure for administration of immunoglobulin replacement for hypogammaglobulinemia.	To clarify guidelines for IV/SCIG replacement therapy and to align with other bispecific clinical study protocols.
6.1.4.9. Skin and Nail Toxicities	Additional guidance added for skin and nail toxicities.	To align with teclistamab and talquetamab protocols.
6.5. Study Treatment Compliance	Added that instructions for self-administration and home storage conditions should be provided as well for any study treatment medications (eg pomalidomide)	To improve clarity.
6.6.1.2.1.1. Talquetamab and Teclistamab Dose Skips/Delays; 7.1.2. Discontinuation of Talquetamab SC (Arms A and B Only); 7.1.3. Discontinuation of Teclistamab SC (Arm B Only)	In Section 6.6.1.2.1.1, removed requirement to discontinue after dose interruption >35 days to avoid duplication with Table 25. In Sections 7.1.2 and 7.1.3, updated talquetamab and teclistamab discontinuation criteria for consistency with dose delay table (Table 25).	To align with teclistamab and talquetamab protocols.
6.6.1.2.1.1. Talquetamab and Teclistamab Dose Skips/Delays (Table 25)	Updated recommendations for restarting therapy after dose delay (Table 25).	To align with teclistamab and talquetamab protocols.
6.6.1.2.1.2. Pomalidomide Dose Skips	Updated to reflect pomalidomide dosing may continue even if bortezomib is delayed within a cycle	To improve clarity.

Section Number and Name	Description of Change	Brief Rationale
6.6.2.2. Talquetamab and Teclistamab Dose Interruption for Non-hematologic Toxicities	Text added to specify that for participants with TEAEs that persist but are stable, discussion with the sponsor is required before proceeding to the next dose of talquetamab SC.	To clarify when talquetamab dosing may resume in the event of TEAEs that persist but are stable.
6.9.3. Prohibited and Restricted Therapies; 11. References	Literature references updated.	To provide current literature guidance.
6.10. SST	Edited to clarify that the decision to start subsequent antimyeloma therapy is at the discretion of the investigator.	Revised per Health Authority feedback.
7.1.2. Discontinuation of Talquetamab SC (Arms A and B Only)	Text added to specify timing for dose delays leading to talquetamab discontinuation for monthly dosing.	To update talquetamab dose delay guidelines after monthly dosing has been initiated.
7.2.1. Withdrawal From the Use of Study Samples; 10.2.6. Appendix 2: Storage, Use, Transfer, and Retention of Data and Samples	The term research sample has been replaced with study sample and the section has been revised.	To align with ICF v12 and clinical protocol template.
	In Section 7.2.1, added instruction to refer to local ICF for details regarding handling of research samples.	Revised per Health Authority feedback.
7.3. Lost to Follow-up	Revised to allow site (rather than sponsor) to engage a third party to search public sources for participant information.	To align with clinical protocol template.
8.1.1.4. Posttreatment Follow-up Phase; 1.3. Schedule of Activities (Tables 3, 7 and 10)	Added details on follow-up assessments prior and following disease progression. Survival assessment in Tables 3, 7, and 10, updated to add reference to Section 8.1.1.4.	To improve clarity.
8.2. Efficacy Assessments	Added details on disease evaluation assessments. Added clarification that the ±7 day window is not applicable to C1D1.	To improve clarity.
	Updated to describe the validated computerized algorithm for disease progression.	Revised per Health Authority feedback.
	Text added: '**The onset of new and/or increased size of soft tissue plasmacytoma or lytic bone lesions observed during Cycle 1 will not be considered disease progression'**.	To improve clarity.
8.2.3. MRD Evaluations	NGS clonoSEQ® assay deidentified data was added as an accepted sample for MRD NGS evaluations. Details added if a clone for MRD assessment cannot be identified or if a sample is deemed of low quality and/or yields no results.	To improve clarity.
8.2.5. Documentation of Soft-Tissue Plasmacytomas	Details added for whole-body imaging.	To improve clarity.
8.3.5.1. HBV Testing at Screening; 8.3.5.2. HCV Testing at Screening; 8.3.5.3. Ongoing HBV Testing and HCV Testing; 8.3.6. HIV Testing	Information added about requirement at screening and requirement to repeat a test.	To improve clarity.
8.4.1. Time Period and Frequency for Collecting AE	Information for reporting of SAEs reporting has been updated.	Clarify the current safety reporting process for clinical studies.

Section Number and Name	Description of Change	Brief Rationale
and SAE Information; 10.3.5. Appendix 3: Procedures		
8.3.7. Pregnancy Testing; 8.4.5. Pregnancy; 10.4. Appendix 4: Contraceptive and Barrier Guidance	Text regarding PPP referencing was revised to align with sourcing of pomalidomide.	To provide additional clarification for PPP for pomalidomide.
8.3.8. Neurologic Examination; 1.3.2. Schedule of Activities for Arm A (Tal-P) and Arm B (Tal-Tec) (Table 3)	Text revised to specify that the ICE Tool evaluation will be performed as described in Table 3. In Table 3, neurologic examination assessment updated to include 'as clinically indicated' during the Treatment phase.	To align across protocol sections.
8.4.4. Regulatory Reporting Requirements for SAEs	Updated for compliance SUSAR reporting as per Regulation [EU] No 536/2014 Article 42.	Revised per Health Authority feedback.
8.4.5. Pregnancy; 10.4. Appendix 4: Contraceptive and Barrier Guidance	Changed all instances referring to "male" and "female", to indicate "participant(s)", as appropriate.	To introduce gender neutral language for inclusivity.
8.4.6. Disease-related Events and Disease-related Outcomes Not Qualifying as AEs or SAEs	List of disease-related events and outcomes not qualifying as AEs updated.	To align with clinical protocol template.
8.7.1. Pharmacodynamic and Exploratory Biomarkers	Updated biomarker assessments for immune and pro-inflammatory activity.	To improve clarity.
9.2. Sample Size Determination; 9.5. Interim Analyses	Additional information added for sample size calculation for 80% power to detect an OS HR of 0.72. Additional details for timing of interim analysis added.	Further details added for OS powering.
9.4.1. General Considerations	Added text regarding monitoring of data collection, handling of missing data, and sensitivity analyses, and clarified that no data imputation will be applied for missing efficacy, safety, PRO or PK evaluations.	Revised per Health Authority feedback.
9.4.11. PRO Analyses	Language updated to 'sustained change in HRQoL', and reference to SAP added.	To improve clarity.
10.2.4. Appendix 2: Recruitment Strategy	Section 10.2.4 added	To align with clinical protocol template and to comply with EU CTR requirements.
10.2.8. Appendix 2: Use of Information and Publication	Revised text regarding publication by Investigator.	To align with clinical protocol template and to comply with EU CTR requirements.
10.2.11. Appendix 2: Source Documents	Record of the AE assessment and related follow-ups by the investigator should be kept as source documents.	To align with clinical protocol template.
10.2.14. Appendix 2: Record Retention	Added text regarding record retention under EU regulation.	To align with clinical protocol template and to comply with EU CTR requirements.
10.3.2. Appendix 3: Attribution Definitions	Added factors to consider when assessing causality.	To align with clinical protocol template.
10.3.5. Appendix 3: Procedures	Added text regarding SAE transmission to the sponsor.	To align with clinical protocol template.

Section Number and Name	Description of Change	Brief Rationale
10.4. Appendix 4: Contraceptive and Barrier Guidance	Updated definition of permanent absence of reproductive potential.	To align with clinical protocol template to introduce gender neutral language for inclusivity, and to use a definition which includes male sterility, per Health Authority feedback.
	Deleted "injectable" administration from the list of combined estrogen and progesterone hormonal contraception.	Revised per Health Authority feedback.
10.5. Appendix 5: Liver Safety: Suggested Actions and Follow-up Assessments	Deleted sections 10.5.1 (Stopping Algorithm) and 10.5.2 (Follow-up Assessments). Added instruction to reference applicable product information for guidance.	Revised per Health Authority feedback.
10.7. Appendix 7: Study Conduct During COVID-19 Pandemic or Other Natural Disaster/Major Disruption	COVID-19 guidance updated to remove broken link to NCCN guidance website.	To provide active links to guidance websites.
10.11. Appendix 11: Formulas for Estimating Glomerular Filtration Rate Using Modified Diet in Renal Disease Formula (in mL/min)	Reference to FDA CDER guidance added.	To align with clinical protocol template.
10.17. Appendix 17: IMWG Response Criteria (2016)	IMWG response criteria updated to include information about FLC levels and soft-tissue plasmacytoma progressive disease criteria.	Align with other teclistamab and talquetamab protocols and improve clarity.
10.18. Appendix 18: Severity Grading for ICANS (ASTCT)	Duplicated information in table footnotes about intracranial hemorrhage deleted.	To avoid duplication.
Throughout the protocol	Minor grammatical, formatting, or spelling changes were made.	Minor errors were noted.

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ABBREVIATIONS AND DEFINITIONS OF TERMS

ADA	anti-drug antibody(ies)
ADL	activities of daily living
AE	adverse event
AESI	adverse event of special interest
ALT	alanine aminotransferase
ANC	absolute neutrophil count
anti-HBc	hepatitis B core antibody
anti-HBs	hepatitis B surface antibody
AST	aspartate aminotransferase
ASTCT	American Society for Transplantation and Cellular Therapy
AxMP	auxiliary medicinal product
BCMA	B-cell maturation antigen
β-hCG	β-human chorionic gonadotropin
BMI	body mass index
BMPC	bone marrow plasma cell
BSA	body surface area
C	Cycle
CnDn	Cycle n Day n
CAR-T	chimeric antigen receptor T cell
CBC	complete blood count
CD	cluster of differentiation
CDER	Center for Drug Evaluation and Research
CTR	Clinical Trials Regulation
CI	confidence interval
CMH	Cochran–Mantel–Haenszel
CNS	central nervous system
CoV-2	coronavirus 2
COVID-19	coronavirus disease 2019
CR	complete response
CRP	C-reactive protein
CRS	cytokine release syndrome
CSR	Clinical Study Report
CT	Computed tomography
CyTOF	cytometry by time of flight
d/D	day
DL	dose level
DLT	dose-limiting toxicity
DOR	duration of response
DWI	diffusion-weighted imaging
ECG	electrocardiogram
ECOG	Eastern Cooperative Oncology Group
eCRF	electronic case report form
eDC	electronic data capture
eGFR	estimated glomerular filtration rate
EMN	European Myeloma Network
EORTC-QLQ-C30	European Organization for Research and Treatment of Cancer Quality of Life Questionnaire
EOS	end of study
EOT	end of treatment
EPd	elotuzumab, pomalidomide, and dexamethasone
EQ-5D-5L	EuroQol Five Dimension Five Level Questionnaire
ESMO	European Society for Medical Oncology
EU	European Union
EU-27	27 Member States of the European Union after the United Kingdom left the European Union
FDA	Food and Drug Administration
FFPE	formalin-fixed, paraffin-embedded
FISH	fluorescence in situ hybridization

FLC	free light chain
GCP	Good Clinical Practice
G-CSF	granulocyte colony stimulating factor
GFR	glomerular filtration rate
GM-CSF	granulocyte-macrophage colony stimulating factor
GPRC5D	G protein-coupled receptor family C group 5 member D
h	hour(s)
H2	histamine
HAART	highly active antiretroviral therapy
HbsAg	hepatitis B surface antigen
HBV	hepatitis B virus
HCV	hepatitis C virus
HIV	human immunodeficiency virus
HR	hazard ratio
HRQoL	health-related quality of life
HRT	hormonal replacement therapy
IAn	interim analysis n
IB	Investigator's Brochure
ICANS	immune effector cell-associated neurotoxicity syndrome
ICE	immune effect cell-associated encephalopathy
ICF	informed consent form
ICH	International Council on Harmonisation
ICP	intracranial pressure
IDMC	Independent Data Monitoring Committee
IEC	Independent Ethics Committee
Ig	immunoglobulin(s)
IHC	immunohistochemistry
IMiD	immunomodulatory drug
IMWG	International Myeloma Working Group
IMP	investigational medicinal product
IND	Investigational New Drug
INR	international normalized ratio
IOP	intraocular pressure
IPPI	Investigational Product Preparation Instructions
irAE	immune-related adverse event
IRB	Institutional Review Board
IRC	Independent Review Committee
ISS	International Staging System
ITT	intent-to-treat
IV	intravenous(ly)
IWRS	interactive web response system
mAb	monoclonal antibody
MedDRA	Medical Dictionary for Regulatory Activities
min	minute(s)
M-protein	monoclonal protein
MRD	minimal residual disease
MRI	magnetic resonance imaging
MRU	medical resource utilization
MUGA	multigated acquisition scan
MySIm-Q	Multiple Myeloma Symptom and Impact Questionnaire
NCI-CTCAE	National Cancer Institute Common Terminology Criteria for Adverse Events
NGF	next-generation flow cytometry
NGS	next-generation sequencing
NIMP	non-investigational medicinal product
NYHA	New York Heart Association
ORR	overall response rate
OS	overall survival
PC	plasma cell

PCC	protocol clarification communication
PCR	polymerase chain reaction
Pd	pomalidomide and dexamethasone
PD	progressive disease
PET	positron emission tomography
PFS	progression-free survival
PFS2	progression-free survival on next-line therapy
PGI-S	Patient Global Impression of Severity
PK	pharmacokinetic(s)
PI	proteasome inhibitor
PML	progressive multifocal leukoencephalopathy
PO	oral(ly) (in relation to route of administration of medication)
POCBP	participant of childbearing potential
PPP	Pregnancy Prevention Plan
PQC	product quality complaint
PR	partial response
PRO	patient-reported outcome (paper or electronic as appropriate for this study)
PT	prothrombin time
PVd	pomalidomide, bortezomib, and dexamethasone
QnD	every n days
QnW	every n weeks
QoL	quality of life
REMS	Risk Evaluation and Mitigation Strategy
RP2D	recommended Phase 2 dose
RNA	ribonucleic acid
SAC	Safety Assessment Committee
SAE	serious adverse event
SAP	statistical analysis plan
SARS	severe acute respiratory syndrome
sARR	systemic administration-related reaction
SC	subcutaneous(ly)
SCIG	subcutaneous immunoglobulin
sCR	stringent complete response
SIFE	serum immunofixation electrophoresis
SIPPM	Site Investigational Product Procedures Manual
SLAMF7	signaling lymphocytic activation molecule F7
SmPC	Summary of Product Characteristics
SoA	Schedule of Activities
SPD	sum of the products of the maximal perpendicular diameters of measured lesions
SPEP	serum M-protein quantitation by electrophoresis
SPM	second primary malignancy
SST	subsequent antimyeloma therapy
SU(n)	step-up dose n
SUSAR	suspected unexpected serious adverse reaction
Tal-P	talquetamab and pomalidomide
Tal-Tec	talquetamab and teclistamab
TEAE	treatment-emergent adverse event
TLS	tumor lysis syndrome
TTE	transthoracic echocardiogram
TTNT	time to next treatment
UIFE	urine immunofixation electrophoresis
UK	United Kingdom
ULN	upper limit of normal
UPEP	urine M-protein quantitation by electrophoresis
US	United States
USPI	United States Prescribing Information
VGPR	very good partial response

Definitions of Terms

sARR	systemic reaction related to study drug administration regardless of the route of administration

1. PROTOCOL SUMMARY

1.1. Synopsis

A Phase 3 Randomized Study Comparing Talquetamab in Combination with Pomalidomide (Tal-P), Talquetamab in Combination with Teclistamab (Tal-Tec), and Investigator's Choice of Either Elotuzumab, Pomalidomide, and Dexamethasone (EPd) or Pomalidomide, Bortezomib, and Dexamethasone (PVd) in Participants with Relapsed or Refractory Myeloma who Have Received 1 to 4 Prior Lines of Therapy Including an Anti-CD38 Antibody and Lenalidomide

Regulatory Agency Identifier Number(s)

Registry	ID
IND:	133811
EU Trial number:	2022-502446-27

A Study Comparing Talquetamab Plus Pomalidomide, Talquetamab Plus Teclistamab, and Elotuzumab, Pomalidomide, and Dexamethasone or Pomalidomide, Bortezomib, and Dexamethasone in Participants with Relapsed or Refractory Myeloma who Have Received an Anti-CD38 Antibody and Lenalidomide

DESCRIPTION OF COMPOUND

Talquetamab (also known as JNJ-64407564) is a humanized IgG4 bispecific antibody designed to target the CD3 receptor complex on T cells and GPRC5D-expressing PCs. Similarly, teclistamab (also known as JNJ-64007957) is a humanized IgG4 bispecific antibody that binds the CD3 receptor complex on T cells and BCMA on PCs. Redirecting T cells to a tumor surface antigen using bispecific antibody approaches may be an effective means to harness the immune system to destroy cancer cells and create meaningful and long-lasting clinical responses.

BENEFIT-RISK ASSESSMENT

Both talquetamab and teclistamab have demonstrated significant activity as monotherapy in participants with relapsed or refractory multiple myeloma. It is hypothesized that the use of talquetamab in combination with pomalidomide or teclistamab in this population could potentially lead to improved efficacy and QoL compared with EPd or PVd. Considering the measures taken to minimize risk to participants of this study, the potential risks identified in association with study treatment are justified by the hypothesized benefits that may be afforded to participants with pretreated relapsed/refractory multiple myeloma.

OBJECTIVES

The primary objective is to compare the efficacy of either Tal-P or Tal-Tec with EPd or PVd. Secondary objectives include to further compare the efficacy of either Tal-P or Tal-Tec with EPd or PVd and to assess the safety and tolerability of Tal-P and Tal-Tec.

HYPOTHESES

The primary hypothesis is that Tal-P or Tal-Tec will significantly improve PFS compared with EPd or PVd in participants with relapsed or refractory multiple myeloma who have received 1 to 4 prior lines of therapy including an anti-CD38 mAb and lenalidomide.

OVERALL DESIGN

This is a randomized, Phase 3, active-controlled, parallel, multicenter, interventional, open-label study in participants with relapsed or refractory multiple myeloma who have received 1 to 4 prior lines of therapy including an anti-CD38 mAb and lenalidomide.

An IRC will be commissioned to adjudicate participants' response to treatment and disease progression. An IDMC will be commissioned to review safety periodically and efficacy at prespecified interim analysis timepoints.

The study will include a Screening Phase, a Treatment Phase, and a Follow-up Phase. The Screening Phase will be up to 28 days before randomization. The Treatment Phase will extend from C1D1 until confirmed disease progression, death, intolerable toxicity, withdrawal of consent, or discontinuation of all study treatment (whichever occurs first). Participants can continue therapy with talquetamab/teclistamab (as appropriate) for up to 26 cycles if they have no sign of PD or toxicity. Following the Treatment Phase, participants will continue in the Follow-up Phase until death, withdrawal of consent, loss to follow-up, or end of the study, whichever occurs first. The EOS is considered as completion of the final OS analysis, which will occur after approximately 311 death events for each comparison.

NUMBER OF PARTICIPANTS

A target of approximately 795 participants randomized in a 1:1:1 ratio to receive Tal-P (Arm A), Tal-Tec (Arm B), or EPd or PVd (Arm C), with stratification by number of prior lines of therapy (1 versus ≥2), ISS stage at screening (I versus II/III), and investigator's intended choice of therapy (EPd versus PVd).

STUDY ARMS AND DURATION

Study treatment will be administered in 28-day cycles for Tal-P (Arm A), Tal-Tec (Arm B), and EPd (Arm C). For PVd (Arm C), study treatment will be administered in 21-day cycles.

At the time of screening, based on the investigator's choice, they must declare whether the participant will be treated with EPd or PVd should they be randomized to Arm C. The protocol includes eligibility criteria related to investigator's choice of EPd or PVd.

EFFICACY EVALUATIONS

Efficacy assessments will occur per IMWG criteria (2016) as defined in the protocol using data from serum, urine, bone marrow, and imaging (if applicable). Responses or progression will be evaluated by investigators, use of a computerized algorithm, and by an IRC; assessment by the IRC will be used as the primary analysis.

PHARMACOKINETIC EVALUATIONS

Sparse blood samples will be collected for the measurement of serum concentrations of talquetamab (Arms A and B) and teclistamab (Arm B) for PK analyses. Population PK analysis may be conducted, and the results reported separately.

IMMUNOGENICITY EVALUATIONS

The detection and characterization of ADAs to talquetamab or teclistamab will be performed using validated assay methods. The ADA-positive samples will be tested for neutralizing antibodies to talquetamab and teclistamab, if feasible.

PHARMACODYNAMIC AND EXPLORATORY BIOMARKER EVALUATIONS

Peripheral blood and bone marrow aspirates from participants in all 3 treatment arms will be assessed for biomarkers to identify pharmacologic activity by study treatment, identify participant subgroups that respond differently to treatments, identify predictive biomarkers of response and resistance, and evaluate the effect of study treatments on MRD negativity.

PRO EVALUATIONS

The participant's symptoms, functioning, and general well-being will be captured using the following PRO instruments: MySIm-Q, EORTC-QLQ-C30, EQ-5D-5L, and the PGI-S. The Epstein Taste Survey will capture data on participant's oral health to further quantify toxicity associated with talquetamab. Additionally, all participants will have the option of participating in a single, post-baseline, qualitative interview.

EXPLORATORY EVALUATIONS

MRU data associated with medical encounters will be collected and may be used to conduct exploratory economic analyses.

SAFETY EVALUATIONS

Safety will be assessed by physical examinations, neurologic examinations, ECOG performance status, clinical laboratory tests, vital signs, ECGs, and AE monitoring. The severity of AEs will be assessed using NCI-CTCAE Version 5.0, except for grading of CRS and ICANS, which will be assessed based on ASTCT guidelines. Concomitant medication use will be recorded.

REAL-WORLD DATA COLLECTION OF PARTICIPANT MEDICAL INFORMATION DURING AND AFTER THE STUDY (US SITES ONLY)

For participants in the US who have provided consent, medical data (electronic health records, claims, and laboratory data from other care settings) obtained after study entry may be accessed utilizing tokenization and matching procedures. These data may be used for exploratory analyses to enhance understanding of progression to advanced disease stages and subsequent treatment decisions and to explore the potential for using this method of data collection versus traditional follow-up data collection.

STATISTICAL METHODS

The primary population for efficacy analysis is the ITT Analysis Set, which will include all randomized participants. The Safety Analysis Set consists of all participants randomized who received ≥1 dose of study treatment.

PFS is the primary endpoint for the study. Stratified log-rank test, Kaplan-Meier methods, and stratified Cox regression with treatment as the sole explanatory variable will be used for analysis of the primary efficacy endpoint of PFS. The PFS primary analysis will be performed after approximately 272 PFS events are observed for each comparison (approximately 408 PFS events across the 3 arms). A PFS interim analysis is planned at approximately 75% of PFS events.

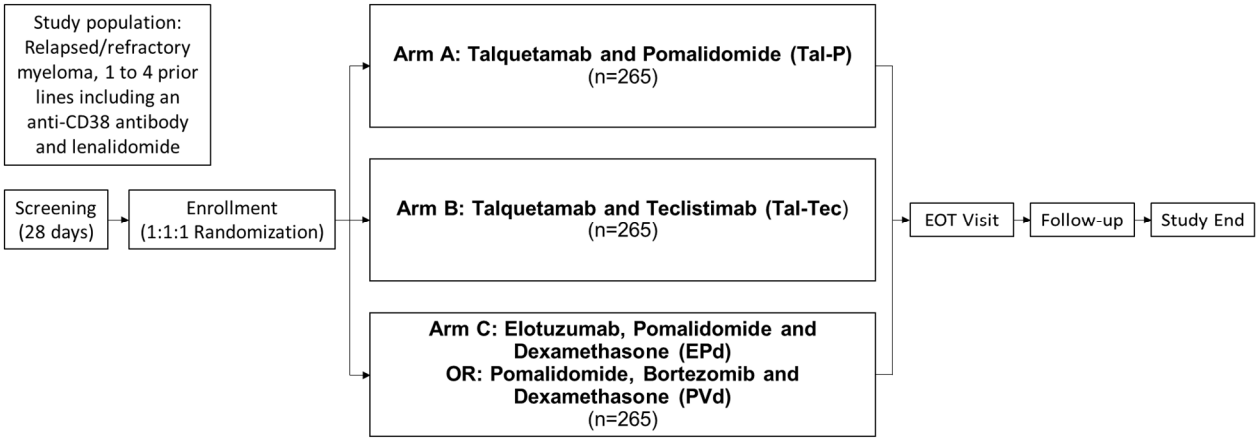
The first OS interim analysis is planned at the time of the PFS primary analysis, and it is estimated that approximately 43% of planned total number of OS events will be accumulated. Additional OS interim analyses are planned approximately every 12 months after the PFS primary analysis. Long-term survival

follow-up will continue until approximately 311 deaths for each comparison are observed (approximately 467 deaths expected across the 3 arms).

A graphic approach of multiple testing procedures will be used to test the primary and key secondary endpoints.

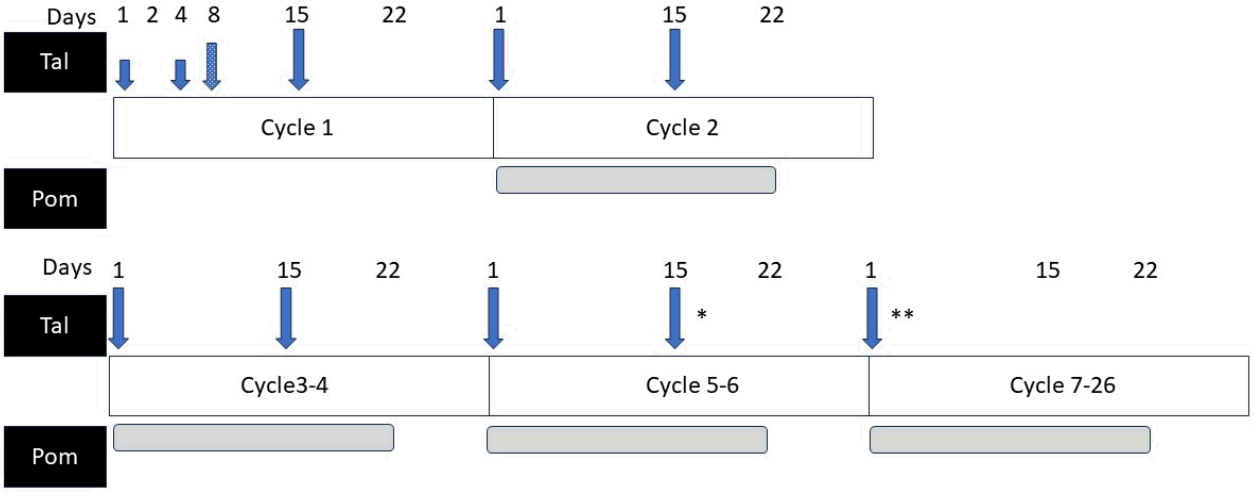
1.2. Schema

Figure 1: Schematic Overview of the Study



Arm A (Tal-P): Participants randomized to Tal-P will receive talquetamab SC step-up doses at 0.01 and 0.06 mg/kg, 1 dose of 0.4 mg/kg followed by 0.8 mg/kg SC Q2W study treatment dose in combination with pomalidomide, which will be administered at 2 mg at Cycle 2 Day 1 and then may escalate to the label approved 4 mg dose per investigator discretion at start of Cycle 3 Day 1 as tolerated. From Cycle 5, participants with response of confirmed VGPR or better can change to talquetamab Q4W per investigator discretion. From Cycle 7, participants with response of confirmed PR or better must be switched to talquetamab 0.8 mg/kg Q4W (Figure 2).

Figure 2: Dosing Schedule – Arm A (Tal-P)

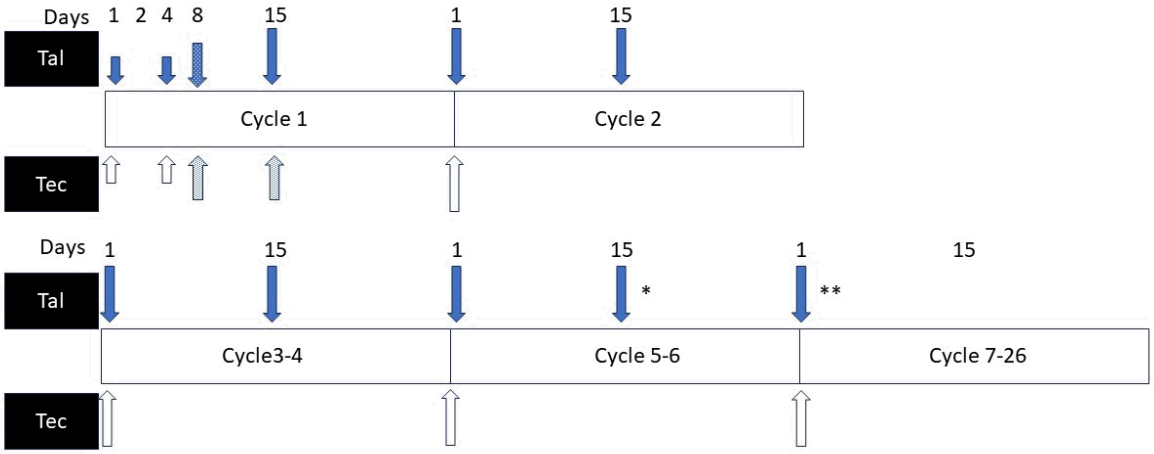


NOTE: In the figure, the large-size downward arrow indicates a talquetamab dosing day, the mid-size dotted downward arrow indicates a talquetamab 0.4 mg/kg dosing day, the shorter downward arrow indicates a talquetamab step-up dosing day, and the bar indicates daily pomalidomide dosing.

*From Cycle 5, if confirmed VGPR or better, can change to talquetamab Q4W dosing (Day 1 only) per investigator discretion.
**From Cycle 7, talquetamab Q4W for participants with response of confirmed PR or better. For all others, continue with Q2W dosing until confirmed PR or better is achieved.

Arm B (Tal-Tec): Participants randomized to Tal-Tec will receive talquetamab SC step-up doses at 0.01 and 0.06 mg/kg, 1 dose of 0.4 mg/kg followed by 0.8 mg/kg SC Q2W study treatment dose in combination with teclistamab administered at step-up doses of 0.06 and 0.3, 2 doses of 1.5 mg/kg, and then 3 mg/kg Q4W study treatment dose starting from Cycle 2. From Cycle 5, participants with response of confirmed VGPR or better can change to talquetamab Q4W per investigator discretion. From Cycle 7, participants with response of confirmed PR or better must be switched to talquetamab 0.8 mg/kg Q4W (Figure 3).

Figure 3: Dosing Schedule – Arm B (Tal-Tec)



NOTE: In the figure, the large-size downward arrow indicates a talquetamab dosing day, the mid-size downward arrow indicates a talquetamab 0.4 mg/kg dosing day, the shorter downward arrow indicates a talquetamab step-up dosing day, the large-size upward arrow indicates a teclistamab dosing day, the mid-size dotted upward arrow indicates a teclistamab 1.5 mg/kg dosing day, the shorter upward arrow indicates a teclistamab step-up dosing day.
*From Cycle 5, if confirmed VGPR or better, can change to talquetamab Q4W dosing (Day 1 only) per investigator discretion.
**From Cycle 7, talquetamab Q4W for participants with response of confirmed PR or better. For all others, continue with Q2W dosing until confirmed PR or better is achieved.

Arm C (EPd or PVd): The dosing regimens for EPd are based on the approved label for the treatment of multiple myeloma, as detailed in the EMPLICITI USPI 2022 or EMPLICITI SmPC 2023. The dosing regimens for PVd are based on the standard doses used in clinical practice for the treatment of multiple myeloma, as detailed in the POMALYST USPI or IMNOVID SmPC 2022.

1.3. SoA

Table 1 summarizes the relevant SoAs and dosing schedules for each treatment regimen. Except as noted, all assessments will occur predose. Evaluations for eligibility determination performed outside the screening window may need to be repeated unless specified otherwise.

Table 1: Guide to SoAs and Dosing

	Arm A (Tal-P) and Arm B (Tal-Tec)	**Arm C (EPd)**	**Arm C (PVd)**
Screening	Table 2		
SoA	Table 3	Table 7	Table 10
Dosing schedule (and pretreatment medications, as applicable)	Table 4 Table 5	Table 8	Table 11
PK, immunogenicity, and biomarker sampling (as applicable)	Table 6	Table 9	Table 12
Bone marrow testing	Table 34		

1.3.1. SoA for Screening

Table 2: SoA for Screening (≤28d Before Randomization, Unless Noted Otherwise) – All Participants

Assessments	Notes
STUDY PROCEDURES	
Informed consent	
Eligibility criteria	
Demography, medical history and prior cancer therapies	
Disease characteristics	See Section 8.1.1.1.1
ECOG performance status	See Section 8.3.9
Physical examination including height and weight	Complete examination; see Section 8.3.1
Neurologic examination including ICE Tool	See Section 8.3.8 and Appendix 14
Vital signs including oxygen saturation	See Section 8.3.2
12-lead ECG	See Section 8.3.3
LABORATORY ASSESSMENTS: See Appendix 1	
Hematology and chemistry and coagulation	Local laboratory; see Section 8.3.4
HIV assessment	For participants with known HIV, local HIV testing by PCR and CD4 count are required if not performed as part of standard of care within 3 months prior to C1D1 (see inclusion criteria #17 [Section 5.1]); see Section 8.3.6
HBV and HCV screening	HBV: see Section 8.3.5.1 HCV: local testing is required if not performed as part of standard of care within 3 months before C1D1; see Section 8.3.5.2
Serum/urine pregnancy test	Local laboratory – for POCBP only. • Serum pregnancy test within 10-14d prior to C1D1 and either serum or urine pregnancy test within 24 h prior to C1D1
DISEASE EVALUATIONS: See Section 8.2	
Serum β2-microglobulin/albumin	• Central laboratory. In exceptional circumstances, local laboratory assessments may be used to establish measurable disease at screening (see Section 5.1); see Section 8.2.1 • If the 24-h urine collection (UPEP) begins the day before informed consent is obtained as part of SoC, the sample can be used for this study if it is sent to the central laboratory for analysis after the informed consent was obtained
Serum quantitative Ig	
SPEP, 24-h UPEP	
Serum FLC and SIFE/UIFE	
Bone marrow aspirate	See Section 8.2.2; disease characterization (morphology and either IHC, immunofluorescence, or flow cytometry for PC clonality) performed locally. If a biopsy was done within 42d before randomization, no need to repeat for morphology; however, fresh aspirate samples need to be obtained and sent to central laboratory for MRD, cytogenetics, immunophenotype and molecular markers
Imaging for disease evaluation (including assessment of lytic bone lesions)	See Section 8.2.4 for acceptable modalities If performed within 42d before randomization, does not need to be repeated at screening
Assessment of bone lesions and soft-tissue plasmacytomas	See Section 8.2.4 and Section 8.2.5 for acceptable modalities and instructions regarding biopsies If performed within 42d before randomization, does not need to be repeated at screening
ONGOING REVIEW	
AEs	Continuous from the time of signing of ICF
SPM	
Concomitant therapy (see Section 6.9)	
RANDOMIZATION	
If eligible	Eligible participants must be randomized within 3 business days of sponsor approval for randomization and must receive the C1D1 dose ≤7 calendar days after date randomized Hematology and chemistry eligibility criteria must be met again within 72 h prior to first dose of study treatment (see Table 3, Table 7, Table 10, and Section 5.1)

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

1.3.2. SoA for Arm A (Tal-P) and Arm B (Tal-Tec)

Table 3: SoA for Treatment Phase, EOT Visit, and Posttreatment Follow-up Phase – Arm A (Tal-P) and Arm B (Tal-Tec)

Assessments	Notes	Treatment Phase (28-day cycle)									Follow-up Phase
Cycle		Cycle 1				Cycle 2		Cycle 3+		EOT	
Day		1	4 (+2d)	8 (+2d)	15 (+2d)	1 (±3d)	15 (±3d)	1 (±3d)	15 (±3d)	≤30d after last dose (+7d) or before SST	**Pre-PD** Q28d (±7d) **Post-PD** Q16W (±14d)
STUDY PROCEDURES											
ECOG performance status	See Section 8.3.9 and Appendix 15	On Day 1 of C1-C4; Day 1 of C7, C10, C13; and Day 1 every 6 cycles thereafter								X	
Physical examination	Symptom- and disease-directed	Once per cycle									
Weight		X				X		X			
Venous thromboembolism prophylaxis (Arm A [Tal-P] only)	See Section 6.9.2.4 and Appendix 20	As clinically indicated									
Neurologic examination	See Section 8.3.8, see ICE Tool in Appendix 14	X	As clinically indicated								
Vital signs including oxygen saturation[a]	See Section 8.3.2	X	X	X	X	X	X	X	C3+[c]	X	
12-lead ECG	See Section 8.3.3	As clinically indicated									
TTE or MUGA Scan	For patients receiving pomalidomide	As clinically indicated. Monitor subjects for clinical signs or symptoms of cardiac failure or cardiac ischemia. Evaluate promptly if cardiac toxicity is suspected.									
LABORATORY ASSESSMENTS: See Appendix 1											
Hematology	Ensure all values meet criteria in Section 5.1 prior to C1D1 (perform within 72 h before dosing). From C1D8 may be performed within 48 h before dosing.	X	X	X	X	X	X	X	C3+[c]	X	
Chemistry		X		X	X	X		X		X	
TLS-related laboratory assessments	May be performed within 72 h before dosing, including for C1D1	X				X					
Hemoglobin A1c	Local laboratory	X						C5D1, C9D1 and D1 of every 4th cycle (±1 week) thereafter			
HBV, HCV	See Section 8.3.5.3	Perform at C1D1 (±7d). PCR testing Q12W (±4 weeks) up to 6 months after last dose of study treatment for participants with history of HBV infection/HCV antibody positivity									
HIV PCR and CD4 count	For participants with history of HIV antibody positivity, see Section 8.3.6	C1D1, C5D1, C9D1 and D1 of every 4th cycle (±1 week) thereafter								3 and 6 months (±1 month) after last dose	
Coagulation		As clinically indicated (eg, CRS)									
Serum or urine pregnancy test (POCBP only)	Additional testing may be required per the global PPP or local PPP/REMS program (Arm A [Tal-P] only; see Appendix 4)	Both arms: • Serum pregnancy test within 10-14d prior to C1D1 and either serum or urine pregnancy test within 24 h prior to C1D1								X	X[b] (until 6 months after last dose)

Assessments	Notes	Treatment Phase (28-day cycle)									Follow-up Phase
Cycle		Cycle 1				Cycle 2		Cycle 3+		EOT	
Day		1	4 (+2d)	8 (+2d)	15 (+2d)	1 (±3d)	15 (±3d)	1 (±3d)	15 (±3d)	≤30d after last dose (+7d) or before SST	**Pre-PD** Q28d (±7d) **Post-PD** Q16W (±14d)
		• Within 24 h of D1 of each cycle Arm A (Tal-P) only: • Weekly for first 4 weeks from start of pomalidomide treatment, including within 24 h prior to start of pomalidomide administration • Thereafter, Q28d (Q14d for participants with irregular menses)									
DISEASE EVALUATIONS: See Section 8.2											
Serum quantitative Ig	Central laboratory; local laboratory assessments may be used under specified circumstances (see Section 8.2.6). Sample may be collected on or within 7 d prior to CnD1. **NOTE**: SPEP and UPEP assessments will continue until 30 d after the start of the first SST for participants with no PD or unconfirmed PD (ie, single instance of PD by laboratory assessment). For participants with measurable disease by light chain, serum FLC will also continue during this period. *In the event of treatment delays, disease evaluations should occur every 28 days (±7 days).	X				X*		X*			X (until PD or start of SST)
SPEP		X				X*		X*			X (until PD or start of SST)
24-h UPEP		X				X*		X*			X (until PD or start of SST)
Serum FLC and SIFE/UIFE		• For participants with measurable disease only by light chain: with every disease evaluation* and, if applicable, each pre-PD disease evaluation visit until PD or 30 d after the start of SST. • For participants with measurable disease by serum and/or urine M spike: at time of suspected CR or sCR.									
Bone marrow aspirate	For local assessments; see Section 8.2.6 (a portion of aspirate should be sent to the central laboratory for biomarker and MRD analysis)	• Whenever CR or sCR are suspected • For participants with confirmed CR or sCR, collect additional samples 12 months post C1D1 (±3 months) and 12 months (±3 months) after time of confirmed CR or sCR, then every 12 months (±3 months) until PD. • Time of PD (even if occurs after treatment is discontinued; should occur prior to SST)									
Imaging	Disease evaluation including assessment of lytic bone lesions; see Section 8.2.4 for acceptable modalities	• As clinically indicated to document response or disease progression before start of SST									
Assessment of soft-tissue plasmacytomas	See Section 8.2.5 for acceptable modalities	• For participants with a history of soft-tissue plasmacytoma until development of confirmed CR, PD or start of SST. ○ For assessment by physical examination (if applicable), Q28d (±7d) from C1D1. ○ For assessment by radiology, Q12W (±7d) from C1D1. • As clinically indicated for other participants.									

Assessments	**Notes**	**Treatment Phase (28-day cycle)**									**Follow-up Phase**
Cycle		**Cycle 1**				**Cycle 2**		**Cycle 3+**		**EOT**	
Day		**1**	**4** (+2d)	**8** (+2d)	**15** (+2d)	**1** (±3d)	**15** (±3d)	**1** (±3d)	**15** (±3d)	**≤30d after last dose** (+7d) **or before SST**	**Pre-PD** Q28d (±7d) **Post-PD** Q16W (±14d)
PRO AND MRU: PRO ASSESSMENTS SHOULD BE COMPLETED BEFORE ANY CLINICAL TESTS OR PROCEDURES (see Section 8.2.7)											
MySIm-Q, EORTC-QLQ-C30, EQ-5D-5L, PGI-S, Epstein Taste Survey	Window for PRO data collection is 2d prior to dosing during Treatment Phase. Should continue to be collected even if SST has begun	To be collected on Day 1 of C1 to C12, then on Day 1 of every 3rd cycle								X	MySIm-Q, EORTC-QLQ-C30, and EQ-5D-5L only Q16W ±14d
MRU	See Section 8.9	Collected continuously after first day of study treatment until PD or 30d after last dose or the SST, whichever is earlier.									
Symptomatic progression evaluation		From 30d prior, to until 30d after confirmed PD (or the start of SST, whichever is earlier), progression-associated symptoms and concomitant medications will be documented in eCRF									
Qualitative interview	Optional; see Section 8.2.7.6	Single interview to be conducted approximately 12 months (±3 months) after the start of study treatment									
ONGOING REVIEW											
AEs	See Section 8.4	Continuous from signing of ICF until 30d after last dose or until the start of SST, if earlier; thereafter, continue to report any AEs/SAEs related to study treatment until EOS									
SPM		Continuous until EOS									
Concomitant therapy	See Section 6.9	Continuous from signing of ICF until 30d after last dose or until the start of SST, if earlier; thereafter, continue to report concomitant therapy given for any AEs/SAEs considered related to study treatment until EOS									
SST	See Section 6.10										X
Survival	See Section 8.1.1.4										Q16W

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

a. Vital signs to only be collected in the eCRF during Cycle 1 (including the step-up doses), and thereafter at any repeat step-up dose(s) and any additional vital signs assessments supporting the start and end dates of an AE.

b. Serum or urine pregnancy tests to be performed, as determined necessary by the investigator, or as required by local regulation, to establish the absence of pregnancy.

c. For participants on Q4W schedule assessments are not required at Cycle n Day 15 but may be performed as clinically indicated.

Table 4: Dosing Schedule and Pretreatment Medications – Arm A (Tal-P)

Cycle	Notes	Treatment Phase (28-day cycle)						
		Cycle 1				Cycle 2-6		Cycle 7+
Day		1	4	8	15	1 ±3d	15 ±3d	1 ±3d
REQUIRED PRETREATMENT MEDICATIONS: From Cycle 2 onwards, all PO pretreatment medications may be administered at home, provided they are taken within the time frames specified below. See Section 6.1.2.1 for event-driven required pretreatment medications.								
Dexamethasone 20 mg or equivalent (see Appendix 13)	• PO/IV: administer 1–3 h (±15 mins) prior to talquetamab SC/teclistamab SC • Prior to all step-up doses and first treatment dose in C1 • Diphenhydramine and acetaminophen: not required at Day 15 if the participant is in Q4W dosing schedule.	X	X	X	X	For participants who develop Grade ≥2 CRS/sARR related to talquetamab and/or teclistamab[a] (see Section 6.1.2.1).		
Diphenhydramine 25 to 50 mg, or equivalent		X	X	X	X			
Acetaminophen 650 to 1000 mg		X	X	X	X			
OPTIONAL PRETREATMENT MEDICATION: Additional pretreatment medications such as H_2 antagonists or antiemetics are optional and may be used at investigator discretion.								
STUDY DRUGS: See Section 6.1. Criteria for dose delays/skips and dose reductions are provided in Section 6.6. **Step-up and Treatment doses MUST be delayed until CRS and/or ICANS associated with prior dose have fully resolved** (see also criteria in Section 6.6.2.2).								
Talquetamab SC SU1 (0.01 mg/kg)[b,c]		X						
Talquetamab SC SU2 (0.06 mg/kg)[b,c]	• Administer ≥2d after SU1		X					
Talquetamab SC Treatment Dose (0.4 mg/kg)[b,c]	• Administer ≥2d after SU2			X				
Talquetamab SC Study Treatment Dose (0.8 mg/kg)[b,c]	• Administer ≥2d after first study treatment dose, between Days 7-15 • C2-C4 talquetamab SC must be administered 14d (±3d) after prior treatment dose *From C5, if confirmed VGPR or better, schedule can change to Q4W dosing (D1 only) per investigator discretion Change to Q4W dosing should occur on C5 or C6 Day 1 (±3d) #At C7 Day 1 (±3d), participants with response of confirmed PR or better must change to Q4W dosing. In exceptional cases only, participants can continue Q2W dosing after sponsor consultation and approval. For all participants who are not in confirmed PR or better at C7D1, continue with Q2W dosing until confirmed PR or better is achieved.				X	X	X*	X[#d]
Dexamethasone 40 mg or equivalent	• Administered weekly (Days 1, 8, 15, and 22) during C2-C4 only • For participants <75 years of age with BMI <18.5 or ≥75 years of age, the weekly dose of dexamethasone is 20 mg					X	X	
Pomalidomide[e]	• Administer ≥7d between the course of pomalidomide treatment for each cycle					Days 1-21[e]		Days 1-21[e]

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

a. Dispensing and compliance, as required for participants with at-home administration. Sites to confirm that any PO pretreatment medications dispensed for at-home administration were taken as directed before administration of study treatment, where applicable.

b. Participants receiving outpatient talquetamab/teclistamab are to check their body temperature (Section 6.1.3), and will receive daily phone call follow-up from after SU1 until 48 h after first treatment dose.

c. Participants receiving talquetamab/teclistamab may be hospitalized or receive outpatient clinic dosing with sponsor approval, if criteria are met (see Section 6.1.3.1.1 and Appendix 16). For outpatient clinic dosing, participants should remain in close proximity (within 30 min) to the site, in the company of a competent adult, starting after SU1 until 48 h after administration of the study treatment dose.

d. Participants will receive therapy with talquetamab/teclistamab (as appropriate) for up to 26 cycles if they have no sign of PD or toxicity. Participants can continue therapy with pomalidomide (Arm A [Tal-P] only) beyond Cycle 26 if no discontinuation criteria are met.

e. Pomalidomide (Arm A [Tal-P] only) will be initiated at 2 mg daily at C2D1, and may be increased to 4 mg daily per investigator discretion from C3D1. The selected dose regimen for pomalidomide is based on standard doses used in clinical practice for the treatment of multiple myeloma.

Table 5: Dosing Schedule and Pretreatment Medications – Arm B (Tal-Tec)

Cycle	Notes	Treatment Phase (28-day cycle)						
		Cycle 1				Cycle 2-6		Cycle 7+
Day		1	4	8	15	1 ±3d	15 ±3d	1 ±3d
REQUIRED PRETREATMENT MEDICATIONS: From Cycle 2 onwards, all PO pretreatment medications may be administered at home, provided they are taken within the time frames specified below. See Section 6.1.2.1 for event-driven required pretreatment medications.								
Dexamethasone 20 mg or equivalent (see Appendix 13)	• PO/IV: administer 1–3 h (±15 mins) prior to talquetamab SC/teclistamab SC • Prior to all step-up doses and first treatment dose in C1	X	X	X	X	For participants who develop Grade ≥2 CRS/sARR related to talquetamab and/or teclistamab[a] (see Section 6.1.2.1).		
Diphenhydramine 25 to 50 mg, or equivalent		X	X	X	X			
Acetaminophen 650 to 1000 mg		X	X	X	X			
OPTIONAL PRETREATMENT MEDICATION: Additional pretreatment medications such as H_2 antagonists or antiemetics are optional and may be used at investigator discretion.								
STUDY DRUGS: See Section 6.1. Criteria for dose delays/skips and dose reductions are provided in Section 6.6. **Step-up and Treatment doses MUST be delayed until CRS and/or ICANS associated with prior dose have fully resolved** (see also criteria in Section 6.6.2.2).								
Talquetamab SC SU1 (0.01 mg/kg)[b,c]		X						
Talquetamab SC SU2 (0.06 mg/kg)[b,c]	• Administer ≥2d after SU1		X					
Talquetamab SC Treatment Dose (0.4 mg/kg)[b,c]	• Administer ≥2d after SU2			X				
Talquetamab SC Study Treatment Dose (0.8 mg/kg)[b,c]	• Administer ≥2d after first study treatment dose, between Days 7-15 • C2-C4 talquetamab SC must be administered 14d (±3d) after prior treatment dose *From C5, if confirmed VGPR or better, schedule can change to Q4W dosing (D1 only) per investigator discretion. Change to Q4W dosing should occur on Cycle 5 or Cycle 6 Day 1 (±3d) #At Cycle 7 Day 1 (±3d), participants with response of confirmed PR or better must change to Q4W dosing. In exceptional cases only, participants can continue Q2W dosing after sponsor consultation and approval. For all participants who are not in confirmed PR or better at C7D1, continue with Q2W dosing until confirmed PR or better is achieved.				X	X	X*	X[#d]
Teclistamab SC SU1[b,c] (0.06 mg/kg)		X						
Teclistamab SC SU2[b,c] (0.3 mg/kg)	• Administer ≥2d after SU1		X					
Teclistamab SC Treatment Dose (1.5 mg/kg)	• Administer ≥2d after SU2 • Administer ≥2d after first treatment dose, between Days 7-15			X	X			
Teclistamab SC Study Treatment Dose (3 mg/kg)	• From the study treatment at C2, Q4W dosing (D1 only) for all participants					X		X[d]

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

a. Dispensing and compliance, as required for participants with at-home administration. Sites to confirm that any PO pretreatment medications dispensed for at-home administration were taken as directed before administration of study treatment, where applicable.

b. Participants receiving outpatient talquetamab/teclistamab are to check their body temperature (Section 6.1.3), and will receive daily phone call follow-up from after SU1 until 48 h after first treatment dose.

c. Participants receiving talquetamab/teclistamab may be hospitalized or receive outpatient clinic dosing with sponsor approval, if criteria are met (see Section 6.1.3.1.1 and Appendix 16). For outpatient clinic dosing, participants should remain in close proximity (within 30 min) to the site, in the company of a competent adult, starting after SU1 until 48 h after administration of the study treatment dose.

Table 6: Sampling for PK, Immunogenicity, and Biomarkers for Treatment Phase, EOT Visit, and Posttreatment Follow-up Phase – Arm A (Tal-P) and Arm B (Tal-Tec)

Assessments		Time[a]	Treatment Phase (28-day cycle)							Follow-up Phase[c]
Cycle			Cycle 1		Cycle 2		Cycle 3-Cycle 7	Cycle 12	EOT[c]	
Day			1	15[f]	1	15	1	1	≤30d after last dose (+7d) or before SST	8 weeks after last dose
PK AND IMMUNOGENICITY SAMPLING[h]										
Talquetamab (Arm A [Tal-P] and Arm B [Tal-Tec]) Teclistamab (Arm B [Tal-Tec] only)		Predose on day of dosing	X		X		Collect sample at dose regimen change[d]	C12, then every 6 cycles	X	X
		Suspected sARR, CRS, or neurotoxicity/ICANS Grade ≥2	Collect additional PK/immunogenicity sample as soon as sARR, CRS, or neurotoxicity/ICANS Grade ≥2 is detected, if feasible.							
BIOMARKER SAMPLING[b]: See Section 8.7. For bone marrow aspirate, see Table 34 for Disease Evaluation										
Immunophenotyping[e] (whole blood)		Predose on day of dosing	X			X	C5, C7	C12 only	X	
		Response-based	Time of PD							
Circulating proteins (serum)	M-protein Mass Spectrometry	Predose on day of dosing	X				C3, C5			
		Response-based	• Time of suspected CR or sCR • For participants with confirmed CR or sCR, additional samples will be collected 6 (±3 months) and 12 months (±3 months) after time of confirmed CR or sCR, and then every 12 months (±3 months) until PD • Time of PD							
	Cytokines/ ferritin/ CRP	Predose on day of dosing	X	X[f]		X				
Molecular markers/ CyTOF[g] (whole blood)		Predose on day of dosing	X			X	C5		X	
Biopsy of soft-tissue plasmacytomas[e]		As clinically indicated	See Section 8.2.5							

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

a. If a dose is delayed or missed, then sample collection should be performed on the actual administration day, not on the originally scheduled administration day.
b. Predose sample window for biomarker samples: -4 to 0h.
c. Sample window +7d only for PK and immunogenicity sampling.
d. For intra-participant dose regimen change between C3 and C7, a PK/immunogenicity sample is to be collected prior to the administration of the changed dose regimen. This should be done each time the participant's talquetamab or teclistamab dose regimen is modified.
e. Immunophenotyping testing will be performed in approximately 50% of the participants.
f. Samples should be collected at the last step-up dose.
g. Molecular marker/CyTOF assessments and biomarker assessments for soft-tissue plasmacytomas will not be performed for participants enrolling in China.
h. Predose sample window for PK samples: -24 to 0h.

1.3.3. SoA for Arm C (EPd)

Table 7: SoA for Treatment Phase, EOT Visit, and Posttreatment Follow-up Phase – Arm C (EPd)

Assessments	Notes	Treatment Phase (28-day cycle)						Follow-up Phase
Cycle		Cycle 1-2				Cycle 3+	EOT	
Day		1 (±3d)	8 (±2d)	15 (±2d)	22 (±2d)	1 (±3d)	≤30d after last dose (+7d) or before SST	Pre-PD Q28d (±7d) Post-PD Q16W (±14d)
STUDY PROCEDURES								
ECOG performance status	See Section 8.3.9 and Appendix 15	On Day 1 of C1-C4 and Day 1 of C7, C10, C13, and D1 of every 6 cycles thereafter					X	
Physical examination	Symptom- and disease-directed	Once per cycle						
Weight		X				X		
Venous thromboembolism prophylaxis	See Section 6.9.2.4 and Appendix 20	As clinically indicated						
Vital signs including oxygen saturation	See Section 8.3.2	X	X	X	X	X	X	
12-lead ECG	See Section 8.3.3	As clinically indicated						
TTE or MUGA Scan		As clinically indicated. Monitor subjects for clinical signs or symptoms of cardiac failure or cardiac ischemia. Evaluate promptly if cardiac toxicity is suspected.						
LABORATORY ASSESSMENTS: See Appendix 1								
Hematology	Ensure all values meet criteria in Section 5.1 prior to C1D1 (perform within 72 h before dosing). From C1D8 may be performed within 48 h before dosing.	X	X	X	X	X	X	
Chemistry		X				X	X	
Uric acid	May be performed within 72 h before dosing, including for C1D1	X						
Hemoglobin A1c	Local laboratory	C1				C5D1, C9D1 and D1 of every 4th cycle (±1 week) thereafter		
HBV, HCV	See Section 8.3.5.3	Perform at C1D1 (±7d). PCR testing **Q12W** (±4 weeks) up to 6 months after last dose of study treatment for participants with history of HBV infection/HCV antibody positivity						
HIV PCR and CD4 count	For participants with history of HIV antibody positivity, see Section 8.3.6	C1D1, C5D1, C9D1 and D1 of every 4th cycle (±1 week) thereafter					3 and 6 months (±1 month) after last dose	
Coagulation		As clinically indicated						
Serum or urine pregnancy test (POCBP only)	Additional testing may be required per the global PPP or local PPP/REMS program (see Appendix 4)	• Serum pregnancy test within 10-14d prior to C1D1 and either serum or urine pregnancy test within 24 h prior to C1D1 • Within 24 h of D1 of each cycle • Weekly for the first 4 weeks of pomalidomide treatment, including within 24 h prior to start of pomalidomide administration • Thereafter, Q28d (or Q14d for participants with irregular menses)					X	X[a] (until 6 months after last dose)
DISEASE EVALUATIONS: See Section 8.2								
Serum quantitative Ig	Central laboratory; local laboratory assessments may be used under specified circumstances (see	X				X*	X	X (until PD or start of SST)
SPEP		X				X*	X	X (until PD or start of SST)

Assessments	Notes	Treatment Phase (28-day cycle)						Follow-up Phase
Cycle		Cycle 1-2				Cycle 3+	EOT	
Day		1 (±3d)	8 (±2d)	15 (±2d)	22 (±2d)	1 (±3d)	≤30d after last dose (+7d) or before SST	Pre-PD Q28d (±7d) Post-PD Q16W (±14d)
24-h UPEP	Section 8.2.6); sample may be collected on or within 7d prior to CnD1	X				X*	X	X (until PD or start of SST)
Serum FLC and SIFE/UIFE	**NOTE**: SPEP and UPEP assessments will continue until 30d after the start of the first SST for participants with no PD or unconfirmed PD (ie, single instance of PD by laboratory assessment). For participants with measurable disease by light chain, serum FLC will also continue during this period. *In the event of treatment delays, disease evaluations should occur every 28 days (±7 days).	• For participants with measurable disease only by light chain: with disease evaluation* and, if applicable, each pre-PD disease evaluation visit until PD or 30 d after the start of SST. • For participants with measurable disease by serum and/or urine M spike: at time of suspected CR or sCR.						
Bone marrow aspirate	For local assessments; see Section 8.2.6 (a portion of aspirate should be sent to the central laboratory for biomarker and MRD analysis)	• Whenever CR or sCR are suspected • For participants with confirmed CR or sCR, collect additional samples 12 months post C1D1 (±3 months) and 12 months (±3 months) after time of confirmed CR or sCR, then every 12 months (±3 months) until PD. • Time of PD (even if occurs after treatment is discontinued; should occur prior to SST)						
Imaging	Disease evaluation including assessment of lytic bone lesions; see Section 8.2.4 for acceptable modalities	• As clinically indicated to document response or disease progression before start of SST						
Assessment of soft-tissue plasmacytomas	See Section 8.2.5 for acceptable modalities	• For participants with a history of soft-tissue plasmacytoma until development of confirmed CR, PD or start of SST ○ For assessment by physical examination (if applicable), Q28d (±7d) from C1D1 ○ For assessment by radiology, Q12W (±7d) from C1D1 • As clinically indicated for other participants						
PRO AND MRU: PRO ASSESSMENTS SHOULD BE COMPLETED BEFORE ANY CLINICAL TESTS OR PROCEDURES (see Section 8.2.7)								
MySIm-Q, EORTC-QLQ-C30, EQ-5D-5L, PGI-S, Epstein Taste Survey	Window for PRO data collection is 2d prior to dosing during Treatment Phase. Should continue to be collected even if SST has begun	To be collected on Day 1 of C1 to C12, then on Day 1 of every 3rd cycle					X	MySIm-Q, EORTC-QLQ-C30, and EQ-5D-5L only Q16W ±14d
MRU	See Section 8.9	Collected continuously from the start of study treatment until 30d after last dose or the start of SST, whichever is earlier.						
Symptomatic progression evaluation		From 30d prior, to until 30d after confirmed PD (or the start of SST, whichever is earlier), progression-associated symptoms and concomitant medications will be documented in eCRF						
Qualitative interview	Optional; see Section 8.2.7.6.	Single interview to be conducted approximately 12 months (±3 months) after the start of study treatment						
ONGOING REVIEW								
AEs	See Section 8.4	Continuous from signing of ICF until 30d after last dose or until the start of SST, if earlier; thereafter, continue to report any AEs/SAEs related to study treatment until EOS						
SPM		Continuous until EOS						

Assessments	Notes	Treatment Phase (28-day cycle)							Follow-up Phase
Cycle		Cycle 1-2				Cycle 3+		EOT	
Day		1 (±3d)	8 (±2d)	15 (±2d)	22 (±2d)	1 (±3d)		≤30d after last dose (+7d) or before SST	Pre-PD Q28d (±7d) Post-PD Q16W (±14d)
Concomitant therapy	See Section 6.9	Continuous from signing of ICF until 30d after last dose or until the start of SST, if earlier; thereafter, continue to report concomitant therapy given for any AEs/SAEs considered related to study treatment until EOS							
SST	See Section 6.10								X
Survival	See Section 8.1.1.4								Q16W

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

a. Serum or urine pregnancy tests to be performed, as determined necessary by the investigator, or as required by local regulation, to establish the absence of pregnancy.

Table 8: Dosing Schedule and Pretreatment Medications – Arm C (EPd)

	Notes	Treatment Phase (28-day cycle)											
Cycle		Cycle 1				Cycle 2				Cycle 3+			
Day		1	8	15	22	1	8	15	22	1	8	15	22
REQUIRED PRETREATMENT MEDICATIONS: From Cycle 2 onwards, all PO pretreatment medications may be administered at home, provided they are taken within the time frames specified below.													
Acetaminophen 650 to 1000 mg PO	• Administer 45–90 min before elotuzumab	X	X	X	X	X	X	X	X	X			
Diphenhydramine (25 to 50 mg IV or PO) or equivalent		X	X	X	X	X	X	X	X	X			
Locally available H2 blocker[a]		X	X	X	X	X	X	X	X	X			
Dexamethasone 8 mg IV/PO		X	X	X	X	X	X	X	X	X			
STUDY TREATMENT ADMINISTRATION: Criteria for dose delays/skips and dose modifications are provided in Section 6.6.													
Elotuzumab IV	10 mg/kg for Cycles 1 and 2, and 20 mg/kg for Cycles 3+	X	X	X	X	X	X	X	X	X			
Pomalidomide PO	4 mg/day There must be ≥7d between the course of pomalidomide for each cycle	Days 1-21				Days 1-21				Days 1-21			
Dexamethasone PO	≤75 years of age (between 3-24 h before elotuzumab)	28 mg	28 mg	28 mg	28 mg	28 mg	28 mg	28 mg	28 mg	28 mg	40 mg	40 mg	40 mg
Dexamethasone PO	>75 years of age (between 3-24 h before elotuzumab)	8 mg	8 mg	8 mg	8 mg	8 mg	8 mg	8 mg	8 mg	8 mg	20 mg	20 mg	20 mg

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

a. H2 blockers that can be used include famotidine 20 mg IV or ranitidine 50mg IV or other locally available H2 blockers.

Table 9: Sampling for Biomarkers for Treatment Phase, EOT Visit, and Posttreatment Follow-up Phase – Arm C (EPd)

Assessments		Time	Treatment Phase (28-day cycle)			Follow-up Phase
Cycle			Cycle 1-2	Cycle 3+	EOT	
Day			1	1	≤30d after last dose (+7d) or before SST	8 weeks after last dose
BIOMARKER SAMPLING: See Section 8.7. For bone marrow aspirate, see Table 34 for Disease Evaluation						
Immunophenotyping[a] (whole blood)		Predose on the day of dosing[c]	C1			
Circulating proteins (serum/plasma)	M-Protein Mass spectrometry	Predose on the day of dosing[c]	C1	C5	X	
		Response-based	• Time of suspected CR or sCR • For participants with confirmed CR or sCR, additional samples will be collected 6 months (±3 months) and 12 months (±3 months) after time of confirmed CR or sCR, and then every 12 months (±3 months) until PD			
Molecular markers/CyTOF[b] (whole blood)		Predose on the day of dosing[c]	C1	C5	X	

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.
a. Immunophenotyping testing will be performed in approximately 50% of the participants in Arm C and only at C1D1.
b. Molecular marker/CyTOF assessments will not be performed for participants enrolling in China.
c. Predose sample window for biomarker samples: -4 to 0h.

1.3.4. SoA for Arm C (PVd)

Table 10: SoA for Treatment Phase, EOT Visit, and Posttreatment Follow-up Phase – Arm C (PVd)

Assessments	Notes	Treatment Phase (21-day cycle)							Follow-up Phase
Cycle		Cycle 1-8				Cycle 9+		EOT	
Day		1 (±3d from Cycle 2)	4 (+2d)	8 (+2d)	11 (+2d)	1 (±3d)	8 (±2d)	≤30d after last dose (+7d) or before SST	Pre-PD Q28d (±7d) Post-PD Q16W (±14d)
STUDY PROCEDURES									
ECOG performance status	See Section 8.3.9 and Appendix 15	X				X		X	
Physical examination	Symptom- and disease-directed	Once per cycle							
Weight		X				X			
Venous thromboembolism prophylaxis	See Section 6.9.2.4 and Appendix 20	As clinically indicated							
Vital signs including oxygen saturation	See Section 8.3.2	X		C1-C3	C1-C3	X		X	
12-lead ECG	See Section 8.3.3	As clinically indicated							
Chest imaging[b]	Prior to administration of bortezomib[b]								
TTE or MUGA Scan	Ejection fraction	As clinically indicated. Monitor subjects for clinical signs or symptoms of cardiac failure or cardiac ischemia. Evaluate promptly if cardiac toxicity is suspected.							
LABORATORY ASSESSMENTS: See Appendix 1.									
Hematology	Ensure all values meet criteria in Section 5.1 prior to C1D1 (perform within 72 h before	X	C1-C3	C1-C3	C1-C3	X		X	
Chemistry		X		C1	C1	X			

Assessments	Notes	Treatment Phase (21-day cycle)							Follow-up Phase
Cycle		Cycle 1-8				Cycle 9+		EOT	
Day		1 (±3d from Cycle 2)	4 (+2d)	8 (+2d)	11 (+2d)	1 (±3d)	8 (±2d)	≤30d after last dose (+7d) or before SST	Pre-PD Q28d (±7d) Post-PD Q16W (±14d)
	dosing). From C1D8 may be performed within 48 h before dosing.								
Uric acid	Perform ≤72 h before dosing	C1							
Hemoglobin A1c	Local laboratory	C1, C5				C9D1 and D1 of every 4th cycle (±1 week) thereafter			
HBV, HCV	See Section 8.3.5.3	Perform at C1D1 (±7d). PCR testing **Q12W** (±4 weeks) up to 6 months after last dose of study treatment for participants with history of HBV infection/HCV antibody positivity							
HIV PCR and CD4 count	For participants with history of HIV antibody positivity, see Section 8.3.6	C1, C5				C9D1 and D1 of every 4th cycle (±1 week) thereafter		3 and 6 months (±1 month) after last dose	
Coagulation		As clinically indicated							
Serum or urine pregnancy test (POCBP only)	Additional testing with sufficient sensitivity may be required per the global PPP or local PPP/REMS program (see Appendix 4)	• Serum pregnancy test within 10-14d prior to C1D1 and either serum or urine pregnancy test within 24 h prior to C1D1 • Within 24 h of D1 of each cycle • Weekly for the first 4 weeks of pomalidomide treatment, including within 24 h prior to start of pomalidomide administration • Thereafter, Q21d (or Q14d for participants with irregular menses)						X	X[a] (until 6 months after last dose)
DISEASE EVALUATIONS									
Serum quantitative Ig	Central laboratory; local laboratory assessments may be used under specified circumstances (see Section 8.2.6); sample may be collected on or within 7d prior to CnD1 **NOTE**: SPEP and UPEP assessments will continue until 30d after the start of the first SST for participants with no PD or unconfirmed PD (ie, single instance of PD by laboratory assessment). For participants with measurable disease by light chain, serum FLC will also continue during this period. *In the event of treatment delays, disease evaluations should occur every 28 days (±7 days).	X				X*		X (until PD or start of SST)	
SPEP		X				X*		X (until PD or start of SST)	
24-h UPEP		X				X*		X (until PD or start of SST)	
Serum FLC and SIFE/UIFE		• For participants with measurable disease only by light chain: with every disease evaluation*and, if applicable, each pre-PD disease evaluation visit until PD or 30 d after the start of SST. • For participants with measurable disease by serum and/or urine M spike: at time of suspected CR or sCR.							
Bone marrow aspirate	For local assessments; see Section 8.2.6 (a portion of aspirate should be sent to the central laboratory for biomarker and MRD analysis)	• Whenever CR or sCR are suspected • For participants with confirmed CR or sCR, collect additional samples 12 months post C1D1 (±3 months) and 12 months (±3 months) after time of confirmed CR or sCR, then every 12 months (±3 months) until PD. • Time of PD (even if occurs after treatment is discontinued; should occur prior to SST)							
Imaging	Disease evaluation including assessment of lytic bone lesions; see Section 8.2.4 for acceptable modalities	• As clinically indicated to document response or progression before start of SST							

Assessments	Notes	Treatment Phase (21-day cycle)							Follow-up Phase
Cycle		Cycle 1-8				Cycle 9+		EOT	
Day		1 (±3d from Cycle 2)	4 (+2d)	8 (+2d)	11 (+2d)	1 (±3d)	8 (±2d)	≤30d after last dose (+7d) or before SST	Pre-PD Q28d (±7d) Post-PD Q16W (±14d)
Assessment of soft-tissue plasmacytomas	See Section 8.2.5 for acceptable modalities	• For participants with a history of soft-tissue plasmacytoma ○ For assessment by physical examination (if applicable), Q3W (±1 week) from C1D1 until development of confirmed CR or PD or start of SST ○ For assessment by radiology, Q12W (±1 week) from C1D1 until confirmed CR or PD or start of SST • As clinically indicated for other participants							
PRO AND MRU: PRO ASSESSMENTS SHOULD BE COMPLETED BEFORE ANY CLINICAL TESTS OR PROCEDURES (see Section 8.2.7)									
MySIm-Q, EORTC-QLQ-C30, EQ-5D-5L, PGI-S, Epstein Taste Survey	Window for PRO data collection is 2d prior to dosing during Treatment Phase. Should continue to be collected even if SST has begun	To be collected on Day 1 of C1 to C12, then on Day 1 of every 3rd cycle						X	MySIm-Q, EORTC-QLQ-C30, and EQ-5D-5L only Q16W ±14d
MRU	See Section 8.9	Collected continuously from the start of study treatment until 30d after last dose or the start of SST, whichever is earlier.							
Symptomatic progression evaluation		From 30d prior, to until 30d after confirmed PD (or the start of SST, whichever is earlier), progression-associated symptoms and concomitant medications will be documented in eCRF							
Qualitative interview	Optional; see Section 8.2.7.6	Single interview to be conducted approximately 12 months (±3 months) after the start of study treatment							
ONGOING REVIEW									
AEs	See Section 8.4	Continuous from signing of ICF until 30d after last dose or until the start of SST, if earlier; thereafter, continue to report any AEs/SAEs related to study treatment until EOS							
SPM		Continuous until EOS							
Concomitant therapy	See Section 6.9	Continuous from signing of ICF until 30d after last dose or until the start of SST, if earlier; thereafter, continue to report concomitant therapy given for any AEs/SAEs considered related to study treatment until EOS							
SST	See Section 6.10								X
Survival	See Section 8.1.1.4								Q16W

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

a. Serum or urine pregnancy tests to be performed, as determined necessary by the investigator, or as required by local regulation, to establish the absence of pregnancy.

b. Baseline before start of bortezomib for potential post-treatment pulmonary changes. Other imaging modality is acceptable per investigator discretion.

Table 11: Dose Schedule – Arm C (PVd)

	Notes	Treatment Phase (21-day cycle)											
Cycle		Cycle 1-8								Cycle 9+			
Day		1	2	4	5	8	9	11	12	1	2	8	9
Pomalidomide PO	4 mg/day There must be ≥7d between the course of pomalidomide for each cycle	Days 1-14								Days 1-14			
Bortezomib SC	1.3 mg/m^2 There must be ≥3d between doses of bortezomib	X		X		X		X		X		X	
Dexamethasone PO	20 mg/day (≤75 years of age) or 10 mg/day (>75 years of age)	X	X	X	X	X	X	X	X	X	X	X	X

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

Table 12: Sampling for Biomarkers for Treatment Phase, EOT Visit, and Posttreatment Follow-up Phase – Arm C (PVd)

Assessments		Time	Treatment Phase (21-day cycle)			Follow-up Phase
Cycle			Cycle 1-2	Cycle 3+	EOT	
Day			1	1	≤30d after last dose (+7d) or before SST	8 weeks after last dose
BIOMARKER SAMPLING: See Section 8.7. For bone marrow aspirate, see Table 34 for Disease Evaluation						
Immunophenotyping[a] (whole blood)		Predose on the day of dosing[c]	C1			
Circulating proteins (serum/plasma)	M-Protein Mass spectrometry	Predose on the day of dosing[c]	C1	C5	X	
		Response-based	• Time of suspected CR or sCR • For participants with confirmed CR or sCR, additional samples will be collected 6 (±3 months) and 12 months (±3 months) after time of confirmed CR or sCR, and then every 12 months (±3 months) until PD			
Molecular markers/CyTOF[b] (whole blood)		Predose on the day of dosing[c]	C1	C5	X	

Abbreviations: see ABBREVIATIONS AND DEFINITIONS OF TERMS.

a. Immunophenotyping testing will be performed in approximately 50% of the participants in Arm C and only at C1D1.
b. Molecular marker/CyTOF assessments will not be performed for participants enrolling in China.
c. Predose sample window for biomarker samples: -4 to 0h.

2. INTRODUCTION

For the most comprehensive nonclinical and clinical information regarding talquetamab and teclistamab, refer to the latest version of the IB and Addenda for talquetamab and teclistamab.

The term "study treatment" throughout the protocol, refers to talquetamab, teclistamab, pomalidomide, elotuzumab, bortezomib, and dexamethasone as defined in Section 6.1 Study Treatment(s) Administered.

The term "sponsor" used throughout this document refers to the entities listed in the Contact Information page(s), which will be provided as a separate document. The term "participant" throughout the protocol refers to the common term "subject".

A list of abbreviations used in this document is provided in ABBREVIATIONS AND DEFINITIONS OF TERMS.

2.1. Study Rationale

2.1.1. Multiple Myeloma

Multiple myeloma is characterized by proliferation of neoplastic clones of PCs derived from B cells, which produce M-proteins comprised of pathologic Ig, or fragments of such, which have lost their function (Kyle 2009, Palumbo 2011). These neoplastic clones grow in the bone marrow, frequently invade adjacent bone, disrupt both bone homeostasis and hematopoiesis, and cause multifocal destructive lesions throughout the skeleton that result in bone pain and fracture (Chung 2017). Common clinical presentations of multiple myeloma are hypercalcemia, renal insufficiency, anemia, bony lesions, bacterial infections, hyperviscosity, and secondary amyloidosis (Orlowski 2013). Multiple myeloma is an incurable, malignant, PC disorder that accounts for approximately 18% of hematologic malignancies and 1.8% of all new cancer cases (Kumar 2020). It is the second most common hematologic malignancy, with 35,842 cases and approximately 23,275 deaths estimated in the EU-27 countries in 2020 (ECIS 2020). The American Cancer Society projects that 34,470 new cases of multiple myeloma and 12,640 deaths will have occurred in the US in 2022 (American Cancer Society 2022). The estimated 5-year relative survival rate in the US patients with multiple myeloma diagnosed from 2012 to 2018 was 57.9% (SEER 2021). By race and ethnicity, the incidence and death rates are highest in the non-Hispanic black population (American Cancer Society 2022; Marinac 2020).

Therapeutic options for multiple myeloma include agents such as PIs, IMiDs, mAbs (eg, daratumumab and isatuximab [anti-CD38] and elotuzumab [anti-SLAMF7]), BCMA-targeted therapies (eg, teclistamab), novel chemotherapy, and stem cell transplantation. Treatment options for multiple myeloma have substantially improved over time and vary depending on the aggressiveness of the disease, underlying prognostic factors, physical condition of the patient, prior therapies, and existing co-morbidities. Despite these therapeutic achievements, the disease most often recurs and remains incurable.

For patients whose myeloma relapses after exposure to daratumumab and lenalidomide in the first or subsequent lines of therapy, treatment options are limited. In a retrospective review of patients

who were refractory to anti-CD38 therapy and who received subsequent therapy after progression on PI, IMiD, and anti-CD38 therapy, the observed efficacy of the next treatment after was dismal, regardless of the salvage therapy chosen (Gandhi 2019). Therefore, there remains a significant and critical unmet need for new therapeutic options directed at alternative mechanisms of action that can better control the disease, provide deeper, more sustained responses, and better long-term outcomes including maintenance of HRQoL (Usmani 2016).

Recently, several approaches were developed to redirect T lymphocytes (T cells) to tumor surface antigens leading to antitumor activity and clinical benefit for patients. Available results suggest that redirecting T cells to tumor surface antigens by using bispecific approaches provides an effective means to harness the immune system to cause cancer cell death and to create meaningful and long-lasting clinical responses (BLENREP USPI 2022; Topp 2011).

Synergistic combinations that target various mechanisms to overcome drug resistance need to be examined in order to realize the full potential of talquetamab and may lead to a better response. To this end, talquetamab is being investigated as part of multidrug regimens. This study is designed to encompass a broad population of patients with unmet medical needs based on different exposures and refractoriness to prior therapies. This Phase 3 study will assess efficacy and safety of talquetamab in combination with pomalidomide or teclistamab compared to investigator's choice of therapy – ie, EPd or PVd in participants with relapsed/refractory multiple myeloma who have received 1 to 4 prior lines of therapy including an anti-CD38 mAb and lenalidomide. The development of an effective, non-anti-CD38 option is expected to address a significant unmet medical need for the treatment of relapsed and refractory multiple myeloma.

2.1.2. GPRC5D

GPRC5D is a 7-transmembrane receptor protein that is classified as a type C G-protein-coupled receptor based on the sequence homology score (Bräuner-Osborne 2001). GPRC5D is an orphan receptor whose ligand and signaling mechanisms are yet to be identified. GPRC5D receptor is a 354 amino acid protein with a short 27 amino acid N-terminus unlike other family members. GPRC5D mRNA is expressed in all malignant PCs from participants with multiple myeloma (Atamaniuk 2012; Frigyesi 2014; Pillarisetti 2020). Levels of GPRC5D expression in participants with multiple myeloma correlated well with PC burden and genetic aberrations such as retinoblastoma-1 deletion (Atamaniuk 2012).

Comprehensive analysis of GPRC5D normal human tissue expression, including both data from the public domain (Inoue 2004; Venkateshaiah 2013; Kodama 2019; Smith 2019) and internally generated data (Goldsmith 2021, Nonclinical Study Report Histo-06292022), showed that GPRC5D expression is limited to normal PCs and epithelial cells in hair follicles (hair bulb and shaft), eccrine sweat glands of the skin, and filiform papillae of the tongue. The relatively restricted expression of GPRC5D makes it a relatively low risk target for on-target/off-tumor toxicity and designates it as a suitable target for T-lymphocyte (T-cell) mediated therapy to treat PC disorders like multiple myeloma.

2.1.3. BCMA

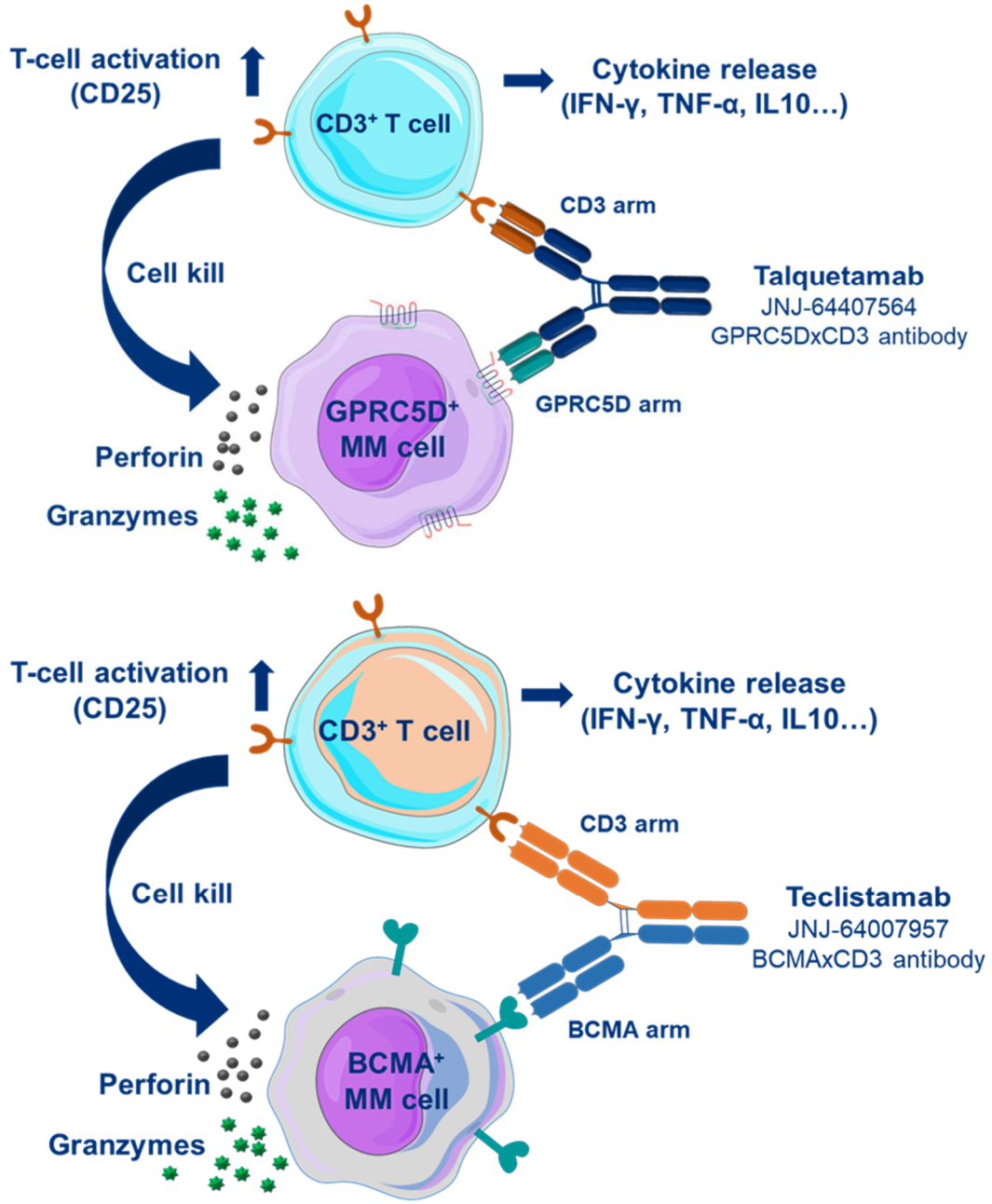
BCMA (also known as CD269 and TNFRSF17) is a 20 kD, Type III membrane protein that is part of the tumor necrosis receptor family (Tai 2015). BCMA is predominantly expressed in B lineage cells and selectively induced during PC differentiation (Carpenter 2013; Darce 2007; Maus 2013). IHC on normal tissues using commercially available anti-BCMA antibodies demonstrated that BCMA is a highly restricted target, with expression limited to normal B cells and PCs in lymph node, spleen, bronchial associated lymphoid tissue, and mucosal associated lymphoid tissue (Bu 2018). BCMA mRNA and protein were universally detected in multiple myeloma cell lines and in all malignant PCs from patients with multiple myeloma by the sponsor and others (Carpenter 2013; Novak 2004). These expression characteristics make BCMA an ideal therapeutic target for the treatment of multiple myeloma (Darce 2007; Tai 2015).

BCMA has been established as a validated target in multiple myeloma with multiple therapeutic modalities demonstrating promising efficacy and acceptable safety profiles in heavily pretreated patients. Idecabtagene vicleucel, a CAR-T targeting BCMA, was approved in the US and EU in 2021 based on results demonstrating an ORR of 73% in the treated population, which was a heavily pretreated, relapsed/refractory patient population (Munshi 2021). Ciltacabtagene autoleucel, also a CAR-T therapy targeting BCMA, was approved in the US in 2022 based on results demonstrating an ORR of 97% in the treated population, which was also heavily pretreated (Berdeja 2021). Teclistamab monotherapy is approved by the European Medicines Agency, the US FDA, and the UK Medicines and Healthcare products Regulatory Agency for the treatment of adult patients with relapsed or refractory multiple myeloma who have received prior therapies (at least 3 [EU, UK]; at least 4 [US]), including a PI, an IMiD and an anti-CD38 mAb and have demonstrated disease progression on the last therapy (TECVAYLI EU SmPC 2023; TECVAYLI UK SmPC 2022; TECVAYLI USPI 2022).

2.1.4. CD3 Redirection

Talquetamab and teclistamab are bispecific antibodies with normal IgG structures that bind the CD3 receptor complex on T cells and GPRC5D or BCMA, respectively, on PCs (Figure 4). As a stable bispecific IgG molecules generated through controlled fragment antigen binding arm exchange, talquetamab and teclistamab have the benefit of a long serum half-life (Labrijn 2013; Pillarisetti 2016; Yu 2019). The dual binding sites on each study drug allow talquetamab and teclistamab to draw CD3-expressing T cells in close proximity to myeloma cells, without regard to T-cell receptor specificity or reliance on major histocompatibility complex Class 1 molecules on the surface of antigen presenting cells for activation, leading to cell death of the GPRC5D- or BCMA-expressing cells.

Figure 4: Schematic Overview of the Mechanism of Action of the Study Drugs



2.1.5. Rationale for Combining Talquetamab and Pomalidomide

IMiDs, such as lenalidomide and pomalidomide, are used in several current multiple myeloma treatment regimens. IMiDs can enhance T-cell activity by increasing effector T-cell proliferation, by augmenting the production of T helper type 1 cytokines, and by inhibiting regulatory T-cell proliferation and suppression (Quach 2010).

In vitro testing of talquetamab with pomalidomide in bone marrow samples from multiple myeloma patients showed that pomalidomide led to an increase of talquetamab-mediated killing in these assays. This effect was associated with increased T cell numbers at suboptimal

talquetamab concentrations, superior T-cell activation, and increased granzyme B secretion. In addition, there was a trend towards a lower frequency of T regulatory cells with the talquetamab pomalidomide treatment, further supporting the rationale for the combination (Verkleij 2021).

The addition of pomalidomide to talquetamab SC provides a new mechanism of action to an established regimen, which could potentially enhance antimyeloma effects by combining the individual cytotoxic and immunomodulatory aspects of each drug. This may lead to enhanced efficacy in patients with relapsed/refractory multiple myeloma. Based on the results of Phase 1 dose escalation Study 64407564MMY1004 (MonumenTAL-2), the combination of talquetamab with pomalidomide has demonstrated high rates of robust responses in heavily pretreated participants (see Section 2.2.4).

2.1.6. Rationale for Combining Talquetamab and Teclistamab

Talquetamab and teclistamab are humanized IgG4 bispecific antibodies targeting the CD3 receptor complex on T cells and GPRC5D or BCMA, respectively, on myeloma cells. The same CD3-targeting region is present on both molecules. By bringing T cells into proximity with myeloma cells, these agents promote the activation of T cells and induce myeloma cell lysis mediated by secreted perforin and various granzymes stored in the secretory vesicles of cytotoxic T cells.

Although combination regimens of PIs, IMiDs, corticosteroids, and chemotherapy have been shown to be effective for patients with multiple myeloma, the disease inevitably relapses. Studies 64407564MMY1001 for talquetamab monotherapy and 64007957MMY1001 (MajesTEC-1) for teclistamab monotherapy have shown extraordinary efficacy in patients with heavily pretreated multiple myeloma. However, a substantial portion of those patients relapse, possibly as a result of heterogeneity of target expression. A combination approach targeting 2 highly-expressed surface proteins (GPRC5D and BCMA) in multiple myeloma simultaneously may improve efficacy, by maximizing T-cell redirection and tumor cell killing. Additionally, it is hypothesized that targeting 2 distinct antigens found on multiple myeloma cells may overcome some of the common mechanisms of resistance to monotherapy treatment by reducing the risk of loss of activity due to single target antigen loss and enhancing antigen antibody interaction. Therefore, dual targeting may also mitigate target molecule-related escape, potentially reducing the risk of relapse (Fernandez de Larrea 2020). Based on the results of Phase 1 Study 64007957MMY1003 (RedirecTT-1), the combination of talquetamab with teclistamab has demonstrated high rates of deep and robust responses in heavily pretreated participants (see Section 2.2.5).

The proposed dosing regimen for talquetamab and teclistamab aims to augment efficacy by simultaneous targeting of 2 distinct surface proteins on myeloma cells. The proposed dosing schedule optimizes efficacy by balancing maximal reduction of disease burden with initial Q2W dosing of talquetamab and Q4W dosing from Cycle 5 for participants with confirmed VGPR or better, and Q4W dosing of teclistamab.

2.1.7. Elotuzumab, Bortezomib, and Pomalidomide

The population to be enrolled in this study has limited treatment options. Current triplet therapy options include EPd or PVd. The rationale for the choice of comparator is the following:

Elotuzumab is a humanized IgG1 mAb that binds SLAMF7 and directly activates natural killer cells via the Fc receptor, thereby enabling selective killing of myeloma cells with minimal of effects on normal tissues.

Bortezomib is a reversible inhibitor of the chymotrypsin-like activity of the 26S proteasome in mammalian cells. Inhibition of the 26S proteasome prevents targeted proteolysis, which can affect multiple intracellular signaling cascades. This disruption of normal homeostatic mechanisms can lead to cell death. Experiments have demonstrated that bortezomib is cytotoxic to a variety of cancer cell types in vitro. Bortezomib causes a delay in tumor growth in vivo in nonclinical tumor models, including multiple myeloma.

Pomalidomide is an analog of thalidomide with immunomodulatory, antiangiogenic, and antineoplastic properties. Cellular activities of pomalidomide are mediated through its target cereblon, a component of a cullin ring E3 ubiquitin ligase enzyme complex.

PVd represents an efficacious, approved treatment for second-line therapy since it can be used for both lenalidomide-sensitive and lenalidomide-refractory patients. As suggested by treatment guidelines (Dimopoulos 2021, Kumar 2022) it is preferred to use a regimen containing at least one drug from a class to which the patient is not refractory; hence, PVd provides an important option for that patient population. PVd is approved in the EU for the treatment of patients with multiple myeloma who have received at least one prior treatment regimen including lenalidomide and is recommended in the most recent ESMO guidelines as a second-line treatment option for patients who have previously been exposed to a daratumumab- and lenalidomide-containing regimen (Dimopoulos 2021). The approval of PVd was based on results from the Phase 3 OPTIMISMM study in lenalidomide-pretreated patients with early-line relapsed or refractory multiple myeloma. In the OPTIMISMM study, median PFS was longer with PVd versus bortezomib and dexamethasone treatment (11.2 versus 7.1 months) in participants who were 100% lenalidomide exposed and had a median of 2 prior lines of therapy (Richardson 2019).

Among treatment options for patients at second or subsequent relapse, EPd is approved in the US and EU for the treatment of patients who have received ≥2 prior lines of therapy, including lenalidomide and a PI and is recommended by ESMO for the treatment of patients who are lenalidomide refractory and bortezomib refractory (Dimopoulos 2021). The approval of EPd was based on the results from the ELOQUENT-3 study, in which median PFS was longer with EPd than Pd (10.3 versus 4.7 months), indicating a 46% reduction in the risk of disease progression (HR 0.54; 95% CI: 0.34 to 0.86; p=0.0078) between EPd and Pd after a minimum follow-up of 9.1 months (Dimopoulos 2018). In all, 68% of patients in EPd group and 72% of patients in Pd group had multiple myeloma that was refractory to both lenalidomide and a PI. In the most recent update of the ELOQUENT-3 study, EPd demonstrated a statistically significant improvement in OS compared with Pd in patients with relapsed or refractory multiple myeloma who have been

previously treated with lenalidomide and a PI and were refractory to last therapy. Results showed that the median (95% CI) OS was significantly improved with EPd (29.8 [22.9 to 45.7] months) versus Pd (17.4 [13.8 to 27.7] months), with a HR of 0.59 (95% CI: 0.37 to 0.93; p=0.0217). OS benefit with EPd was also observed in most of the patient subgroups (Dimopoulos 2023). Recently a Phase 2 study of EPd in patients who were refractory to daratumumab showed utility of EPd as an effective regimen in this patient population and significantly superior PFS if they received EPd immediately following progression on CD38 antibody (Ailawadhi 2022). It is recognized that EPd after treatment with daratumumab may be a viable option in relapsed or refractory multiple myeloma due to the relatively less frequent use of pomalidomide in earlier lines of therapy. Hence, as shown in ELOQUENT-3 and as recommended in ESMO guidelines, EPd provides an effective option for patients who are refractory to both lenalidomide and PI. Therefore, this study includes EPd as therapeutic option for patients randomized to standard of care.

2.2. Background

2.2.1. Talquetamab and Teclistamab Nonclinical Studies

Details of the in vitro and in vivo pharmacology, safety pharmacology, toxicology, and local tolerance of talquetamab and teclistamab are presented in the most recent version of the respective IB.

2.2.2. Talquetamab Clinical Studies

Talquetamab has demonstrated significant activity as a monotherapy in participants with relapsed or refractory multiple myeloma (Minnema 2022). In Study 64407564MMY1001, talquetamab monotherapy (0.4 mg/kg SC weekly and 0.8 mg/kg SC Q2W) is effective in terms of overall response, time to first response, and DOR, and is well tolerated with manageable safety profiles, in participants with heavily pretreated relapsed or refractory multiple myeloma (see IB).

Talquetamab is also currently being investigated in ongoing studies as part of combination treatments with daratumumab, pomalidomide, lenalidomide, teclistamab, and cetrelimab (Study 64407564MMY1002, Study 64007957MMY1003, Study 64407564MMY1004, Study 64407564MMY1005, and Study 64407564MMY3002).

Safety data suggest talquetamab has a manageable safety profile. Risks include CRS, ICANS, skin and nail changes, oral toxicity, cytopenias, sARR, infection, injection-site reaction, irAE, TLS, and hypogammaglobulinemia. The majority of CRS events occurred either at the step-up doses or with the first full dose; they were low-grade, manageable, and reversible. Drug-related neurotoxicity events were rare and not dose dependent.

For comprehensive efficacy and safety details on ongoing clinical studies refer to the talquetamab IB.

2.2.3. Teclistamab Clinical Studies

The safety and preliminary antimyeloma activity of teclistamab monotherapy is being evaluated in an ongoing dose escalation/dose expansion Phase 1/2 study in participants with relapsed or

refractory multiple myeloma (Study 64007957MMY1001 [MajesTEC-1]). Based on the results of MajesTEC-1, teclistamab monotherapy has demonstrated a high rate of deep and durable response in heavily pretreated patients (Moreau 2022). Safety data suggest teclistamab has a manageable safety profile. CRS and ICANS are considered TEAEs of special interest. In MajesTEC-1, AEs were common and included low-grade CRS and Grade 3 or 4 cytopenia and infection. Dose reductions and discontinuations due to AEs were infrequent (Moreau 2022).

Based on safety, efficacy, PK, and pharmacodynamic data from MajesTEC-1, a weight-based weekly treatment dose of 1.5 mg/kg teclistamab (administered SC) was selected for teclistamab monotherapy; step-up doses of teclistamab are administered prior to the first treatment dose. This weight-based dosing regimen has been evaluated in the Phase 2 component of MajesTEC-1 in heavily pretreated (≥3 lines of prior therapy) participants (Moreau 2022).

For comprehensive efficacy and safety details on ongoing clinical studies refer to the teclistamab IB.

2.2.4. Talquetamab/Pomalidomide Combination Studies

Clinical evidence to support the Tal-P combination for the treatment of patients with relapsed and refractory multiple myeloma comes from the Phase 1 dose escalation study 64407564MMY1004. In this study, talquetamab was given at the 0.4 mg/kg weekly and 0.8 mg/kg Q2W RP2D schedule and pomalidomide was started on C2D1 at 2 mg with the option to escalate to 4 mg in subsequent cycles.

As of 16 February 2023, 55 participants have been treated with Tal-P in Study 64407564MMY1004. Of these, 15 and 18 participants were treated at the 0.4 mg/kg weekly and 0.8 mg/kg Q2W RP2Ds, respectively. With a median duration of follow-up of 8.3 and 4.2 months, there are 13 and 12 disease evaluable participants in the respective 0.4 mg/kg weekly and 0.8 mg/kg Q2W cohorts. The ORR for the participants in the 0.4 mg/kg weekly cohort was 92.3%, with 92.3% ≥VGPR. The ORR for participants in the 0.8 mg/kg Q2W cohort was 91.7%, with 83.3% ≥VGPR.

TEAEs observed with Tal-P in the 0.4 mg/kg QW and 0.8 mg/kg Q2W cohorts were manageable and consistent with its mechanism of action.

Following treatment with Tal-P, participants exhibited pharmacodynamic changes that were characteristic of the mechanism of action for talquetamab and consistent with talquetamab monotherapy, including increased expression of T-cell activation markers such as CD25 on CD8-expressing T cells. Induction of T-cell activation was consistent between the 2 RP2Ds in combination with pomalidomide.

2.2.5. Talquetamab/Teclistamab Combination Studies

In the Phase 1 study (64007957MMY1003 [RedirecTT-1]), different dose combinations of talquetamab and teclistamab were evaluated. Preliminary data from the 0.8 mg/kg Q2W talquetamab and 3 mg/kg teclistamab Q2W dose regimen has demonstrated robust clinical efficacy

with manageable safety profile, as summarized below. For additional information refer to the latest version of the talquetamab IB.

As of 22 February 2024, 114 participants have been treated with the Tal-Tec combination across 6 dose escalation cohorts (median duration of follow-up ranging from 35.88 months in the DL1 cohort to 4.3 months in the DL6 cohort). From DL1 to DL6, weekly to biweekly and monthly dose levels were explored (Table 13).

Table 13: Talquetamab-Teclistamab Dose Combinations in Study 64007957MMY1003 (RedirecTT-1)

Dosing Cohorts	Treated participants (N)	Talquetamab (monthly equivalent)	Teclistamab (monthly equivalent)
Dose Level 1 (DL1)	6	0.2 mg/kg QW (0.8 mg/kg per month)	0.75 mg/kg QW (3 mg/kg per month)
Dose Level 2 (DL2)	5	0.2 mg/kg QW (0.8 mg/kg per month)	1.5 mg/kg QW (6 mg/kg per month)
Dose Level 3 (DL3)	28	0.4 mg/kg QW (1.6 mg/kg per month)	1.5 mg/kg QW (6 mg/kg per month)
Dose Level 4 (DL4)	11	0.8 mg/kg Q2W (1.6 mg/kg per month)	1.5 mg/kg Q2W (3 mg/kg per month)
Dose Level 5 (DL5)	44	0.8 mg/kg Q2W (1.6 mg/kg per month)	3.0 mg/kg Q2W (6 mg/kg per month)
Dose Level 6 (DL6)	20	0.8 mg/kg Q4W (0.8 mg/kg per month)	3.0 mg/kg Q4W (3 mg/kg per month)

Step-up doses (mg/kg): DL1: 0.01, 0.06 (Tal)/0.06, 0.3 (Tec), DL2: 0.01, 0.06 (Tal)/0.06, 0.3 (Tec), DL3: 0.01, 0.06 (Tal)/0.06, 0.3 (Tec), DL4: 0.01, 0.06, 0.3 (Tal)/0.06, 0.3 (Tec), DL5: 0.01, 0.06, 0.3 (Tal)/0.06, 0.3 (Tec), and DL6: 0.01, 0.06 (Tal)/0.06, 0.3 (Tec).
Clinical cutoff date: 22 February 2024.

For the DL5 cohort (one of the cohorts with the highest total monthly exposure at Tal 0.8 mg/kg Q2W [1.6 mg/kg per month] and Tec 3.0 mg/kg Q2W [6 mg/kg per month] and a median duration of follow-up of 17.51 months), ORR was 81.8% (95% CI: 67.3%; 91.8%), with 79.5% ≥VGPR (95% CI: 64.7%; 90.2%). Approximately 40% of participants in DL5 were reported as having EMD.

Cohort DL6 was added to provide data at a lower exposure (Tal 0.8 mg/kg Q4W [0.8 mg/kg per month]; Tec 3.0 mg/kg Q4W [3 mg/kg per month]). For the DL6 cohort (Tal 0.8 mg/kg Q4W [0.8 mg/kg per month] and Tec 3.0 mg/kg Q4W [3 mg/kg per month] and a median duration of follow-up of 4.3 months), ORR was 83.3% (95% CI: 58.6%; 96.4%), with 66.7% ≥VGPR (95% CI: 41.0%; 86.7%).

Across the 6 cohorts, 3 participants (2.6%) had DLT TEAEs. The incidence of CRS, ICANS, skin-related TEAEs, and oral toxicity was comparable across cohorts and consistent with talquetamab's and/or teclistamab's mechanism of action and comparable to incidence observed for talquetamab and teclistamab monotherapies. The majority of CRS events were low grade (Grade 1 or 2), manageable, and reversible, with 2 Grade 3 events and 14.9% had a treatment cycle delay or dose modification. Seven (7) ICANS events were reported in 4 participants, with maximum severity of Grade 1 (2 participants) and 1 participant for each of Grades 2 and 3.

Neutropenia was manageable. Across the 6 cohorts, cycle delay or dose modification due to neutropenia ranged from 60% of participants DL2 to 5.0% of participants in DL6. Incidence of Grade 3 or higher infections ranged from 40.0% in the DL6 cohort to 83.3% in the DL1 cohort (comparably shorter follow-up for DL6 is noted).

Following treatment with Tal-Tec, participants exhibited pharmacodynamic changes that were characteristic of the mechanism of action for talquetamab and teclistamab and consistent with each monotherapy, including increased expression of T-cell activation markers such as CD38 on CD3-expressing T cells.

Data from 114 participants treated with the Tal-Tec regimen demonstrate a manageable safety profile and favorable efficacy was observed.

2.2.6. Comparator Drugs

For further information regarding elotuzumab, bortezomib, pomalidomide, and dexamethasone, refer to the appropriate prescribing information.

2.3. Benefit-risk Assessment

2.3.1. Risks for Study Participation

An overview of the risks associated with the study drugs is presented in the sections below. More detailed information about the known and expected benefits and risks may be found in the IB for talquetamab and teclistamab and local prescribing information for elotuzumab, pomalidomide, bortezomib, and dexamethasone. Management guidelines for potential toxicities are presented in Section 6.1.4.

Additional details regarding specific risks for participating in this study may be found in the associated ICF documents.

2.3.1.1. Risks Associated with Talquetamab and Teclistamab

Given that talquetamab and teclistamab act by stimulating the endogenous immune system, there is potential for toxicity on other tissues or organs by activating immune cells through an inflammatory mechanism. The risks for talquetamab and teclistamab detailed in Table 14 are based on: 1) results of nonclinical studies; 2) presumed mechanism of action; 3) route of administration; and 4) results from participants treated in the ongoing monotherapy (64407564MMY1001 and 64007957MMY1001 [MajesTEC-1], respectively) and combination studies, including 64407564MMY1002 and 64007957MMY1003.

More detailed information about the known and expected benefits and risks of talquetamab and teclistamab may also be found in the respective IBs.

Table 14: Mitigation Strategies for Risks Associated with Talquetamab and Teclistamab (Arms A and B)

Risk	Mitigation Strategies
CRS	Step-up dosing is hypothesized to reduce frequency and severity of CRS. Administer pretreatment medications per Table 4 and Table 5; see also Section 6.1.2.1. Participants must be willing to be hospitalized or remain in close proximity (within 30 minutes) to the hospital starting after step-up dose 1 until 48 hours after administration of the first treatment dose of talquetamab or teclistamab (Section 6.1.3.1), and carry a "wallet study card" with pertinent information (including signs and symptoms of CRS/ICANS) for the duration of study participation. Follow guidance for management of CRS (Section 6.1.4.1.1) and follow instructions for dosing delays/skips and dose reduction (Sections 6.6.1 and 6.6.2) or treatment discontinuation (Section 7.1) as applicable. CRS is most commonly reported after administration of step-up doses and the first treatment dose of talquetamab and teclistamab.
ICANS	Early recognition of ICANS is critical to management. Participants should be advised to seek medical evaluation if they notice impairment in motor function (eg, weakness), changes in sensation (eg, numbness), or symptoms suggestive of possible CNS abnormalities such as new onset of headache or mental status changes. Suspected ICANS will be evaluated using the ICE Tool (see Appendix 14). Recommended management of ICANS is presented in Section 6.1.4.2.1.
Skin and nail changes (risk is specific to talquetamab)	Skin and nail changes may occur. Skin changes may include skin peeling, itching, and rash, and nail changes may include nail loss, peeling, ridging, discoloration, breaking and separation. Early use of emollients is encouraged to mitigate skin changes. Rashes may be local, at the site of an SC injection, or may be systemic. Monitoring of rash progression is critical and early intervention may be necessary. Rashes occurring during the first cycle should be treated with topical steroids and early consideration of a short course of PO steroids to reduce the risk of rash progression (see Section 6.1.4.9). Symptomatic treatment is recommended for nail dysfunction (see Section 6.1.4.9). A dermatologist may be consulted.
Oral toxicity (risk is specific to talquetamab)	Oral side effects may occur, and may include dry mouth, altered taste, loss of taste, and difficulty swallowing. Administer supportive care, based on best clinical judgment and institutional standards, such as saliva stimulating agents, steroid mouth wash, or consultation with a nutritionist (see Section 6.1.4.10).
Weight loss (risk is specific to talquetamab)	Over time, notable weight loss may occur. Weight change should be monitored regularly during therapy. Clinically significant weight loss should be further evaluated.
sARR	Administer pretreatment medications per Table 4 and Table 5 (see also Section 6.1.2). Follow guidance for management of sARRs (Section 6.1.4.5) and instructions for dosing interruption (Section 6.6.2.1 for hematologic toxicities and Section 6.6.2.2 for non-hematologic toxicities) and treatment discontinuation (Section 7.1), as applicable.
Cytopenias	Frequently monitor hematologic parameters and provide supportive care (eg, transfusions) per institutional guidelines. Prolonged neutropenia may increase the risk of infection. Severe thrombocytopenia may increase the risk of bleeding. See Section 6.9.1 regarding use of permitted therapies for treatment of cytopenias.
Infections	See Section 6.1.4.4 for detailed guidance. Frequently monitor for the presence of infections, with the acquisition of cultures and/or implementation of empiric antibiotic therapy as appropriate, based on clinical judgment and institutional standards. Perform screening for HBV and HCV (see Section 8.3.5), monitor as clinically indicated (Sections 6.1.4.4.4 and 6.1.4.4.5). Initiate prophylaxis or treatment therapies for infections as appropriate (Section 6.1.4.4.1). For participants with history of HBV infection, monitor for clinical and laboratory signs of HBV reactivation up to 6 months following the last treatment dose

Table 14: Mitigation Strategies for Risks Associated with Talquetamab and Teclistamab (Arms A and B)

Risk	Mitigation Strategies
	(Section 8.3.5.3). Withhold treatment if reactivation diagnosed and initiate treatment as appropriate (Section 6.1.4.4.4). PML, which can be fatal, has also been reported in patients receiving teclistamab. Monitor any new onset of or changes in pre-existing neurological signs or symptoms. If PML is suspected, withhold treatment with teclistamab and initiate appropriate diagnostic testing. Discontinue teclistamab if PML is confirmed.
Injection-site reaction	Monitor for local reactions surrounding the injection site and manage per institutional standards (see Section 6.1.4.6).
irAEs	Monitor closely for possible development of irAE(s) and treat per institutional standards (Section 6.1.4.8). May require dose delays/skips and dose reduction (Sections 6.6.1 and 6.6.2), or lead to treatment discontinuation (see Section 7.1).
TLS	Frequently monitor chemistry parameters. Early recognition of signs and symptoms. Initiation of preventive measures in high-risk participants prior to treatment, as well as the prompt initiation of supportive care for participants who develop acute TLS during treatment per institutional standards (see Section 6.1.4.7).
Hypogammaglobulinemia	Monitor Ig levels on or after treatment and treat according to local guidelines, including administration of Ig replacement (Section 6.1.4.3). Monitor for infection. Additionally, immune response to vaccines may be reduced. Vaccination with live virus vaccines is not permitted for at least 4 weeks prior to start of treatment, during treatment, and for the 30 days following the last dose of study treatment (annual influenza or SARS-CoV-2 vaccines approved or authorized for emergency use are allowed).

2.3.1.2. Risks Associated with Elotuzumab and EPd

Key AEs reported with elotuzumab have included infusion-related events. Infusion reactions have been reported in between 3% and 10% in clinical studies of participants treated with elotuzumab (in combination with lenalidomide/dexamethasone or pomalidomide/dexamethasone) (EMPLICITI USPI 2022). All reports of infusion reaction were Grade ≤3. The most common symptoms of an infusion reaction included fever, chills, and hypertension. Infusion reactions have all been managed by medications and resolved in less than 24 hours. The frequency and intensity of infusion-related AEs has been mitigated with premedications, including corticosteroids, histamine-1 and -2 antagonists, and acetaminophen. For the EPd arm in this protocol (Arm C), dexamethasone is an integral part of the premedication regimen, in addition to serving as a therapeutic agent against myeloma cells. Therefore, for participants in this arm, dexamethasone dosing is optimized to provide adequate infusion reaction protection via doses of PO and IV dexamethasone (Table 8). Guidelines for the management of infusion reactions are provided in Section 6.1.4.6.3.

The most common adverse reactions (20% or higher) for EPd have been constipation and hyperglycemia (EMPLICITI USPI 2022). The most frequent serious adverse reactions included pneumonia (13% of participants) and respiratory tract infection (7% of participants).

Cardiac events, including congestive cardiac failure, pulmonary oedema and atrial fibrillation have been reported with pomalidomide. Periodic monitoring should be undertaken as clinically indicated. Thrombosis and embolism are rare but serious side effects of IMiDs. Prevention of deep vein thrombosis and pulmonary embolism is discussed in Section 6.9.2.4.

Pomalidomide is an analog of thalidomide, which is a known human teratogen that causes severe birth defects or embryo-fetal death. Participants treated with pomalidomide must be willing to adhere to the pregnancy prevention guidelines as described in Section 5.1, Section 5.3, and Appendix 4. Participants of childbearing potential treated with pomalidomide must undergo pregnancy testing as described in the relevant SoA in Section 1.3 and adhere to the pomalidomide global PPP or local PPP/REMS program.

For further information on potential risks refer to the prescribing information.

2.3.1.3. Risks Associated with PVd (Pomalidomide and Bortezomib)

The most commonly reported hematologic adverse reactions associated with PVd in clinical studies were neutropenia (46.8%) and thrombocytopenia (36.7%). The most commonly reported serious adverse reaction was pneumonia (11.5%), and the most commonly reported adverse reaction was peripheral sensory neuropathy (47.8%). AEs were manageable with dose modifications (72% of the PVd arm in OPTIMISMM had a dose reduction of any study drug) and supportive care. Management of neurologic toxicity associated with bortezomib administration is discussed in Section 6.6.2.3.2. Prevention of deep vein thrombosis and pulmonary embolism is discussed in Section 6.9.2.4.

Refer to Section 2.3.1.2 for details of risks associated with pomalidomide.

For further information on potential risks refer to the prescribing information.

2.3.1.4. Risk of Overlapping Toxicities

2.3.1.4.1. Talquetamab and Teclistamab

Given the binding to CD3 regions on T cells and to antigens found primarily on malignant myeloma cells by both study drugs, there is potential for overlapping toxicity profiles. These include CRS, ICANS, infection, and hypogammaglobinemia, due to activation of a broad array of immune cells that secrete inflammatory cytokines while targeting B cells. The current study has been designed to safely combine talquetamab and teclistamab by incorporating various measures. Treatment is initiated at reduced doses of each study drug (ie, step-up dosing) prior to administration of treatment doses. To mitigate risk of infection and hypogammaglobulinemia, treatment with antimicrobials is recommended (Section 6.9.2.1) and Ig replacement should be considered (Section 6.1.4.3). All safety data will be reviewed periodically to reduce the risk of toxicity to participants enrolled on the study.

2.3.1.4.2. Talquetamab and Pomalidomide

Hypothetically, the immunomodulatory effects of IMiDs on the immune microenvironment may affect the occurrence of CRS with talquetamab. To account for potential worsening of CRS toxicities, the initiation of pomalidomide administration in Cycle 1 will be delayed so as to occur after the window when CRS is most likely to occur (ie, after the step-up doses and first treatment dose of talquetamab SC). Specifically, pomalidomide will be initiated on C2D1 and will be initiated at a reduced dose of 2 mg daily, to be increased to 4 mg daily as tolerated (per investigator

discretion) from C3D1. Importantly, addition of pomalidomide to the combination of talquetamab SC and daratumumab SC in Study 64407564MMY1002 does not appear to increase risk of CRS and may induce promising responses (see Section 2.2.2 and talquetamab IB).

Cytopenias have been commonly reported in participants receiving talquetamab SC and pomalidomide (see talquetamab IB). Participants will be monitored closely with frequent laboratory testing, and appropriate supportive care will be implemented based on institutional standards to mitigate any potential overlapping risk of cytopenias and infection (Section 6.9.1 and Section 6.9.2.1) with talquetamab SC and pomalidomide, especially for respiratory infections in the first cycle after pomalidomide commencement for IMiDs. Additionally, guidance is provided in Section 6.6.2.3.1 for dose modification of pomalidomide due to hematologic toxicity.

Other risk mitigation measures planned for Arm A (Tal-P) include the initiation of pomalidomide in Cycle 2, outside the window of the highest risk for CRS. The risk of infection is mitigated with antimicrobial prophylaxis (Section 6.9.2.1) and the risk of deep vein thrombosis/pulmonary embolism with thromboprophylaxis (Section 6.9.2.4). The risk of cytopenia is mitigated by use of dose adjustment and growth factor support (Section 6.6.2.3.1).

2.3.2. Benefits for Study Participation

Taking into account the measures taken to minimize risk to participants of this study (Section 2.3.1), the potential risks identified for the 2 combination therapies (talquetamab in combination with teclistamab; talquetamab in combination with pomalidomide) are justified by the benefits that may be afforded to participants with relapsed/refractory multiple myeloma. The study will enroll participants with relapsed or refractory multiple myeloma who have received 1 to 4 prior lines of therapy including an anti-CD38 mAb and lenalidomide.

Based on the results of Studies 64007957MMY1003 (RedirecTT-1), and 64407564MMY1004 (MonumenTAL-2), the combinations of talquetamab with teclistamab and talquetamab with pomalidomide have demonstrated high rates of deep and robust response in heavily pretreated participants. As these drugs rely on the immune system of the patient, it is hypothesized that receiving these combinations at earlier lines of therapy could lead to improved antimyeloma effects.

Participants randomized to Arm C (investigator's choice of EPd or PVd) will receive an accepted therapy in this setting with careful monitoring. Favorable benefit and risk profiles for EPd and PVd have been demonstrated. EPd is approved in both the US and EU in this patient population, and PVd is approved in the EU; both EPd and PVd are recommended in the most recent ESMO guidelines (Dimopoulos 2021).

2.3.3. Benefit-risk Assessment for Study Participation

Taking into account the measures taken to minimize risk to participants of this study, the potential risks identified in association with study treatment are justified by the hypothesized benefits that may be afforded to participants with pretreated relapsed or refractory multiple myeloma.

3. OBJECTIVES AND ENDPOINTS

Objectives	Endpoints
Primary	
• To compare the efficacy of either Tal-P or Tal-Tec with EPd or PVd	• PFS
Secondary	
• To further compare the efficacy of either Tal-P or Tal-Tec with EPd or PVd	• Overall response • CR or better • VGPR or better • MRD-negative CR • OS • PFS2 • TTNT
• To characterize the PK of talquetamab and teclistamab	• Talquetamab and teclistamab serum concentrations
• To assess the immunogenicity of talquetamab and teclistamab	• Presence of ADAs to talquetamab or teclistamab
• To assess participants' symptoms, functioning, and HRQoL with Tal-P, Tal-Tec, and EPd or PVd	• Time to worsening in symptoms, functioning, and overall HRQoL • Change from baseline in symptoms, functioning, and HRQoL • Percent of participants with meaningful improvement in HRQoL
Exploratory	
• To evaluate and explore pharmacodynamic biomarkers of antimyeloma and immune activity to identify differences in pharmacologic activity between participants treated with either Tal-P or Tal-Tec and EPd or PVd	
• To explore the relationship between MRD-negativity and duration and depth of clinical response in participants treated with either Tal-P or Tal-Tec compared with EPd or PVd	
• To explore the relationship between PK, pharmacodynamic activity, AEs, and clinical response in participants treated with Tal-P and Tal-Tec	
• To explore predictive biomarkers of response and resistance to Tal-P and Tal-Tec including prognostic and disease markers at baseline,	

Objectives	Endpoints
during treatment, and in relation to efficacy parameters	
• To explore DOR in participants treated with Tal-P, Tal-Tec, and EPd or PVd, where DOR is defined as the time from first evidence of PR or better to the earliest of disease progression or death due to any cause for participants with a best response of PR or better	
• To explore taste-related symptoms associated with Tal-P, Tal-Tec, and EPd or PVd	
• To explore time to symptomatic progression in participants treated with Tal-P, Tal-Tec, and EPd or PVd	
• To compare sustained MRD-negative ≥CR (duration ≥12 months) of either Tal-P or Tal-Tec with EPd or PVd	
• To compare Tal-Tec versus Tal-P in terms of efficacy endpoints using descriptive statistics	
• To evaluate MRU	

ESTIMANDS

Refer to Section 8 for evaluations related to endpoints.

HYPOTHESES

The primary hypothesis is that Tal-P or Tal-Tec will significantly improve PFS compared with EPd or PVd in participants with relapsed or refractory multiple myeloma who have received 1 to 4 prior lines of therapy including an anti-CD38 mAb and lenalidomide.

4. STUDY DESIGN

4.1. Overall Design

This is a randomized, Phase 3, active-controlled, parallel, multicenter, interventional, open-label study in participants with relapsed or refractory multiple myeloma who have received 1 to 4 prior lines of therapy including an anti-CD38 mAb and lenalidomide. There are 2 experimental arms (Tal-P and Tal-Tec) and one control arm (investigator's choice of EPd or PVd).

A target of approximately 795 participants will be randomized in this study (with a 1:1:1 randomization ratio resulting in 265 participants planned per treatment arm).

An IDMC will be commissioned to review safety periodically and efficacy at prespecified interim analysis timepoints. Refer to Committees Structure in Appendix 2 for details.

The study will include a Screening Phase, a Treatment Phase, and a Follow-up Phase. The Screening Phase will be up to 28 days before randomization. The Treatment Phase will extend from C1D1 until confirmed disease progression, death, intolerable toxicity, withdrawal of consent, or discontinuation of all study treatment (whichever occurs first). Participants can continue therapy with talquetamab/teclistamab (as appropriate) for up to 26 cycles if they have no sign of PD or toxicity. Following the Treatment Phase, participants will continue in the Follow-up Phase until death, withdrawal of consent, loss to follow-up, or end of the study, whichever occurs first.

A diagram of the study design is provided in Section 1.2.

4.2. Scientific Rationale for Study Design

Control and Treatment Groups

Investigator's choice of the EPd or PVd regimens is to be used as a control arm (Arm C) in this study as representative of standard of care therapy. The investigator's intended choice of therapy (EPd or PVd) for each participant will be recorded prior to the participant's randomization. Details and rationale for these therapies are presented in Section 2.1.6.

Randomization will be used to minimize bias in the assignment of participants to treatment arms, to increase the likelihood that known and unknown participant attributes (eg, demographic and baseline characteristics) are evenly balanced across treatment arms, and to enhance the validity of statistical comparisons across treatment arms. Randomization will be stratified by number of prior lines of therapy (1 versus ≥2), ISS stage at screening (I versus II/III), and investigator's intended choice of therapy (EPd versus PVd).

Study Population

For patients whose myeloma relapses after exposure to daratumumab and lenalidomide in the first or subsequent lines of therapy, treatment options are limited. Therefore, there remains a significant and critical unmet need for new therapeutic options directed at alternative mechanisms of action that can better control the disease, provide deeper, more sustained responses, and better long-term outcomes including maintenance of HRQoL (Usmani 2016). This study is designed to encompass a broad population of patients with unmet medical needs based on different exposures and refractoriness to prior therapies.

Rationale for Pharmacodynamics and Exploratory Biomarker Evaluations

Biomarker studies are designed to: 1) identify differences in pharmacologic activity between study treatments, including pharmacodynamic biomarkers of antimyeloma and immune activity; 2) evaluate the effect of study treatments on MRD negativity and its relationship with clinical responses; 3) identify participant subgroups that respond differently to treatments by evaluation of prognostic and disease markers; 4) help understand relationships between treatment intervention, PK, safety, and efficacy responses; and 5) identify predictive biomarkers of response and resistance to Tal-P and Tal-Tec. For additional information about exploratory objectives relating to biomarkers, refer to Section 3.

MRU Data Collection

Treatment of multiple myeloma with Tal-P or Tal-Tec compared with EPd or PVd may result in lower resource utilization characterized by:

- Number of outpatient clinic visits.
- Duration of hospitalization (excluding protocol-mandated hospitalization) and emergency room visits.

Therefore, MRU including these data will be collected for all treatment arms.

Real-world Data Collection of Participant Medical Information During and After the Study (US Sites Only)

Real-world data play a critical role in improving understanding of progression to advanced disease stages and subsequent treatment decisions, as well as explore the potential for using this method of data collection versus traditional follow-up data collection.

To allow the linking of participant records from different sources, ie, data obtained as part of the study as specified in the SoA and longitudinal real-world data such as electronic health records, claims, and laboratory data from other care settings, without compromising the participant's confidentiality, tokenization and matching procedures will be utilized. The tokenization process starts with each data provider generating a token behind a firewall via proprietary software. Personal information such as names and dates of birth from study participants are removed from real-world data sources and replaced with encrypted, one-way, hashed identifiers, and then further encrypted using asymmetric keys in compliance with Health Insurance Portability and Accountability Act (HIPAA 1996). This encrypted anonymized information is sent for matching to the anonymized participant master index. While it is not possible to reverse the hash, source-specific tokens can be decrypted and re-encrypted so that records can be linked across sources. The result of the process is a unique anonymized identifier for each participant, which can be used to link participant records across sources.

For participants who consent to the optional collection of real-world medical data, the sponsor is committed to protecting their data and privacy. Tokenization and matching procedures will be utilized to allow for those participants' medical data to be obtained without violation of participant confidentiality. Participants will be informed that consent to this part of the study is completely optional and that they can withdraw their consent at any given time. In the event of withdrawal of consent, the sponsor will remove the token generated and any associated linked real-world data. Participation in, or withdrawal from, this optional part of the study will not affect participation in the main study.

4.2.1. Study-specific Ethical Design Considerations

Potential participants will be fully informed of the risks and requirements of the study, and during the study, participants will be given any new information that may affect their decision to continue participation. They will be told that their consent to participate in the study is voluntary and may be withdrawn at any time with no reason given and without penalty or loss of benefits to which

they would otherwise be entitled. Only participants who are fully able to understand the risks, benefits, and potential AEs of the study, and provide their consent voluntarily will be enrolled.

To ensure the well-being of participants treated in this study, the study design includes risk mitigation measures (summarized in Section 2.3) and guidance for the management of potential toxicities (Section 6.1.4). Given the data from the ongoing studies for talquetamab and teclistamab and the inclusion of well-studied, marketed control arm therapies, there is reason to believe in a positive benefit-risk profile for participation in this study. In this study, participants in both arms will be closely monitored for both safety and efficacy.

If the participant requires a legally designated representative (unless prohibited by local regulations), the legally designated representative must confirm with their signature on the ICF that the participant understands the purpose of and procedures required for the study and the participant is willing to participate. Written consent/assent may be obtained through various sources (eg, paper or electronic such as eConsent, eSignature, or digital signature) as determined by regulations as well as study and patient preferences.

The total blood volume to be collected is considered to be a customary and acceptable amount of blood to be collected over this time period in a clinical cancer study.

4.3. Justification for Dose

4.3.1. Treatment Dose and Schedule for Tal-Tec

The combination of talquetamab 0.8 mg/kg SC Q2W and teclistamab SC 3 mg/kg Q4W is the proposed dose regimen for Arm B. The doses were selected based on the respective monotherapy data (see Sections 2.2.2 and 2.2.3) and on the following considerations: 1) ease and convenience of SC administration compared with IV administration; 2) PK data indicating the selected treatment dose achieved desired target exposure; 3) pharmacodynamic data demonstrating T-cell activation and redistribution; 4) favorable clinical safety profile that appears equivalent to those observed in the lower SC dosing cohorts; 5) a favorable efficacy profile of the selected SC dose; and 6) clinical outcome of teclistamab in the RedirecTT-1 study (see Section 2.2.5).

The data in support of the combination of talquetamab and teclistamab in participants with relapsed and refractory multiple myeloma come from the Phase 1 dose escalation study 64007957MMY1003 (RedirecTT-1). In the RedirecTT-1 study, different dose combinations of talquetamab and teclistamab were evaluated. Preliminary data from RedirecTT-1 shows that 0.8 mg/kg Q2W of talquetamab and 3 mg/kg Q4W of teclistamab provides robust clinical efficacy with manageable safety profile, as summarized in Section 2.2.5. Other dosing regimens, such as weekly dosing for both bispecifics were less well tolerated. Study RedirecTT-1 exploratory E-R analysis demonstrated a trend for improved depth of response associated with Q2W talquetamab dosing in the initial treatment cycles. A reduction in talquetamab dose frequency after response does not compromise efficacy and may ameliorate GPRC5D specific AEs. Monthly administration of teclistamab at 3 mg/kg at Cycle 2 is expected to achieve comparable exposure to 1.5 mg/kg Q2W based on Study 64007957MMY1001 (MajesTEC-1) and study RedirecTT-1 data. Moreover,

there may be improved tolerability with lower frequency dosing while maintaining response (refer to Section 2.2.5 for RedirecTT-1 data).

Data from Study 64407564MMY1001 have shown a median time to first response of 1.2 months and a median time to best response of 2.2 months for participants treated with talquetamab at 0.8 mg/kg SC Q2W. The median concentration-time profile exposure following Q4W dosing is predicted to be at or above the EC_{90} value identified in an ex vivo cytotoxicity assay and is expected to be sufficient to maintain efficacy in low tumor burden setting. Thus, in the current study, dosing for talquetamab will proceed in Q4W intervals from Cycle 7 onward (as early as Cycle 5 for participants in confirmed VGPR or better) in order to decrease exposure while maintaining efficacy and improving convenience for participants.

In preclinical models, it has been shown that T-cell exhaustion induced by persistent antigen stimulation or tonic receptor signaling can lead to compromised outcomes of T cell-based immunotherapies (Philipp 2022). It has been postulated that treatment-free intervals ameliorate- T-cell exhaustion induced by continuous bispecific molecule exposure. Various bispecific therapies in large cell lymphoma have already been studying fixed duration of therapy, eg, glofitamab (Dickinson 2022) and mosunetuzumab. In a Phase 1 study of cevostamab (an FcRH5xCD3 T-cell-engaging bispecific mAb) for patients with relapsed and refractory multiple myeloma, cevostamab was limited to 17 cycles (approximately 12 months). As presented at the American Society of Hematology 2022, 13 of the 16 participants remained in remission, with 8 participants (5 sCR, 1 CR, 2 VGPR) maintaining a response ≥6 months after completion of therapy and 3 participants (2 sCR, 1 CR) maintaining a response ≥12 months after completion of therapy. Study results suggest feasibility of and potential for a finite duration of bispecific therapy (Lesokhin 2022). Also, emerging evidence from retrospective studies shows that the risk of infection is associated with the length of bispecific therapy treatment, highlighting the need to explore finite duration of therapy (Hammons 2022). Lastly, in a patient survey, a finite duration of therapy was also identified as a preferred approach in the treatment of their multiple myeloma (Mohyuddin 2022). In summary, based on early data from various bispecific therapies, a finite duration of therapy provides an opportunity to mitigate toxicity while maintaining efficacy and improving patient compliance. In relapsed refractory multiple myeloma, a pragmatic approach would include providing therapy for a duration that would provide meaningful clinical benefit over the current standard of care options. As reflected in cevostamab Phase 1 study where a 12-month duration was considered given the observed PFS in late lines of relapse, 24 months of therapy, would provide patients with a duration of therapy that provides meaningful clinical benefit in earlier lines of therapy in this study (MonumenTAL-6). This pragmatic approach of ~24 months is also reflective of experience from other clinical studies in myeloma like IFM2009, CASSIOPIEA, and TOURMALINE-MM3 study which applied 2 years of lenalidomide, daratumumab and ixazomib, respectively in the context of maintenance therapy. With the above body of evidence, in the current study, participants will continue talquetamab treatment (Arms A and B) and teclistamab treatment (Arm B) for 26 cycles (approximately 24 months). Participants can continue pomalidomide in Arm A beyond 26 cycles in line with its prescribing information.

For the most comprehensive information regarding talquetamab and teclistamab, including available PK, pharmacodynamic, efficacy, and safety data supportive of the selected dose regimens, refer to the latest version of the respective IB.

4.3.2. Treatment Dose and Schedule for Tal-P

In this study, participants randomized to Tal-P (Arm A) will initiate treatment with pomalidomide 2 mg at C2D1 and then may escalate to the label approved 4 mg dose per investigator discretion at start of C3D1 as tolerated.

In the Phase 1 dose escalation study (64407564MMY1004), the Tal-P combination was evaluated in participants with relapsed and refractory multiple myeloma. In this study, talquetamab was given at the weekly or Q2W RP2D schedule. Safety and efficacy data from 55 participants treated with Tal-P at the RP2D support the favorable risk/benefit profile for the 0.8 mg/kg Q2W talquetamab dose. Dose modifications of pomalidomide were noted to occur in 7/8 (88%) participants who initiated treatment with a daily pomalidomide dose of 4 mg, mainly due to neutropenia. In contrast, data from additional cohorts for talquetamab 0.4 mg/kg weekly, 0.6 mg/kg Q2W, or 0.8 mg/kg Q2W show a manageable safety profile with limited pomalidomide dose reductions and robust efficacy when pomalidomide is started at 2 mg.

Additionally, Lacy 2011 conducted a nonrandomized study in participants with refractory multiple myeloma in which they examined whether starting treatment with pomalidomide 4 mg daily yields better response rates than starting treatment with a pomalidomide dose of 2 mg daily. Participants received PO pomalidomide at a dose of 2 mg (N=35) or 4 mg (N=35) daily on Days 1 to 28 of each 28-day treatment cycle. Pomalidomide dose reductions occurred in 11/35 (31%) of participants on the 2 mg dose and 12/35 (35%) of participants on the 4 mg dose and were mainly due to neutropenia. ORR for participants receiving 2 mg pomalidomide was 49% and 43% for participants receiving 4 mg pomalidomide. The study demonstrated that safety and efficacy were not compromised by choosing a lower starting dose of pomalidomide.

Recently, an alternative strategy of 4 mg of pomalidomide every other day was explored in a Phase 2 study exploring reduced dosing of pomalidomide in relapsed refractory multiple myeloma (Zander 2023). The study was prematurely closed due to a business decision; however, the 34 participants treated on the alternate-day pomalidomide schedule had an almost 50% lower rate of Grade 3/4 events (infections, neutropenia, anemia, thrombocytopenia) than that reported for studies using standard dosing, despite the lack of G-CSF prophylaxis and a higher proportion of participants aged >75 years. Authors compared the response rate of this study to the original MM-003 using 4 mg of pomalidomide. They reported an ORR of 27% (compared to 32% with 4 mg pomalidomide in MM-003) and a median PFS of 4.2 months (95% CI: 1.9 to 5.5) compared to 4.0 months (95% CI: 3.6 to 4.7) in the MM-003 study (Miguel 2013).

Based on the above evidence, pomalidomide will be initiated at 2 mg in Arm A (Tal-P) of the current study, with the option to increase to 4 mg in subsequent cycles based on participant tolerance and investigator discretion. Participants will continue talquetamab for 26 cycles of

treatment in Arm A (Tal-P). Participants can continue pomalidomide (Arm A) beyond 26 cycles in line with its prescribing information.

4.3.3. Treatment Dose and Route of Administration for Other Study Drugs

For the EPd option (Arm C), the dosing regimens for elotuzumab, pomalidomide, and dexamethasone are based on the approved label for the treatment of multiple myeloma, as detailed in the EMPLICITI USPI and SmPC (EMPLICITI USPI 2022; EMPLICITI SmPC 2023). Participants receiving the EPd regimen require prior treatment with both a PI and lenalidomide as per the approved indication.

For the PVd option (Arm C), the dosing regimens for pomalidomide, bortezomib SC, and dexamethasone are based on the standard doses used in clinical practice for the treatment of multiple myeloma, as detailed in the IMNOVID SmPC (IMNOVID SmPC 2022).

4.4. End of Study Definition

EOS Definition

The EOS is considered as completion of the final OS analysis, which will occur after approximately 311 death events for each comparison (see Section 9.2). The sponsor will establish 1 or more clinical cutoff date for CSR analysis reporting, which may occur after the primary analysis and before the end of the study. The clinical cutoff date(s) will be communicated to the sites. Participants who continue to receive study treatment or who are in follow-up after the data cutoff will continue to be monitored until completion of the Posttreatment Follow-up Period or the end of the study, as applicable. The final data from the study site will be sent to the sponsor (or designee) after completion of the final participant visit at that study site, in the time frame specified in the Clinical Trial Agreement.

Participant Study Completion Definition

A participant will be considered to have completed the study if the participant has completed the last scheduled contact as per the relevant SoA (Section 1.3) or dies, whichever comes first.

Participants who prematurely discontinue study treatment for any reason must continue to be followed for protocol-specified efficacy assessments and follow-up procedures. However, if a participant develops disease progression, they should be discontinued from the efficacy follow-up portion of the study but continued to be followed for survival status and SST per Section 1.3. The only exception to this is when a participant specifically withdraws consent for any further contact with him/her or persons previously authorized by the participant to provide this information. All reasonable efforts should be made to maintain the participant on study to ensure accurate assessment of all protocol-specified endpoints. Those participants who do prematurely discontinue study treatment will not be considered to have completed the study.

5. STUDY POPULATION

Screening for eligible participants will be performed within 28 days before randomization.

The inclusion and exclusion criteria for enrolling participants in this study are described below. Investigators should ensure that all study enrollment criteria have been met at screening. If there is a question about these criteria, the investigator must consult with the appropriate sponsor representative and resolve any issues before enrolling a participant in the study. Waivers are not allowed.

A participant is considered eligible if the last observation before administration of the study treatment satisfies the inclusion and exclusion criteria. If a participant's clinical status changes (including any available laboratory results or receipt of additional medical records) after screening but before administration of the study treatment such that the participant no longer meets all eligibility criteria, then the participant must be excluded from participation in the study. Refer to Section 5.4, Screen Failures for conditions under which the repeat of any screening procedures is allowed.

For a discussion of the statistical considerations of participant selection, refer to Section 9.2, Sample Size Determination.

5.1. Inclusion Criteria

Each potential participant must satisfy all of the following criteria to be enrolled in the study:

Age

1. Be ≥18 years of age (or the legal age of majority in the jurisdiction in which the study is taking place, whichever is greater) at the time of informed consent.

Type of Participant and Disease Characteristic(s)

2. Documented multiple myeloma as defined by the criteria below:
   a. Multiple myeloma diagnosis according to the IMWG diagnostic criteria (Appendix 9).
   b. Measurable disease at screening as assessed by central laboratory, defined by any of the following:
      i. Serum M-protein level ≥0.5 g/dL (central laboratory); or
      ii. Urine M-protein level ≥200 mg/24 hours (central laboratory); or
      iii. Light chain multiple myeloma without measurable M-protein in the serum or the urine: serum Ig FLC ≥10 mg/dL (central laboratory) and abnormal serum Ig kappa lambda FLC ratio (central laboratory) (see Appendix 9).

   NOTE: All attempts should be made to determine eligibility of the participant based on the central laboratory results of screening blood and urine M-protein measurements. In exceptional circumstances and after discussion with and written approval by the sponsor, the local laboratory results of blood and urine M-protein measurements may be used to determine initial eligibility, but only if the results are clearly (ie, ≥25% or more) above the

thresholds for measurability. In such cases, central laboratory results should still be obtained prior to the start of administration of study treatment in order to establish baseline values and confirm the results from the local laboratory.

3. Relapsed or refractory disease as defined below:
   a. Relapsed disease is defined as an initial response to prior treatment, followed by confirmed PD by IMWG criteria >60 days after cessation of treatment.
   b. Refractory disease is defined as <25% reduction in M-protein or confirmed PD by IMWG criteria during previous treatment or ≤60 days after cessation of treatment.
4. Documented evidence of PD or failure to achieve a minimal response to the last line of therapy based on investigator's determination of response by IMWG criteria on or after their last regimen.
5. Have an ECOG performance status score of 0, 1, or 2 at screening and immediately prior to the start of administration of study treatment.

Sex and Contraceptive/Barrier Requirements

6. A POCBP must have a negative highly sensitive serum pregnancy test within 10 to 14 days prior to C1D1 and a further negative serum or urine pregnancy test within 24 hours prior to the start of study treatment, and must agree to further urine or serum pregnancy tests during the study and within 6 months after receiving the last dose of study treatment.
7. A participant must be (as defined in Appendix 4):
   a. Not of childbearing potential, or
   b. Of childbearing potential and
      i. Practicing true abstinence; or
      ii. Practicing at least 2 reliable methods of contraception simultaneously, including one highly effective method of contraception and one other effective method of contraception (details in Appendix 4). Contraception must begin 4 weeks prior to dosing, continue during study treatment, including during dose interruptions, and through 6 months after the last dose of study treatment.

   Reliable contraception is indicated even where there has been a history of infertility, unless due to hysterectomy or bilateral oophorectomy (Appendix 4).

   NOTE: If a participant becomes of childbearing potential after start of the study they must comply with point (b) as described above. If a participant's reproductive status is questionable, additional evaluation should be considered.

   NOTE: An interaction between hormonal contraception and talquetamab or teclistamab has not been formally studied. Therefore, it is unknown whether talquetamab or teclistamab may reduce the efficacy of the contraception method. If a participant is receiving talquetamab or teclistamab and is using hormonal contraceptives, an additional barrier method must be used.

   NOTE: Sexual abstinence is considered a highly effective method only if defined as refraining from heterosexual intercourse during the entire period of risk associated with the study treatment. The reliability of sexual abstinence needs to be evaluated in

relation to the duration of the study and the preferred and usual lifestyle of the participant.

8. A participant using PO contraceptives must use an additional contraceptive method (details in Appendix 4).
9. A participant must agree not to be pregnant, breastfeeding, or planning to become pregnant while enrolled in this study or within 6 months after the last dose of study treatment.
10. A participant must agree not to donate gametes (ova, oocytes, sperm) or freeze for future use for the purposes of assisted reproduction during the study and for 6 months after receiving the last dose of study treatment. Participants should consider preservation of gametes prior to study treatment as anticancer treatments may impair fertility.
11. A participant must wear a condom when engaging in any activity that allows for passage of ejaculate to another person during the study and for a minimum of 6 months after receiving the last dose of study treatment.
    NOTE: If the participant's partner is a person of childbearing potential, the participant must use condoms (with or without spermicide) and the partner of the participant must also be practicing a highly effective method of contraception (see Appendix 4). A participant who is vasectomized must still use a condom (with or without spermicidal), but the partner is not required to use contraception.
12. A participant must agree not to plan to father a child while enrolled in this study or within 6 months after the last dose of study treatment.

Informed Consent

13. Criterion modified per Amendment 2:
    13.1. Must sign an ICF indicating that participant understands the purpose of, and procedures required for, the study and is willing to participate in the study. In certain circumstances, when it is determined that a potential research participant is cognitively impaired, has fluctuating or limited decision-making capacity or prospective incapacity, consent may be obtained from a legally designated representative, if permitted by local regulations.
14. Must be willing and able to adhere to the lifestyle restrictions specified in this protocol (Section 5.3; including to not donate blood or blood components), including adherence to the global PPP or local PPP/REMS program for pomalidomide.

Prior Therapy Restrictions or Requirements

15. Criterion modified per Amendment 2:
    15.1. Received 1 to 4 prior lines of antimyeloma therapy (Appendix 10) including a minimum of 2 consecutive cycles of an anti-CD38 mAb at the dosing schedule (or minimum of 6 doses if anti-CD38 mAb was only part of a maintenance regimen) in any prior line and 2 consecutive cycles of lenalidomide in any prior line.
    NOTE: Participants who have received only one prior line of myeloma therapy must be considered lenalidomide refractory or intolerant to treatment (ie, have demonstrated PD by IMWG criteria on or within 60 days of completion of lenalidomide containing regimen). Participants who received ≥2 prior lines of antimyeloma therapy must be considered lenalidomide exposed.

NOTE: A single line of therapy may consist of 1 or more agents and may include induction, hematopoietic stem cell transplantation, and maintenance therapy. Radiotherapy, bisphosphonate, or a single short course of corticosteroids (no more than the equivalent of dexamethasone 40 mg/day for 4 days) would not be considered prior lines of therapy.

Clinical Laboratory Values

16. Have clinical laboratory values meeting the following criteria during the Screening Phase and within 72 hours of the first dose of study treatment. If 1 or more criteria are not met 72 hours prior to dosing, one repeat of laboratory testing is permitted.

Table 15: Adequate Organ Function

Hematology	
Hemoglobin	≥8 g/dL (≥5 mmol/L; without transfusion support or erythropoietin use within 7 days before the laboratory test)
Platelets	≥75×10^9/L in participants in whom <50% of bone marrow nucleated cells are PCs and ≥50×10^9/L in participants in whom ≥50% of bone marrow nucleated cells are PCs (without transfusion support or thrombopoietin receptor agonist within 7 days before the laboratory test)
ANC	≥1×10^9/L (prior growth factor support is permitted but must be without support for 7 days for G-CSF or GM-CSF and for 14 days for pegylated G-CSF before the laboratory test)
Chemistry	
AST and ALT	≤2.5×ULN
eGFR	≥30 mL/min based on Modified Diet in Renal Disease 4-variable formula calculation (see Appendix 11) or creatinine clearance measured by a 24-hour urine collection
Total bilirubin	≤2×ULN; except in participants with congenital bilirubinemia, such as congenital nonhemolytic hyperbilirubinemia (in which case direct bilirubin ≤1.5×ULN is required)
Serum calcium corrected for albumin	≤14 mg/dL (≤3.5 mmol/L) or free ionized calcium ≤6.5 mg/dL (≤1.6 mmol/L; see Appendix 12)

17. HIV-positive participants are eligible if they meet all of the following:
    a. No detectable viral load (ie, <50 copies/mL) at screening
    b. CD4+ count >300 cells/mm^3 at screening
    c. No AIDS-defining opportunistic infection within 6 months of screening
    d. Receiving HAART. Any changes in HAART due to resistance/progression should occur at least 3 months prior to screening. A change in HAART due to toxicity is allowed up to 4 weeks prior to screening.
    NOTE: HAART that could interfere with study treatment is excluded (consult the sponsor for a review of medications prior to enrollment).

5.2. Exclusion Criteria

Any potential participant who meets any of the following criteria will be excluded from participating in the study:

Medical Conditions

1. Criterion modified per Amendment 2:
   1.1. Contraindications or life-threatening allergies, hypersensitivity, or intolerance to any study drug or its excipients (refer to the talquetamab IB, teclistamab IB, and appropriate prescribing information). Additional exclusion criteria pertaining to specific study drugs include:
      a. GPRCD5-directed therapy or pomalidomide.
      b. A participant is not eligible to receive PVd in Arm C if any of the following are applicable:
         i. Does not meet criteria for bortezomib retreatment (failure to achieve at least PR to prior bortezomib treatment, or progression by IMWG criteria on therapy or within 6 months after cessation of prior bortezomib treatment)
         ii. Intolerance, defined as prior therapy discontinued due to any AE related to bortezomib
         iii. Grade 1 peripheral neuropathy with pain or Grade ≥2 peripheral neuropathy as defined by NCI-CTCAE Version 5.0
         iv. Received a strong CYP3A4 inducer (see Section 6.9.3.3) within 5 half-lives prior to randomization.
      c. A participant is not eligible to receive EPd (Arm C)
         i. if they have received prior elotuzumab therapy.
         ii. If they have NOT received prior proteosome inhibitor AND lenalidomide.
      d. Received prior teclistamab therapy.
      e. Participants with history of multiple myeloma that is refractory to any T-cell-redirected therapy per IMWG diagnostic criteria (Appendix 9).
2. Stroke, transient ischemic attack, or seizure within 6 months prior to signing ICF.
3. Criterion modified per Amendment 2:
   3.1. Presence of the following cardiac conditions:
      a. NYHA Class III or IV congestive heart failure (see Appendix 19)
      b. Myocardial infarction, unstable angina, or coronary artery bypass graft ≤6 months prior to randomization
      c. History of clinically significant ventricular arrhythmia or unexplained syncope, not believed to be vasovagal in nature or due to dehydration
      d. History of severe non-ischemic cardiomyopathy.
4. Major surgery or had significant traumatic injury within 2 weeks prior to the start of administration of study treatment, or will not have fully recovered from surgery, or has major surgery planned during the time the participant is expected to be treated in the study or within 2 weeks after administration of the last dose of study treatment. **NOTE**: Participants with planned surgical procedures to be conducted under local anesthesia may participate. Kyphoplasty or vertebroplasty are not considered major surgery. If there is a question whether a procedure is considered a major surgery, the

investigator must consult with the appropriate sponsor representative and resolve any issues before enrolling a participant in the study.

5. Concurrent medical or psychiatric condition or disease that is likely to interfere with study procedures or results, or that in the opinion of the investigator would constitute a hazard for participating in this study, such as:
   a. Acute diffuse infiltrative pulmonary disease.
   b. Evidence of active systemic viral, fungal, or bacterial infection, requiring systemic antimicrobial therapy.
   c. Active autoimmune disease requiring systemic immunosuppressive therapy within 6 months before start of study treatment. **EXCEPTION**: Participants with vitiligo, controlled type I diabetes, and prior autoimmune thyroiditis that is currently euthyroid based on clinical symptoms and laboratory testing are eligible regardless of when these conditions were diagnosed.
   d. Disabling psychiatric conditions (eg, alcohol or drug abuse), severe dementia, or altered mental status.
   e. Any other issue that would impair the ability of the participant to receive or tolerate the planned treatment at the investigational site, to understand informed consent or any condition for which, in the opinion of the investigator, participation would not be in the best interest of the participant (eg, compromise the well-being) or that could prevent, limit, or confound the protocol-specified assessments.
   f. History of noncompliance with recommended medical treatments.

Prior/Concomitant Therapy

6. Prior or concurrent exposure to any of the following, in the specified time frame prior to randomization:
   a. T-cell redirection therapy (eg, antibody therapy or BiTEs) within 3 months.
   b. Gene-modified adoptive cell therapy (eg, CAR-T cells, NK cells) within 3 months.
   c. Targeted therapy, epigenetic therapy, mAb therapy, cytotoxic therapy, or treatment with an investigational drug or an invasive investigational medical device within 21 days or ≥5 half-lives, whichever is less.
   d. Investigational vaccine other than SARS-CoV-2 vaccine approved or authorized for emergency use within 4 weeks.
   e. Live, attenuated vaccine within 4 weeks. Non-live and non-replicating vaccines approved or authorized for emergency use (eg, COVID-19) by local health authorities are allowed.
   f. PI therapy within 14 days.
   g. IMiD agent therapy within 14 days.
   h. Focal radiation within 7 days.
7. Received either of the following:
   a. An allogeneic stem cell transplantation within 6 months before the first dose of study treatment. Participants who received an allogeneic transplant must be off all immunosuppressive medications during the 6 weeks before the start of study treatment administration without signs of graft-versus-host disease.
   b. An autologous stem cell transplantation within 12 weeks before the start of study treatment administration.

8. A maximum cumulative dose of corticosteroids of ≥140 mg of prednisone or equivalent within 14-day period before the first dose of study drug (does not include pretreatment medications; Appendix 13).

Diagnostic Assessments

9. Any of the following:
   a. Hepatitis B infection (ie, HbsAg or HBV-DNA positive): In the event the infection status is unclear, quantitative viral levels are necessary to determine the infection status. See Section 8.3.5 for further required assessments.
   b. Active hepatitis C infection as measured by positive HCV-RNA testing. Participants with a history of HCV antibody positivity must undergo HCV-RNA testing. If a participant with history of chronic hepatitis C infection (defined as both HCV antibody and HCV-RNA positive) completed antiviral therapy and has undetectable HCV-RNA for at least 12 weeks following the completion of therapy, the participant is eligible for the study.

Other Exclusions

Not applicable.

Disease Characteristics

10. Known active CNS involvement or exhibits clinical signs of meningeal involvement of multiple myeloma. If either is suspected, negative whole brain MRI and lumbar cytology are required.
11. PC leukemia at the time of screening, Waldenström's macroglobulinemia, POEMS syndrome (polyneuropathy, organomegaly, endocrinopathy, M-protein, and skin changes), or primary amyloid light chain amyloidosis.
12. Any of the following:
    a. Ongoing myelodysplastic syndrome or B cell malignancy (other than multiple myeloma).
    b. Any history of malignancy, other than multiple myeloma, which is considered at high risk of recurrence requiring systemic therapy.
    c. Any active malignancy (ie, progressing or requiring treatment change in the last 24 months) other than multiple myeloma. The only allowed exceptions are malignancies treated within the last 24 months that are considered cured:
       i. Non-muscle invasive bladder cancer (solitary Ta-PUN-LMP or low grade, <3 cm, no CIS)
       ii. Non-melanoma skin cancers treated with curative therapy or localized melanoma treated with curative surgical resection alone
       iii. Non-invasive cervical cancer
       iv. Breast cancer: adequately treated lobular carcinoma in situ or ductal carcinoma in situ or history of localized breast cancer (antihormonal therapy is permitted)
       v. Localized prostate cancer (M0, N0) with a Gleason Score ≤7a, treated locally only (RP/RT/focal treatment)
       vi. Other malignancy that is considered cured with minimal risk of recurrence in consultation with the sponsor's medical monitor.

NOTE: In the event of any questions, consult with the sponsor's medical monitor prior to enrolling a participant.

Note: Investigators must ensure that all study enrollment criteria have been met at screening. If a participant's clinical status changes (including any available laboratory results or receipt of additional medical records) after screening but before the first dose of study treatment is given such that the participant no longer meets all eligibility criteria, then the participant must be excluded from participation in the study. Section 5.4, describes options for retesting. The required source documentation to support meeting the enrollment criteria are noted in Appendix 2.

5.3. Lifestyle Considerations

Potential participants must be willing and able to adhere to the following lifestyle restrictions during the study to be eligible for participation:

1. Refer to Section 6.9 and subsections therein for details regarding prohibited and restricted therapy during the study.
2. Agree to follow all requirements that must be met during the study as noted in the Inclusion and Exclusion Criteria (Sections 5.1 and 5.2) and the relevant SoA (Section 1.3).
3. Carry a "wallet study card" with pertinent information about the study for the duration of study participation.

Participants in Arm A (Tal-P) and Arm B (Tal-Tec)

4. Agree not to donate blood during therapy following local regulations
5. Be willing to be hospitalized or remain in close proximity (within 30 minutes) to the hospital starting after step-up dose 1 until 48 hours after administration of the first treatment dose of talquetamab or teclistamab, as described in Section 6.1.3.1.
6. Be willing to be hospitalized for administration of talquetamab or teclistamab following specified AEs, as described in Section 6.1.3.1.1.
7. Agree to self-monitor for signs and symptoms of CRS (such as fever) and ICANS and to seek immediate medical intervention if required. In particular, if receiving outpatient talquetamab/teclistamab administration, to be able to self-monitor body temperature (via preferred method of measurement) at least twice daily (at intervals ≥8 hours apart) and record in a temperature diary, from after step-up dose 1 until 48 hours after administration of the first treatment dose; and accept safety follow-up phone call(s) from the study site to report temperature or any AEs, as applicable.
8. Criterion modified per Amendment 2:
   8.1. Participants should also be willing to refrain from driving or operating heavy machinery during and for 48 hours after completion of talquetamab step-up dosing schedule and in the event of new onset of any neurologic toxicity symptoms until symptoms resolve.

Participants in Arm A (Tal-P) and Arm C (EPd and PVd options)

9. Due to the embryo-fetal risk associated with IMiDs, all participants must adhere to the global PPP or local PPP/REMS program for pomalidomide.

5.4. Screen Failures

Participant Identification, Enrollment, and Screening Logs

The investigator agrees to complete a participant identification and enrollment log to permit easy identification of each participant during and after the study. This document will be reviewed by the sponsor study site contact for completeness. This study will use IWRS. The investigator will not generate screening and enrollment logs directly from IWRS.

The participant identification and enrollment log will be treated as confidential and will be filed by the investigator in the study file. To ensure participant confidentiality, no copy will be made. All reports and communications relating to the study will identify participants by participant identification and age at initial informed consent. In cases where the participant is not randomized into the study, the date seen and age at initial informed consent will be used.

Participants who fail to meet the inclusion and exclusion criteria (ie, screen failures) may be rescreened once if their condition changes. Rescreening must be discussed with and approved by the sponsor on a case-by-case basis. Participants who are determined to be eligible for rescreening must sign a new ICF and will then be assigned a new participant number.

5.5. Criteria for Temporarily Delaying Administration of Study Treatment

Dose delay/interruption is the primary method for managing talquetamab- and teclistamab-related toxicities. Toxicities requiring dose delay as well as procedures for delay are outlined in Section 6.6.

6. STUDY TREATMENT AND CONCOMITANT THERAPY

6.1. Study Treatment Administered

<table>
<tr><th>Designation</th><th>Product</th></tr>
<tr><td>IMP(s)</td><td>Talquetamab, teclistamab, pomalidomide, elotuzumab, bortezomib, dexamethasone

Authorization status in the EU/European Economic Area
<table>
<tr><td>Authorized</td><td>talquetamab, teclistamab, pomalidomide, elotuzumab, bortezomib, dexamethasone

Used in accordance with Marketing Authorization: pomalidomide, elotuzumab, bortezomib and dexamethasone
Used not in accordance with Marketing Authorization: talquetamab and teclistamab</td></tr>
<tr><td>Unauthorized</td><td></td></tr>
</table></td></tr>
</table>

Study treatment administration must be captured in the source documents and the eCRF.

Talquetamab and teclistamab will be manufactured and provided under the responsibility of the sponsor. Refer to the IB for a list of excipients. Other study drugs will be provided under the responsibility of the sponsor. The study drug labels will contain information to meet the applicable regulatory requirement. Refer to the relevant IPPI and SIPPM for additional details.

For a definition of study treatment overdose, refer to Section 6.8, Treatment of Overdose.

Dosing schedules and pretreatment medications are presented in Table 4 (Arm A [Tal-P]), Table 5 (Arm B [Tal-Tec]), Table 8 (Arm C [EPd]), and Table 11 (Arm C [PVd]), and additional information about administration of the study drugs is provided in Table 16, Table 17, and Table 18, respectively.

Table 16: Description of Treatments (Arm A [Tal-P] and Arm B [Tal-Tec])

Arm Name	Arm A / Arm B	Arm A	Arm B
Treatment Name	Talquetamab	Pomalidomide	Teclistamab
Type	Drug	Drug	Drug
Dose Formulation	Sterile solution for SC injection	Capsule	Sterile solution for SC injection
Dosing Instructions	All step-up and treatment doses of talquetamab (in mg) are based on the participant's weight (kg). Refer to SIPPM and IPPI for weight and dose determination. Dosing schedule and pretreatment medications are presented in Table 4 and Table 5. Dose delays permitted as outlined in Section 6.6. • SC injections will be prepared as described in the IPPI and administered with the use of a syringe and needle by a manual push. • Administer required pretreatment medications (Section 6.1.2). • Doses should be administered at rotating locations on the abdomen. The anatomical area of administration must be recorded in the eCRF and the area observed for injection-site reaction(s). See the IPPIs for further details.	Pomalidomide will be self-administered PO. Pomalidomide should be taken as a single dose at approximately the same time daily. Breaking or dividing pomalidomide capsules is strongly discouraged. Dosing schedule and pretreatment medications are presented in Table 4.	All step-up and treatment doses of teclistamab (in mg) are based on the participant's weight (kg). Refer to SIPPM and IPPI for weight and dose determination. Dosing schedule and pretreatment medications are presented in Table 5. Dose delays permitted as outlined in Section 6.6. • SC injections will be prepared as described in the IPPI and administered with the use of a syringe and needle by a manual push. • Administer required pretreatment medications (Section 6.1.2). • Doses should be administered at rotating locations on the abdomen. The anatomical area of administration must be recorded in the eCRF and the area observed for injection-site reaction(s). See the IPPIs for further details.
Order of Study Treatment Administration	Arm A (Tal-P): Pomalidomide will be self-administered at approximately the same time of day. Arm B (Tal-Tec): When talquetamab SC and teclistamab SC are to be administered on the same day, the 2 drugs are to be given at least 15 minutes apart (it is suggested to administer talquetamab first).		
Safety Monitoring Requirements	• Step-up doses and first treatment dose: Section 6.1.3.1 • Subsequent administration for participants who experience specified AEs: Section 6.1.3.1.1	Not applicable	• Step-up doses and first treatment dose: Section 6.1.3.1 • Subsequent administration for participants who experience specified AEs: Section 6.1.3.1.1
Vital Signs Monitoring	On dosing days, vital signs should be collected as follows: • Step-up doses and first treatment dose: perform before start of administration	Not applicable	On dosing days, vital signs should be collected as follows: • Step-up doses and first treatment dose: perform before start of administration

Table 16: Description of Treatments (Arm A [Tal-P] and Arm B [Tal-Tec])

	and 0.5, 1, 2, and 3 hours after end of administration • For all other administrations: perform before start of administration and as clinically indicated • Monitor until normalized after CRS or sARR • Any additional vital signs assessments supporting the start and end dates of an AE should be reported in the eCRF		and 0.5, 1, 2, and 3 hours after end of administration • For all other administrations: perform before start of administration and as clinically indicated • Monitor until normalized after CRS or sARR • Any additional vital signs assessments supporting the start and end dates of an AE should be reported in the eCRF
Route of Administration	SC injection	PO	SC injection
Use	Experimental	Experimental	Experimental
IMP	Yes	Yes	Yes
NIMP/AxMP	No	No	No
Sourcing **Packaging and Labeling** **Delivery Instructions[a]**	Talquetamab will be manufactured and provided under the responsibility of the sponsor. Refer to the IB for a list of excipients.	Sourcing and other details will be provided in the applicable local label or SIPPM.	Teclistamab will be manufactured and provided under the responsibility of the sponsor. Refer to the IB for a list of excipients.
Food/Fasting Requirement[a]	None.	Pomalidomide can be taken with or without food.	None.
Current/Former Name(s) or Alias(es)[a]	JNJ-64407564, TALVEY	POMALYST, IMNOVID	JNJ-64007957, TECVAYLI

a. Labels will contain information to meet the applicable regulatory requirements.

Table 17: Description of Treatments (Arm C [EPd])

Arm Name	Arm C (EPd)	Arm C (EPd)	Arm C (EPd)
Treatment Name	Elotuzumab	Pomalidomide	Dexamethasone
Type	Drug	Drug	Drug
Dose Formulation	Lyophilized powder for reconstitution	Capsule	Tablet
Dosing Instructions	Dosing schedule and pretreatment medications are presented in Table 8. • IV elotuzumab infusions will be prepared and administered as described in the elotuzumab package insert. • Administer required pretreatment medications (Section 6.1.2).	Pomalidomide will be self-administered PO. Pomalidomide should be taken as a single dose at approximately the same time daily. Breaking or dividing pomalidomide capsules is strongly discouraged. Dosing schedule and pretreatment medications are presented in Table 8.	Dexamethasone will be self-administered by the participant. Dexamethasone may be given PO. See Table 8 for details on dexamethasone as a pretreatment medication and study drug.
Order of Study Treatment Administration	When elotuzumab, pomalidomide, and dexamethasone are administered on the same day: • Dexamethasone will be administered 3 to 24 hours prior to elotuzumab. • Pomalidomide will be self-administered by the participant at approximately the same time of day.		
Vital Signs Monitoring	On dosing days, vital signs should be collected as follows: • Elotuzumab IV: prior to the start of the elotuzumab infusion • Monitor until normalized infusion-related reaction (if any) • Any additional vital signs assessments supporting the start and end dates of an AE should be reported in the eCRF		
Route of Administration	IV injection	PO	PO
Use	Active comparator	Active comparator	Active comparator
IMP	Yes	Yes	Yes
NIMP/AxMP	No	No	No
Sourcing **Packaging and Labeling** **Delivery Instructions**[a]	Sourcing and other details will be provided in the applicable local label or SIPPM.	Sourcing and other details will be provided in the applicable local label or SIPPM.	Sourcing and other details will be provided in the applicable local label or SIPPM.
Food/Fasting Requirement[a]	None.	Pomalidomide can be taken with or without food.	None.
Current/Former Name(s) or Alias(es)[a]	EMPLICITI	POMALYST, IMNOVID	Dexamethasone

a. Labels will contain information to meet the applicable regulatory requirements.

Table 18: Description of Treatments (Arm C [PVd])

Arm Name	Arm C (PVd)	Arm C (PVd)	Arm C (PVd)
Treatment Name	Pomalidomide	Bortezomib	Dexamethasone
Type	Drug	Drug	Drug
Dose Formulation	Capsule	Solution for injection	Tablet

Dosing Instructions	Pomalidomide will be self-administered PO.
	Pomalidomide should be taken as a single dose at approximately the same time daily.
	Breaking or dividing pomalidomide capsules is strongly discouraged.
	Dosing schedule is presented in Table 11.
		• Calculate the actual dose (mg) for administration based on the participant's BSA (m^2) calculated according to a standard nomogram (Appendix 11).
		  • The total calculated dose of bortezomib may be rounded to the nearest decimal point (eg, a calculated dose of 2.47 mg can be rounded to 2.5 mg).
		  • The anatomical area of administration must be recorded in the eCRF and the area observed for injection-site reaction(s). See the IPPI, if applicable, for further details
		• Bortezomib dosing may be delayed up to 48 hours; in such cases, the timing of subsequent doses must be adjusted to account for the delay. There must be ≥3 days between doses of bortezomib
		Dosing schedule is presented in Table 11.
		• SC Bortezomib injections will be prepared and administered as described in the bortezomib package insert.
			Dexamethasone will be self-administered PO by the participant. See Table 11 for details on dexamethasone as a study drug.

Order of Study Treatment Administration	When bortezomib, pomalidomide, and dexamethasone are administered on the same day: • Dexamethasone will be administered 1 to 3 hours prior to bortezomib. • Pomalidomide will be self-administered by the participant at approximately the same time of day.		

Vital Signs Monitoring	On dosing days, vital signs should be collected as follows: • Bortezomib SC: prior to the start of the bortezomib injection • Any additional vital signs assessments supporting the start and end dates of an AE should be reported in the eCRF.		
Route of Administration	PO	SC injection (see Section 6.1.4.6.2 for participants who experience injection-site reactions related to bortezomib)	PO
Use	Active comparator	Active comparator	Active comparator
IMP	Yes	Yes	Yes
NIMP/AxMP	No	No	No
Sourcing **Packaging and Labeling** **Delivery Instructions**[a]	Sourcing and other details will be provided in the applicable local label or SIPPM.	Sourcing and other details will be provided in the applicable local label or SIPPM.	Sourcing and other details will be provided in the applicable local label or SIPPM.
Food/Fasting Requirement[a]	Pomalidomide can be taken with or without food.	None.	None.
Current/Former Name(s) or Alias(es)[a]	Pomalyst, Imnovid	Velcade	Dexamethasone

a. Labels will contain information to meet the applicable regulatory requirements.

6.1.1. Additional Criteria to be Met Prior to First Dose of Study Treatment

- Have an ECOG performance status of 0 to 2 immediately prior to the first dose of study treatment (Appendix 15).
- Have clinical laboratory values meeting the criteria in Table 15 within 72 hours of the first dose of study treatment. If 1 or more criteria are not met 72 hours prior to dosing, one repeat of laboratory testing is permitted. If >1 repeat laboratory test is necessary, the sponsor must be consulted prior to dosing.
- A POCBP must have a negative serum or urine pregnancy test within 24 hours prior to the first dose of study treatment.
- If a participant becomes ineligible between randomization and the start of study treatment for any reason, apart from the criteria above, ie, clinical status changes (including any available laboratory results or receipt of additional medical records), dosing must not occur and the sponsor must be consulted.

6.1.2. Pretreatment Medications

Pretreatment medications will be administered for participants in Arm A (Tal-P), Arm B (Tal-Tec), and Arm C (EPd) per the relevant dosing schedule in Table 4, Table 5, and Table 8. For participants receiving talquetamab, dexamethasone should not be administered as pretreatment medication for talquetamab after C1D15 except as described in Section 6.1.2.1 or upon discussion with the sponsor. Additional pretreatment medications such as H_2-antagonists or antiemetics may be used per investigator discretion. Additionally, Section 6.1.2.1 summarizes pretreatment medications that are required for participants who experience specified AEs. See Section 6.9.2 for recommended concomitant medications for all participants.

6.1.2.1. Event-driven Required Pretreatment Medications (Arm A and Arm B Only)

In the event that a participant has an event during study drug administration, the pretreatment medications below are required for participants who experience the specified AEs. These event-driven pretreatment medications are not required if already being administered as planned pretreatment as described in the dosing schedule in Table 4 and Table 5.

Arm A (Tal-P) and Arm B (Tal-Tec) Only

- A participant who experiences Grade ≥2 CRS/sARR related to talquetamab (Arm A/B) or teclistamab (Arm B only) must receive dexamethasone 20 mg, diphenhydramine 25 to 50 mg or equivalent, and acetaminophen 650 to 1000 mg as pretreatment medication for at least the subsequent dose of talquetamab SC or teclistamab SC to which the event was related. Administration of pretreatment medication beyond this should be discussed with the sponsor. If dexamethasone as pretreatment medication is continued beyond Cycle 1 for CRS/sARR, the total dose of dexamethasone in combination with pomalidomide should be limited to 40 mg weekly, and for participants <75 years of age with BMI <18.5 or those ≥75 years of age the weekly dose of dexamethasone should be limited to 20 mg.

6.1.3. Required Safety Monitoring

6.1.3.1. Safety Monitoring for Talquetamab SC and Teclistamab SC Dosing Through the First Treatment Dose

At investigator discretion and the participant's willingness, the participant may be admitted for inpatient monitoring for a minimum of 48 hours after each step-up dose and the first full treatment dose of talquetamab SC and teclistamab SC **AND/OR** may receive the step-up doses and study treatment dose of talquetamab and teclistamab as an outpatient after discussion with the sponsor; see Appendix 16 for guidance to be considered for outpatient dosing. If the participant experiences any signs of CRS, ICANS, or other clinically significant event(s), they must be hospitalized until 48 hours after administration of the first treatment dose and improvement of symptoms. Additionally, if Grade 2 or higher CRS/ICANS occurs at talquetamab or teclistamab Step-up dose 1, then Step-up dose 2 must occur inpatient. If Grade 2 or higher CRS/ICANS occurs at Step-up dose 2, then the first treatment dose must occur inpatient. If CRS/ICANS occurs at the first treatment dose or beyond, see Table 19 for guidance.

For doses administered in the outpatient setting during Cycle 1, the participant should be observed at the site for a minimum of 2 hours after administration of talquetamab and teclistamab. If no acute events are observed, the participant may leave the site. Following Cycle 1, participants should be observed in the clinic after the administration of talquetamab and teclistamab for at least 30 minutes or longer at the discretion of the investigator. The participant should remain in close proximity (within 30 minutes) to the site and in the company of a competent adult starting after Step-up Dose 1 until 48 hours after administration of the study treatment dose. During this period, participants will be asked to check their temperature at least twice daily at an interval of ≥8 hours apart and record in a participant diary. They will be instructed to report any fever (≥38°C or ≥100.4°F) to the investigator immediately and be admitted to the hospital to initiate monitoring for development of CRS (see additional details in Appendix 16).

6.1.3.1.1. Required Hospitalization for Talquetamab SC and Teclistamab SC Dosing Following Specified AEs

Any participant who experiences any of the specified AEs in Table 19 starting from the second treatment dose of talquetamab and/or teclistamab must be hospitalized for administration of the next dose of talquetamab SC and/or teclistamab SC as indicated.

Table 19: AEs Requiring Hospitalization for Subsequent Administration of Talquetamab SC and Teclistamab SC

Prior AE(s)	Safety Monitoring Requirement[a]
Grade 3 sARR	For the subsequent administration of talquetamab SC and/or teclistamab SC, ≥24 hours
Grade 3 injection-site reaction lasting longer than 72 hours	For the subsequent administration of talquetamab SC and/or teclistamab SC, ≥24 hours
Grade 3 CRS	For the subsequent administration of talquetamab SC and/or teclistamab SC, ≥48 hours
Grade 3 ICANS	For the subsequent administration of talquetamab SC and/or teclistamab SC, ≥48 hours or longer as clinically indicated
Grade ≥2 non-ICANS neurotoxicity	For the subsequent administration of talquetamab SC and/or teclistamab SC, ≥48 hours or longer as clinically indicated

a. See Table 23 regarding optional hospitalization if Grade 2 laryngeal edema or Grade 2 bronchospasm occurs.

6.1.3.1.2. Hospitalization Discharge Criteria

If a participant is hospitalized for administration of talquetamab or teclistamab, the following criteria must be met at minimum before the participant is discharged:

- Absence of fever, defined as a temperature <100.4ºF (38ºC), for ≥8 hours without use of antipyretics.
- Absence of ICANS or suspected ICANS (any grade).

6.1.4. Management Guidelines for Potential Toxicities

Management guidelines for potential toxicities of the study drugs are described in the following sections. Trained study staff at the clinic should be prepared to intervene and resources necessary for resuscitation (eg, agents such as epinephrine and aerosolized bronchodilator and medical equipment such as oxygen tanks, tracheostomy equipment, and a defibrillator) must be available nearby when participant is at high-risk for developing CRS (ie, during step-up dosing and after the first treatment dose of talquetamab and teclistamab for participants in Arm A and Arm B) or sARR (ie, during step-up dosing and after the first treatment dose of talquetamab and teclistamab for participants in Arm A and Arm B). Tocilizumab or alternative treatment must be available at the site prior to administration of talquetamab SC and/or teclistamab SC.

6.1.4.1. CRS (Participants Receiving Talquetamab and Teclistamab)

As the mechanisms of action of talquetamab and teclistamab are based on the binding and activation of T cells and the release of cytokines in the tumor environment, CRS is anticipated. As noted in Sections 2.2 and 2.3.1.1, CRS is most likely to occur during step-up dosing and the first treatment dose of talquetamab/teclistamab or potentially after a prolonged interruption in dosing of talquetamab/teclistamab. The risk for CRS has been mitigated as discussed in Section 2.3.1.1. Additionally, the participant will undergo safety monitoring, which is more intensive during the time of highest risk for CRS (see Section 6.1.4.1.1).

Clinical symptoms indicative of CRS may include, but are not limited to, fever (with or without rigors), arthralgia, nausea, vomiting, tachypnea, hypoxia, tachycardia, hypotension, headache, confusion, tremor, delirium, dyspnea, pulmonary edema, and capillary leak (Klinger 2012;

Lee 2019). Potentially life-threatening complications of CRS may include cardiac dysfunction, adult respiratory distress syndrome, neurologic toxicity, renal failure, hepatic failure, and disseminated intravascular coagulation. Trained clinical personnel should be prepared to intervene in the event of CRS and resources described in Section 6.1.3 should be available.

6.1.4.1.1. Management Guidelines for CRS

In June 2018, members of the ASTCT convened to develop a consensus for severity grading of CRS and ICANS that may be induced by CAR-T cells and can be applied to other immunomodulatory therapeutics. The grading system and description of symptoms and signs was published in April 2019. Organ toxicities associated with CRS may be graded according to NCI-CTCAE Version 5.0 and also reported in eCRF but they do not influence the overall CRS grading, which is per ASTCT (see below).

Table 20 provides toxicity grading for CRS per ASTCT guidelines and recommendations for the clinical management of CRS. At the first sign of CRS (such as fever), dosing of all study drugs should be interrupted, and the participant should be hospitalized immediately for evaluation, if not already hospitalized (see Section 6.1.3).

Infection and CRS may have a similar presentation. Therefore, investigators are strongly encouraged to evaluate for an infection at the first signs or symptoms of CRS. However, treatment for CRS should not be delayed. Cultures and imaging should be obtained; the clinical signs and symptoms should determine which tests are appropriate.

Supportive care for CRS (including but not limited to antipyretic agents, IV fluid support, vasopressors, supplemental oxygen, etc) should be administered according to the clinical manifestations of the participant's illness (see Table 20). Laboratory testing including daily monitoring of chemistry and hematology assessments and coagulation laboratory tests should be performed; pulmonary, renal, and hepatic function must be monitored closely. Rarely, severe CRS can evolve into a presentation consistent with hemophagocytic lymphohistiocytosis/macrophage activation syndrome that may require additional therapy.

Tocilizumab treatment should be considered in response to a presenting symptom of fever per investigator discretion if infection is not suspected or in the presence of persistent (>24 hours) fever. Early administration of tocilizumab should be considered in participants at high risk of severe CRS (eg, high baseline tumor burden or early fever onset). The use of growth factors, particularly G-CSF and GM-CSF, should be avoided during CRS.

See Section 6.1.3.1.1 for required monitoring for subsequent administration of talquetamab and teclistamab following an event of CRS. After an event of CRS, prophylactic pretreatment medication(s) is(are) required as described in Section 6.1.2.1.

Table 20: Recommended Management of CRS

CRS Grade[e]	Presenting Symptoms	Treatment Options	
		Tocilizumab[a]	Corticosteroids[b]
Grade 1	Temperature ≥38°C[c]	May be considered	May be considered
Grade 2	Temperature ≥38°C[c] with either: Hypotension responsive to fluids and not requiring vasopressors. Or, oxygen requirement of low-flow nasal cannula[d] or blow-by	Administer tocilizumab[b] 8 mg/kg IV over one hour (not to exceed 800 mg). Repeat tocilizumab every 8 hours as needed if not responsive to IV fluids or increasing supplemental oxygen. Limit to a maximum of 3 doses in a 24-hour period; maximum total of 4 doses.	Manage per guidance below if no improvement within 24 hours of starting tocilizumab.
Grade 3	Temperature ≥38°C[c] with either: Hypotension requiring one vasopressor with or without vasopressin. Or, oxygen requirement of high-flow nasal cannula[d], facemask, non-rebreather mask, or Venturi mask	Administer tocilizumab 8 mg/kg IV over one hour (not to exceed 800 mg). Repeat tocilizumab every 8 hours as needed if not responsive to IV fluids or increasing supplemental oxygen. Limit to a maximum of 3 doses in a 24-hour period; maximum total of 4 doses.	If no improvement, administer methylprednisolone 1 mg/kg IV twice daily or equivalent dexamethasone (eg, 10 mg IV every 6 hours). Continue corticosteroids use until the event is Grade 1 or less, then taper over 3 days.
Grade 4	Temperature ≥38°C[c] with either: Hypotension requiring multiple vasopressors (excluding vasopressin). Or, oxygen requirement of positive pressure (eg, continuous positive airway pressure, bilevel positive airway pressure, intubation, and mechanical ventilation)	Administer tocilizumab 8 mg/kg IV over one hour (not to exceed 800 mg). Repeat tocilizumab every 8 hours as needed if not responsive to IV fluids or increasing supplemental oxygen. Limit to a maximum of 3 doses in a 24-hour period; maximum total of 4 doses.	As above or administer methylprednisolone 1000 mg IV per day for 3 days per investigator discretion. If no improvement or if condition worsens, consider alternate immunosuppressants[b]

a. Refer to tocilizumab prescribing information for details.
b. Monoclonal antibodies targeting cytokines may be considered per institutional practice for unresponsive CRS.
c. Attributed to CRS. Fever may not always be present concurrently with hypotension or hypoxia as it may be masked by interventions such as antipyretics or anti-cytokine therapy (eg, tocilizumab or steroids).
d. Low-flow nasal cannula is ≤6 L/min, and high-flow nasal cannula is >6 L/min.
e. Grading is per Lee 2019 (ASTCT guidelines). All other AEs should be graded using NCI-CTCAE Version 5.0.

6.1.4.2. ICANS (Participants Receiving Talquetamab and Teclistamab) and Other Neurotoxicity

Based on the modes of action of talquetamab and teclistamab, ICANS or other neurotoxicity may occur. Early recognition of neurotoxicity is critical to management. Therefore, participants should be monitored for speech disorders; convulsions; disturbances in consciousness, confusion, disorientation, or coordination; and balance disorders. Participants should be advised to seek medical evaluation if they notice impairment in motor function (eg, weakness); changes in sensation (eg, numbness); or symptoms suggestive of possible CNS abnormalities, such as new onset of headache or mental status changes. If these or other neurologic AEs are observed, the

medical monitor must be consulted. The investigator should also exclude other causes of neurologic symptomatology (infarction/bleeding or infections [bacterial and neurotrophic viruses]) and consider cerebrospinal fluid analysis and CT/MRI.

6.1.4.2.1. Management Guidelines for ICANS

ICANS events will be graded per the ASTCT guidelines (Table 21), including administration of the ICE tool (Appendix 14) to assess encephalopathy and evaluation of other neurologic domains. Symptoms of ICANS will be graded according to NCI-CTCAE Version 5.0 and also reported in the eCRF.

At the first sign of ICANS, dosing of all study drugs should be interrupted, and the participant should be hospitalized immediately for evaluation, if not already hospitalized (see Section 6.1.3). Table 21 summarizes recommendations for the management of ICANS. Table 22 presents guidelines for the management of increased ICP/cerebral edema.

Table 21: Recommended Management of ICANS

ICANS Grade[a]	Presenting Symptoms[b]	Concurrent CRS	No Concurrent CRS
Grade 1	ICE score 7-9[c] or depressed level of consciousness[c]: awakens spontaneously.	Management of CRS as appropriate per Table 20. Monitoring of neurologic symptoms and consider neurology consultation and evaluation, per investigator discretion.	Monitor neurologic symptoms and consider neurology consultation and evaluation, per investigator discretion.
		Consider non-sedating, anti-seizure medicines (eg, levetiracetam) for seizure prophylaxis.	
Grade 2	ICE score-3-6[c] or depressed level of consciousness[c]: awakens to voice.	Management of CRS as appropriate per Table 20. If no improvement after starting tocilizumab, administer dexamethasone[e] 10 mg IV every 6 hours if not already taking other corticosteroids. Continue dexamethasone use until the event is Grade 1 or less, then taper.	Administer dexamethasone[e] 10 mg IV every 6 hours. Continue dexamethasone use until the event is Grade ≤1, then taper.
		Consider non-sedating, anti-seizure medicines (eg, levetiracetam) for seizure prophylaxis. Consider neurology consultation and other specialists (ie, intensivists) for further evaluation, as needed.	
Grade 3	ICE score-0-2[c] or depressed level of consciousness[d]: awakens only to tactile stimulus, or seizures[d], either: • any clinical seizure, focal or generalized, that resolves rapidly, or • non-convulsive seizures on electroencephalogram that resolve with intervention, or • raised ICP: focal/local edema on neuroimaging[d].	Management of CRS as appropriate per Table 20. In addition, administer dexamethasone[e] 10 mg IV with the first dose of tocilizumab and repeat dose every 6 hours. Continue dexamethasone use until the event is Grade 1 or less, then taper.	Administer dexamethasone[e] 10 mg IV every 6 hours. Continue dexamethasone use until the event is Grade ≤1, then taper.
		Consider non-sedating, anti-seizure medicines (eg, levetiracetam) for seizure prophylaxis. Consider neurology consultation and other specialists (ie, intensivists) for further evaluation, as needed.	
Grade 4	ICE score-0[c] or depressed level of consciousness[d] either: • participant is unarousable or requires vigorous or repetitive tactile stimuli to arouse, or • stupor or coma,	Management of CRS as appropriate per Table 20. As above or consider administration of methylprednisolone 1000 mg IV per day with first dose of tocilizumab and continue methylprednisolone 1000 mg IV per day for 2 or more days, per investigator discretion.	As above or consider administration of methylprednisolone 1000 mg IV per day for 3 days; if improves, then manage as above.
		Consider non-sedating, anti-seizure medicines (eg, levetiracetam) for seizure prophylaxis. Consider neurology consultation and other specialists (ie, intensivists) for further evaluation, as needed. In case of raised ICP/cerebral edema, refer to Table 22 for additional management guidelines.	

Table 21: Recommended Management of ICANS

ICANS Grade[a]	Presenting Symptoms[b]	Concurrent CRS	No Concurrent CRS
	or seizures[d], either: • life-threatening prolonged seizure (>5 min), or • repetitive clinical or electrical seizures without return to baseline in between, or motor findings[d]: • deep focal motor weakness such as hemiparesis or paraparesis, or raised ICP/cerebral edema[d], with signs/symptoms such as: • diffuse cerebral edema on neuroimaging, or • decerebrate or decorticate posturing, or • cranial nerve VI palsy, or • papilledema, or Cushing's triad.		

a. Grading per ASTCT guidelines (Lee 2019).
b. Management is determined by the most severe event, not attributable to any other cause.
c. If the participant is arousable and able to perform Mental Status assessment, the following domains should be tested: orientation, naming, following commands, writing, and attention (see ICE Tool in Appendix 14).
d. Attributable to no other cause.
e. All references to dexamethasone administration are dexamethasone or equivalent (Appendix 13).

Table 22: Guidelines for the Management of Raised ICP/Cerebral Edema

- Elevate head of participant's bed to an angle of 30 degrees.
- If participant has ommaya reservoir, drain cerebrospinal fluid to target opening pressure of <20 mm Hg.
- Hyperventilation to achieve target $PaCO_2$ (partial pressure of arterial carbon dioxide) of 28 to 30 mm Hg but maintained for no longer than 24 hours.
- Consider neurology and/or neurosurgery consultation.
- Use high-dose corticosteroids with methylprednisolone IV 1 g/day, as recommended above.
- Hyperosmolar therapy with either mannitol (20 g/dL solution) or hypertonic saline (3% or 23.4%, as detailed below):
  - Mannitol: initial dose 0.5 to 1 g/kg; maintenance at 0.25 to 1 g/kg every 6 hours while monitoring metabolic profile and serum osmolality every 6 hours and withhold mannitol if serum osmolality is ≥320 mOsm/kg, or the osmolality gap is ≥40.
  - Hypertonic saline: initial 250 mL of 3% hypertonic saline; maintenance at 50 to 75 mL/hr while monitoring electrolytes every 4 hours, and withhold infusion if serum Na levels reach ≥155 mEq/L.
  - For participants with imminent herniation: initial 30 mL of 23.4% hypertonic saline; repeat after 15 min, if needed.
- Consider IV anesthetics for burst-suppression pattern on electroencephalography.

6.1.4.3. Hypogammaglobulinemia (Participants Receiving Talquetamab and Teclistamab)

Administration of Ig replacement for hypogammaglobulinemia should be considered per institutional guidelines. Due to the potential for reactions to Ig replacement, it is recommended to avoid administration for a minimum of 48 hours after each step-up dose and after the first treatment dose of talquetamab/teclistamab, as well as during any event of CRS. The following guidance should be considered:

- Ig replacement should be used to maintain serum IgG levels ≥4 g/L regardless of active or history of infection. It is recommended to administer Ig (ie, IV Ig 0.4 g/kg every 3 to 6 weeks after talquetamab/teclistamab). After reaching a steady state, IgG levels should be measured every 3 months (Hayden 2022; Ludwig 2023).
- For serious or recurrent/chronic infections, Ig replacement should be administered per institutional guidelines.

6.1.4.4. Infection

Infection is frequently reported in patients with multiple myeloma due to both disease- and treatment-related factors causing hypogammaglobulinemia and immunosuppression (Terpos 2015; Raje 2022). BCMA-directed therapies cause depletion of B cells, resulting in hypogammaglobulinemia, and may increase the risk of infection (Mazahreh 2023). Infection, including severe events, is a known risk for participants receiving talquetamab and teclistamab (see Table 14 and the respective Investigator's Brochure).

6.1.4.4.1. Infection Prophylaxis

Prophylactic measures per institutional guidelines are recommended due to the susceptibility of patients with multiple myeloma to infections (Drayson 2019; Mohyuddin 2020). These include:

- Prophylactic Ig replacement (see Section 6.1.4.3).
- Prophylactic administration of antibiotics and antivirals per institutional guidelines.
- Pneumocystis carinii/jirovecii pneumonia prophylaxis.
- Prophylaxis for herpes zoster reactivation.
  - Initiate antiviral prophylaxis to prevent herpes zoster reactivation within 1 week after the start of administration of study treatment and continue for 3 months following study treatment. Acceptable antiviral therapy includes acyclovir, famciclovir, or valacyclovir.

In addition to the above, see permitted therapies for management of cytopenias in Section 6.9.1.

6.1.4.4.1.1. Vaccinations

Vaccination is allowed per local guidelines (including annual influenza and SARS-CoV-2 vaccines approved or authorized for emergency use; see also Appendix 7); however, some types of vaccines (eg, live, attenuated) are not permitted (see Section 6.9.3). Note that antibody responses to vaccines may be suboptimal during study treatment (Ariza-Heredia and Chemaly 2015) and that the expected adverse reactions to vaccines (eg, fever) and CRS may have a similar presentation.

For COVID-19 vaccinations, refer to Appendix 7.

6.1.4.4.2. Infection Management

See guidance regarding non-hematologic AEs for which dosing should be interrupted in Table 27 for talquetamab and teclistamab and Section 6.6.2.3.2 for pomalidomide and bortezomib.

It is recommended that empirical broad-spectrum antibiotics are commenced while performing diagnostic tests in participants with febrile neutropenia or signs/symptoms of infection per institutional guidelines (Raje 2022).

Targeted antimicrobial agents are recommended depending on clinical, radiological, and microbiological findings (Raje 2022).

For participants with persistent fever with undetermined cause, consider testing for opportunistic infections including new onset or reactivation of viral infections such as herpesviruses (eg, herpes simplex virus, varicella zoster virus, cytomegalovirus, Epstein-Barr virus), parvovirus B19, and adenovirus. Diagnostic imaging should be considered as clinically indicated.

PML, which can be fatal, has also been reported in patients receiving teclistamab. Monitor any new onset of or changes in pre-existing neurological signs or symptoms. If PML is suspected, withhold treatment with teclistamab and initiate appropriate diagnostic testing. Discontinue teclistamab if PML is confirmed.

See guidance regarding requirements for restarting talquetamab and teclistamab therapy pertinent to infection in Section 6.6.2.2. Additionally, following any dosing interruption of talquetamab/teclistamab, there must be no evidence of an active bacterial, viral, or fungal infection before proceeding to the next dose of study drug.

6.1.4.4.3. COVID-19 Infection

For prevention and treatment of COVID-19 infections, refer to Appendix 7.

6.1.4.4.4. HBV Reactivation

HBV reactivation is a potential risk of study treatment. Primary antiviral prophylaxis is permitted per local standard of care. HBV-DNA testing by PCR is mandatory for participants at risk for HBV reactivation (for timing of testing, see Section 8.3.5.3). For participants who are diagnosed with HBV reactivation (ie, HBV-DNA becomes detectable) while on treatment, suspend treatment and any steroids and institute appropriate treatment. Manage participants according to current clinical guidelines and consider consulting a hepatitis disease expert as clinically indicated. Resumption of treatment with concomitant antiviral prophylaxis as per standard of care in participants whose HBV reactivation is adequately controlled should be discussed with physicians with expertise in managing HBV and approved by the sponsor.

6.1.4.4.5. HCV and HIV Reactivation

For participants who are diagnosed with HCV reactivation (ie, HCV-RNA becomes detectable) while on treatment, suspend study treatment and any steroids and institute appropriate treatment. Resumption of study treatment should be discussed with physicians with expertise in managing HCV and approved by the sponsor.

For participants who are diagnosed with HIV reactivation (ie, HIV-RNA becomes detectable) while on treatment, suspend study treatment and any steroids and institute appropriate treatment. Resumption of study treatment should be discussed with physicians with expertise in managing HIV and approved by the sponsor.

6.1.4.5. sARRs

sARRs may manifest as wheezing, flushing, hypoxemia, fever, chills, rigors, bronchospasm, headache, rash, pruritus, arthralgia, hypo- or hypertension, or other symptoms and may occur with SC administration of talquetamab and teclistamab. If a sARR occurs during the administration of talquetamab or teclistamab, the injection should be interrupted immediately if possible. Symptoms of sARRs should be managed per the recommendations in Table 23.

After a sARR event, participants being treated with talquetamab (Arms A and B) and teclistamab (Arm B) should receive pretreatment medication per Section 6.1.2.1 for at least the subsequent dose of the study drug to which the event was related.

Table 23: Guidelines for the Management of sARRs

NCI-CTCAE Grade(s)	Presenting Symptoms	Recommended Treatment/Intervention
Grade 1 or Grade 2	Mild or moderate reaction; requires therapy interruption but responds promptly to symptomatic treatment	Start IV fluids; give diphenhydramine 50 mg (or equivalent) IV and/or paracetamol 650 to 1000 mg (acetaminophen); consider corticosteroids and bronchodilator therapy; monitor participant closely until recovery from symptoms. Consider hospitalization if Grade 2 laryngeal edema or Grade 2 bronchospasm occurs.

Table 23: Guidelines for the Management of sARRs

NCI-CTCAE Grade(s)	Presenting Symptoms	Recommended Treatment/Intervention
Grade 3	Severe, prolonged (ie, not rapidly responsive to symptomatic medication; recurrence of symptoms following initial improvement; hospitalization indicated for other clinical sequelae [eg, renal impairment, pulmonary infiltrates])	Start IV saline infusion; recommend bronchodilators (if indicated), epinephrine 0.2 to 1 mg of a 1:1000 solution for SC administration or 0.1 to 0.25 mg of a 1:10000 solution injected slowly for IV administration, and/or diphenhydramine 50 mg IV with methylprednisolone 100 mg IV (or equivalent), as needed, and other drugs as appropriate. Investigators should follow their institutional guidelines for the treatment of anaphylaxis. Monitor until medically stable, per the investigator's medical judgment. See Section 6.1.3.1.1 for required hospitalization for subsequent administration(s) of talquetamab/teclistamab after a Grade 3 sARR related to talquetamab/teclistamab. Participants who experience a Grade 4 sARR must discontinue study treatment (see below).
Grade 4 Life-threatening; pressor or ventilator support indicated		
Discontinuation of treatment		See sARRs requiring discontinuation of study treatment in Section 7.1.2 (talquetamab SC) and Section 7.1.3 (teclistamab SC).
General		**Prophylactic medications (after initial event)** must be used as described in Section 6.1.2.1.
		In the case of late-occurring hypersensitivity symptoms (eg, appearance of a localized or generalized pruritis within 1 week after treatment), symptomatic treatment must be given (eg, PO antihistamine or corticosteroids), as appropriate.

6.1.4.6. Infusion and Injection-site Reactions

6.1.4.6.1. Infusion and Injection-site Reactions Related to Talquetamab and Teclistamab

SC administration of any medication can be associated with localized infusion/injection-site reactions including, but not limited to, induration and erythema. Infusion/injection-site reactions should be managed per institutional standards.

6.1.4.6.2. Injection-site Reactions Related to Bortezomib

For participants with unacceptable toxicity at the local injection site despite dose modifications or change in injection concentration (see Section 6.6.2.3.2), bortezomib may be administered IV as a 3 to 5 second bolus injection. Refer to local prescribing information for further details regarding SC or IV administration.

6.1.4.6.3. Infusion Reactions in Participants Treated with Elotuzumab

If a Grade ≥2 infusion reaction occurs during elotuzumab administration, the infusion must be interrupted and appropriate medical and supportive measures instituted. Upon resolution to Grade 1 or lower, elotuzumab should be restarted at 0.5 mL per minute and gradually increased at a rate of 0.5 mL per minute every 30 minutes as tolerated to the rate at which the infusion reaction occurred. The escalation regimen can be resumed if there is no recurrence of the infusion reaction.

In participants who experience an infusion reaction, vital signs should be monitored every 30 minutes for 2 hours after the end of the elotuzumab infusion. If the infusion reaction recurs, stop the elotuzumab infusion and do not restart on that day. Severe infusion reactions may require permanent discontinuation of elotuzumab therapy and emergency treatment.

6.1.4.7. TLS

Participants should be monitored for evidence of TLS. Management of TLS, including forced diuresis and correction of electrolyte disturbances (eg, hyperkalemia, hyperuricemia, and hypocalcemia), is highly recommended. It is also recommended that high-risk participants (ie, those with a high tumor burden [a participant with ≥60% PC infiltrate on the bone marrow biopsy or aspirate, whichever is higher or a participant with multiple soft-tissue plasmacytomas]), be treated prophylactically in accordance with institutional standards (eg, rehydration, diuretics, allopurinol 300 mg daily, and medication to increase urate excretion).

TLS has been observed in patients receiving talquetamab. Participants should be monitored for evidence of TLS and treated prophylactically in accordance with local standards (eg, rehydration, allopurinol 300 mg daily, and medications to increase urate excretion) at investigator's discretion.

6.1.4.8. Immune-related AEs (irAEs)

Treatment with talquetamab and teclistamab may lead to specific irAEs including but not limited to pneumonitis, colitis, hypophysitis, and hypothyroidism. Continuous, careful monitoring and timely management of irAEs may help to mitigate more severe toxicity. Symptomatic and best supportive care measures for potential irAEs should be in progress as soon as clinically indicated in accordance with institutional standards. These treatments may include corticosteroids and/or other immunosuppressive agents as required and interrupting or discontinuing talquetamab and/or teclistamab.

6.1.4.9. Skin and Nail Toxicities

A variety of rashes have been seen in clinical studies of talquetamab, ranging from local injection site erythema to generalized rashes. Skin changes may also include skin peeling and itching. Early use of topical treatments including emollients is encouraged to mitigate skin peeling that has been seen in clinical studies of talquetamab. It is unknown if these rashes are mediated by target expression. Cutaneous reactions have also been reported in association with IMiD administration. Rashes can appear with early doses or be a late manifestation (beyond one cycle) and should be managed per institutional standards.

Nail changes may include nail loss, peeling, ridging, discoloration, breaking, and separation. Nail changes are often a late manifestation (beyond the first cycle of talquetamab) and symptomatic management with nail soaks, topical moisturizers, and topical steroids is recommended. Nail complications that persist should be evaluated by a dermatologic consult. Good hygienic practices should be followed. Participants should be advised to keep fingers and toes cleaned and nails trimmed (avoid aggressive trimming), and to wear comfortable shoes with extra room around the toes. Nail polish and imitation fingernails should not be worn until the nails have grown out and returned to normal. Participants should use gloves for housecleaning and gardening to minimize

nail damage and prevent infection. The presence of infection should be monitored (periungual edema, erythema, tenderness, and/or discharge) with the acquisition of cultures and/or implementation of empiric antibiotic therapy as appropriate, based on clinical judgment and institutional standards.

It is important for site staff to recognize rashes on the body that are distinct from injection-site reactions. Monitoring of rash progression is critical and early intervention, including treatment with systemic steroids, may be necessary. Rashes occurring during the first cycle should be managed more aggressively with topical steroids and early consideration of a short course of PO steroids to reduce the risk of rash progression. Rashes that have not responded to treatment should be evaluated by a dermatologic consult. If a rash occurs, a skin biopsy for sponsor review may be obtained.

6.1.4.10. Oral Toxicity

Oral side effects may occur, and may include dry mouth, altered taste, loss of taste, and difficulty swallowing.

Oral toxicities should be managed as appropriate, based on best clinical judgment and institutional standards which may include supportive care, such as saliva stimulating agents, steroid mouth wash, or consultation with a nutritionist.

6.1.4.11. Weight Loss

Over time, notable weight loss may occur. Weight change should be monitored regularly during therapy. Clinically significant weight loss should be further evaluated. Adjustment to weight-based concomitant therapy (eg, cardiac medications) should be considered, as required.

6.2. Preparation/Handling/Storage/Accountability

Talquetamab and Teclistamab Preparation/Handling/Storage

Talquetamab and teclistamab must be protected from light during storage and stored at controlled temperatures according to the label. Talquetamab and teclistamab must be prepared according to the sponsor's instructions provided in the IPPI. Detailed instructions for storage conditions and handling will accompany clinical drug supplies to the clinical study sites. Talquetamab and teclistamab will be stored according to applicable expiry dates. Refer to the IPPI for additional guidance regarding talquetamab and teclistamab preparation, handling, and storage.

Preparation/Handling/Storage of Combination Agents

Refer to the SIPPM and respective labels for elotuzumab, bortezomib, pomalidomide, and dexamethasone for guidance on preparation, handling, and storage.

Accountability

Except as noted in Table 16 (Arm A [Tal-P] and Arm B [Tal-Tec]), Table 17 (Arm C [EPd]), and Table 18 (Arm C [PVd]), study drugs will be administered or prescribed by qualified site staff,

and the details of the components of each treatment combination will be recorded in the eCRF. For any study medication taken at home, participants should bring the container(s) with all remaining doses to each study visit for counting by site staff. Study accountability and additional details regarding study treatment are provided in the SIPPM or equivalent document.

The investigator is responsible for ensuring that all study treatment received at the site is inventoried and accounted for throughout the study. The study treatment administered to the participant must be documented on the treatment accountability form. All study treatment will be stored and disposed of according to the sponsor's instructions. Study site personnel must not combine contents of the study treatment containers.

Study treatment must be handled in strict accordance with the protocol and the container label and must be stored at the study site in a limited-access area or in a locked cabinet under appropriate environmental conditions. Unless otherwise instructed, unused study treatment, and study treatment returned by the participant, must be available for verification by the sponsor's study site monitor during on-site monitoring visits. The return to the sponsor of unused study treatment, or used returned study treatment for destruction, will be documented on the treatment return form. When the study site is an authorized destruction unit and study treatment supplies are destroyed on-site, this must also be documented on the treatment return form.

Potentially hazardous materials containing hazardous liquids, such as used ampules, needles, syringes and vials, must be disposed of immediately in a safe manner and therefore will not be retained for treatment accountability purposes.

Study treatment must be dispensed under the supervision of the investigator or a qualified member of the study site personnel, or by a hospital/clinic pharmacist. Study treatment will be supplied only to participants participating in the study. Returned study treatment must not be dispensed again, even to the same participant. Study treatment may not be relabeled or reassigned for use by other participants. The investigator agrees neither to dispense the study treatment from, nor store it at, any site other than the study sites agreed upon with the sponsor. Further guidance and information for the final disposition of unused study treatments are provided in the IPPI and/or SIPPM.

6.3. Assignment to Study Treatment

Treatment Allocation

Procedures for Randomization and Stratification

Central randomization will be implemented in this study. Participants will be randomly assigned to 1 of 3 treatment sequence groups based on a computer-generated randomization schedule prepared before the study by or under the supervision of the sponsor. The randomization will be balanced by using randomly permuted blocks and will be stratified by number of prior lines of therapy (1 versus ≥2), ISS stage at screening (I versus II/III), and investigator's intended choice of therapy (EPd versus PVd). The IWRS will assign a unique treatment code, which will dictate the treatment assignment and matching study treatment kit for the participant. The requestor must use

his or her own user identification and personal identification number when contacting the IWRS, and will then give the relevant participant details to uniquely identify the participant.

6.4. Blinding, Masking

As this is an open-label study, blinding procedures are not applicable.

6.5. Study Treatment Compliance

Study treatment is to be prescribed only by the principal investigator or a qualified physician listed as a subinvestigator on required forms. Drug supplies for each participant will be inventoried and accounted for throughout the study. Dispensing/administration of the study drug(s) must also be recorded in the participant's source documents.

For study treatment administered at the study site, the details of each administration will be recorded in the eCRF (including date, start and stop times of the administration of study drug(s), location of administration, and the volume injected, as applicable). Precautions associated with the use of the components of study treatment and prohibited or restricted concomitant therapies will be reviewed with the participant. Study drug(s) start and stop dates, including dates for any dosing interruptions or dose reductions, will also be recorded in the eCRF. Additional details are provided in the SIPPM or equivalent document.

Instructions for proper self-administration and at home storage conditions should be provided for PO premedications and any study treatment medications (eg pomalidomide) that will be administered at home. The amount dispensed will be recorded and compared with the amount brought to each study visit. Compliance of premedication administered at home will be assessed at each visit (by recording and reconciling the number of tablets dispensed to and taken by each participant) and documented in the source documents.

Alternative, site-specific methods to ensure compliance with outpatient medication administration is permissible. Deviation(s) from the prescribed dose regimen should be recorded in the eCRF. Any dose/dosage adjustment should be overseen by medically-qualified study site personnel (principal or subinvestigator unless an immediate safety risk appears to be present). Except for management of TEAEs, if the dosing compliance is not 100%, then the investigator or designated study site personnel should reinstruct the participant regarding proper dosing procedures, and the participant may continue study treatment. If multiple instances of noncompliance occur, the sponsor's medical monitor should be notified.

6.6. Dose Modification

Any dose/dosage adjustment must be overseen by medically-qualified study site personnel (principal or subinvestigator unless an immediate safety risk appears to be present).

Toxicities should be attributed, whenever possible, to a specific component of study treatment so that dose delays/interruption, dose skipping, or dose reduction can be made accurately.

For talquetamab and teclistamab, dosing interruption is the primary method of management of AEs. Dose reductions during step-up dosing and Cycle 1 are not permitted. Changes in dosing frequency and/or dose reduction(s) of talquetamab and teclistamab by 50% may be considered starting in Cycle 2 for participants with talquetamab/teclistamab-related AEs after consultation with and approval by the sponsor. A re-escalation or return to protocol-specified dosing schedule may be permitted in the absence of a recurrence of the AE that led to a reduction and if there is a clinical benefit and acceptable risk profile in the judgement of the investigator.

Discontinuation criteria are presented in Section 7.1. Participants who discontinue treatment with any 1 or more component(s) of study treatment (talquetamab, teclistamab, elotuzumab, bortezomib, or pomalidomide) may continue to receive treatment with the other components of study treatment, as assigned.

6.6.1. Guidance for Dosing Interruption or Skipping (All Study Drugs)

6.6.1.1. Cycle Delays

If a participant experiences toxicity and the study treatment cannot be given on Day 1 of a cycle, the cycle must be delayed (applicable to Cycle 2+; see Section 6.1 regarding special instructions for Cycle 1). If Day 1 of a cycle is delayed, the full cycle duration will remain (28 days for Arm A, Arm B and the EPd option for Arm C; 21 days for the PVd option for Arm C), and Day 1 of subsequent cycles should be adjusted accordingly to maintain the cycle duration. Day 1 of a cycle must not be skipped.

If at least one component of the regimen can be given, the cycle can begin as planned.

6.6.1.2. Guidance for Dosing Interruption of a Study Drug During a Cycle

See Section 7.1 for information regarding permanent discontinuation of study drug(s).

If a component of the regimen cannot be given on a dosing day after Day 1 for reasons other than CRS or ICANS (see Table 27), the other study drugs should be administered as scheduled.

Dosing decisions should be discussed with the sponsor. If the step-up dose schedule is interrupted and later reinitiated, the dose, schedule, and associated safety assessments must be discussed with the sponsor before resuming treatment.

6.6.1.2.1. Delay/Skipping of Study Drugs

6.6.1.2.1.1. Talquetamab and Teclistamab Dose Skips/Delays

Dose reductions of talquetamab/teclistamab could be considered after consultation with the sponsor. Repeat of step-up dose(s) and pretreatment medication may be required (according to Table 25).

Skipped Doses

Guidance for skipped doses is provided in Table 24 and guidance for restarting doses is provided in Table 25.

Table 24: Talquetamab and Teclistamab Dose Skips

Study Drug(s)	Dosing Frequency	Skip Dose if Dosing Interrupted	Resume Dosing
Talquetamab[a]	Q2W	>7 days from planned dose date[b]	Next planned Q2W dosing day
	Q4W	Not applicable[c]	Next planned Q4W dosing day
Teclistamab[a]	Q4W	Not applicable[c]	Next planned Q4W dosing day

Note: Doses of talquetamab and teclistamab during the first cycle should not be skipped

a. If one drug is held for an AE then the other can continue, and when the AE has resolved both drugs should be synchronized to the same day at the next scheduled dosing day.

b. If a dose is delayed for <7 days, then it should be given and not skipped. The subsequent dose should be scheduled a minimum of 7 days later, and next cycle should be resumed as scheduled. The next cycle can be delayed if treatment criteria are not yet met.

c. Day 1 of a cycle cannot be skipped. If talquetamab or teclistamab cannot be given, delay cycle and adjust Day 1 of subsequent cycles accordingly to maintain a 28-day cycle.

Restarting Therapy

If a dose of teclistamab or talquetamab is delayed, therapy should be restarted based on the recommendations in Table 25. The treatment schedule should be adjusted to maintain the appropriate interval as specified in the SoA (Table 4 and Table 5).

As noted in Table 25, dose delays beyond a specified interval from the planned dose must be discussed and approved by the sponsor before restarting treatment therapy. Pretreatment medications, including dexamethasone, must be administered prior to all repeat step-up doses, and the first repeat treatment dose (see Section 6.1.2).

Table 25: Recommendations for Restarting Therapy with Talquetamab and Teclistamab After Dose Delay (Arm A [Tal-P] and Arm B [Tal-Tec])

Last Dose Administered	Duration Since Last Dose	Action[a]
Talquetamab 0.01 mg/kg dose Teclistamab 0.06 mg/kg dose	>7 days	Talquetamab: repeat talquetamab 0.01 mg/kg Teclistamab: repeat teclistamab 0.06 mg/kg
Talquetamab 0.06 mg/kg dose Teclistamab 0.3 mg/kg dose	8 to 28 days	Talquetamab: repeat talquetamab 0.06 mg/kg Teclistamab: repeat teclistamab 0.3 mg/kg
	>28 days	Must be discussed with and approved by the sponsor Talquetamab: restart talquetamab at 0.01 mg/kg Teclistamab: restart teclistamab at 0.06 mg/kg
Talquetamab 0.4 mg/kg dose Teclistamab initial 1.5 mg/kg dose	8 to 35 days	Talquetamab: repeat talquetamab 0.4 mg/kg Teclistamab: repeat teclistamab 1.5 mg/kg
	36 to 56 days	Must be discussed with and approved by the sponsor Talquetamab: restart talquetamab at 0.06 mg/kg Teclistamab: restart teclistamab at 0.3 mg/kg
	>56 days	Must be discussed with and approved by the sponsor Talquetamab: restart talquetamab at 0.01 mg/kg Teclistamab: restart teclistamab at 0.06 mg/kg

Last Dose Administered	Duration Since Last Dose	Action[a]
Talquetamab initial 0.8 mg/kg dose Teclistamab subsequent 1.5 mg/kg dose	36 to 56 days	Must be discussed with and approved by the sponsor Talquetamab: restart talquetamab at 0.4 mg/kg Teclistamab: restart teclistamab at 1.5 mg/kg
	>56 days	Must be discussed with and approved by the sponsor Talquetamab: restart talquetamab at 0.01 mg/kg Teclistamab: restart teclistamab at 0.06 mg/kg
Talquetamab Q2W 0.8 mg/kg dose	36 to 56 days	Must be discussed with and approved by the sponsor Talquetamab: restart talquetamab at 0.4 mg/kg
	>56 days	Must be discussed with and approved by the sponsor Talquetamab: restart talquetamab at 0.01 mg/kg
Talquetamab Q4W 0.8 mg/kg dose Teclistamab Q4W 3.0 mg/kg dose	43 to 70 days	Must be discussed with and approved by the sponsor Talquetamab: restart talquetamab at 0.06 mg/kg Teclistamab: restart teclistamab at 0.3 mg/kg
	>70 days	Must be discussed with and approved by the sponsor. Talquetamab: restart talquetamab at 0.01 mg/kg Teclistamab: restart teclistamab at 0.06 mg/kg

[a] Pretreatment medications must be administered prior to dose

6.6.1.2.1.2. Pomalidomide Dose Skips

Pomalidomide should be taken at the same time each day. If the expected dose time of pomalidomide is missed and <12 hours have elapsed from the time of regular dose administration, the participant should take the missed dose as soon as possible and continue with the next dose at the normal time. If the expected dose time is missed and ≥12 hours have passed since the time of regular dose administration, the missed dose should be skipped and not made up (see the prescribing information). The participant should not take 2 doses of pomalidomide at the same time.

Except as noted in Section 6.6.1 and Section 6.6.2.3.2, pomalidomide dosing may continue even if talquetamab, bortezomib, elotuzumab, and/or dexamethasone are delayed within a cycle.

6.6.1.2.1.3. Bortezomib Dose Skips

Bortezomib dosing may be delayed up to 48 hours, however subsequent doses must be adjusted to account for the delay. Note that there should be at least 3 days between doses of bortezomib. Skipped doses of bortezomib will not be made up later in the cycle.

6.6.1.2.1.4. Elotuzumab Dose Skips and Dose Delays

In Cycles 1 to 2, elotuzumab doses that fall outside of the prespecified window of -1 to +3 days must be skipped.

From Cycle 3 onwards, elotuzumab dosing may be delayed for up to 1 week. If unable to administer within 1 week, then the dose should be skipped and resumption of the elotuzumab continues per the protocol defined schedule.

6.6.2. Guidance for Dose Modification

Recommendations for dose reduction and interruption of study treatments due to AEs are presented in the relevant subsections below. Dose modifications are suggested, but physician discretion and clinical judgment should prevail. Once dose reduction has been implemented for a participant, a return to the protocol-specified dosing level should not occur for that participant unless in the judgment of the investigator there is clinical benefit and a reasonable and acceptable risk profile.

6.6.2.1. Talquetamab and Teclistamab Dose Modification/Dose Interruption for Hematologic Toxicity

Doses of talquetamab and teclistamab must be interrupted due to hematologic toxicity per Table 26.

In case of hematologic toxicities not related to talquetamab (Arms A and B) and teclistamab (Arm B only) during Cycle 1 and 2, after review and approval by the sponsor's clinical team, talquetamab and teclistamab (as appropriate) might resume before achieving laboratory results called out in Table 26.

Table 26: Talquetamab and Teclistamab Dosing Interruption for Hematologic AEs

Hematologic AE	Talquetamab	Teclistamab
Neutropenia[a]		
• ANC <0.5×10^9/L • febrile neutropenia (fever ≥38.5°C and ANC <1×10^9/L) • ANC <1×10^9/L and infection of any grade	**Interrupt until ANC is ≥1×10^9/L**	**Interrupt until ANC is ≥1×10^9/L and fever resolves**
	Monitor CBC at least weekly	Monitor CBC at least weekly
Thrombocytopenia		
Grade 3: Platelet count 25-49×10^9/L or evidence of bleeding with platelet count ≥25 to 50×10^9/L *Grade 4:* Platelet count <25×10^9/L	**Interrupt until platelet count is ≥50×10^9/L**	**Interrupt until platelet count is ≥50×10^9/L**
	Monitor CBC at least weekly	Monitor CBC at least weekly
Anemia		
Grade 4	**Interrupt**	**Interrupt**
	Monitor CBC at least weekly	Monitor CBC at least weekly
Return to Grade ≤3	Resume	Resume

NOTE: See Section 6.6.2 for additional guidance regarding dose reduction and requirements for subsequent dose escalation.

a. The investigator should consider the use of growth factors.

6.6.2.2. Talquetamab and Teclistamab Dose Interruption for Non-hematologic Toxicities

Doses of talquetamab and teclistamab must be interrupted due to non-hematologic toxicity per Table 27.

Following a dose interruption of talquetamab or teclistamab due to 1 or more of the non-hematologic AEs listed in Table 27, any clinically significant non-hematologic AEs other than

weight loss, CRS, or ICANS must resolve to Grade ≤1 or to baseline (as documented in the medical history) before proceeding to the next dose. For TEAEs that persist but are stable (ie, weight loss), discuss with sponsor. Participants with Grade 3 weight loss with improvement to Grade 2 may resume dosing with sponsor approval. CRS (fever, hypoxia, and hypotension) and ICANS (in the opinion of the investigator) must fully resolve (ie, end date reported in the eCRF) before the next administration of talquetamab or teclistamab and until the following criteria must be met:

- Minimum of 12 hours since the resolution of fever without use of antipyretics.
- Minimum of 24 hours since the last administration of tocilizumab or equivalent.

Additionally, following dosing interruption of talquetamab or teclistamab, there must be no evidence of an active bacterial, viral, or fungal infection before proceeding to the next dose of either study drug.

Table 27: Talquetamab and Teclistamab Dosing Interruption for Non-hematologic AEs

Non-hematologic Toxicity	Talquetamab	Teclistamab
First sign of CRS (see Section 6.1.4.1.1)[a]	**Interrupt**	**Interrupt**
First sign of ICANS (see Section 6.1.4.2.1)[a]	**Interrupt**	**Interrupt**
Grade ≥3 TEAEs (except disease-related pain) or Grade ≥3 laboratory toxicity that are clinically significant (eg, requiring intervention and/or associated with an AE)	**Interrupt**	**Interrupt**
HBV reactivation (see Section 6.1.4.4.4)	**Interrupt**	**Interrupt**
Grade 1 or 2 infection occurring on or before first treatment dose and per clinical judgment of the investigator for other doses	**Interrupt**	**Interrupt**

a. See Sections 7.1.2 and 7.1.3 for CRS or ICANS events that require permanent discontinuation of talquetamab and teclistamab.

6.6.2.3. Guidance for Pomalidomide and Bortezomib

The starting doses for pomalidomide are provided in Table 4 (Arm A), Table 8 (Arm C [EPd]), and Table 11 (Arm C [PVd]). If dose reductions are required for pomalidomide (for criteria see Sections 6.6.2.3.1 and 6.6.2.3.2), the dose reduction level is to be 1 mg below the current dose.

The starting dose for bortezomib is 1.3 mg/m^2 (Table 11; Arm C [PVd]). If dose reductions are required for bortezomib (for criteria see Sections 6.6.2.3.1 and 6.6.2.3.2), the dose reduction levels are as follows:

- Dose level –1: 1.0 mg/m^2
- Dose level –2: 0.7 mg/m^2
- Dose level –3: Discontinue bortezomib

6.6.2.3.1. Hematologic AEs

Dosing of pomalidomide or bortezomib must be interrupted due to hematologic AEs per Table 28. Subsequently, the dose of pomalidomide or bortezomib should be reduced as indicated. In Arm C (PVd), neutrophil count must be ≥1×10^9/L and platelet count must be ≥50×10^9/L to initiate a new cycle (per prescribing information).

Table 28: Pomalidomide and Bortezomib Dosing Interruption or Dose Reduction Due to Hematologic AEs

Hematologic AE	Pomalidomide	Bortezomib
Neutropenia[a]		
Any of the following: • ANC <0.5×10^9/L • febrile neutropenia (fever ≥38.5°C and ANC <1×10^9/L) • ANC <1×10^9/L and infection of any grade	**Interrupt**	**Interrupt**
	Monitor CBC at least weekly.	
Return to ANC ≥1×10^9/L	Resume at one dose level lower than the previous dose for each occurrence (see Section 6.6.2.3). If pomalidomide was at 2 mg prior to interruption, consider resuming pomalidomide at 2 mg and reducing the bortezomib dose.	First occurrence: Resume at same dose. For each subsequent occurrence, resume bortezomib at the same dose as previously (if pomalidomide is being reduced) or one dose level lower (see Section 6.6.2.3)
Thrombocytopenia		
Platelet count <25×10^9/L	**Interrupt**	**Interrupt**
	Monitor CBC at least weekly.	
Return to platelet count ≥50×10^9/L	Resume one dose level lower than the previous dose for each occurrence (see Section 6.6.2.3). If pomalidomide was at 2 mg prior to interruption, consider resuming pomalidomide at 2 mg and reducing the bortezomib dose	Resume bortezomib at the prior dose level (if pomalidomide is being reduced) or one dose level lower (see Section 6.6.2.3)
Anemia		
Grade 4 (Life-threatening consequences; urgent treatment indicated)	**Interrupt**	**Interrupt**
	Monitor CBC at least weekly.	
Return to Grade ≤3	May resume at previous dose	Consider resuming at one dose level lower

NOTE: See Section 6.6.2 for additional guidance regarding dose reduction and requirements for subsequent dose escalation.

a. The investigator should consider the use of growth factors.

6.6.2.3.2. Non-hematologic AEs

Pomalidomide

Dosing of pomalidomide should be interrupted due to non-hematologic AEs per Table 29; dose reduction may be considered as noted.

Table 29: Pomalidomide Dosing Interruption or Dose Reduction for Non-hematologic AEs

Non-hematologic AE	Dosing Interruption	Dosing Resumption[a]
Grade ≥3 or 4 events that do not require discontinuation (Section 7.1.4)	**Interrupt**	May resume pomalidomide at one dose level less than previous dose (see Section 6.6.2.3) at investigator discretion when the AE has resolved to Grade ≤2
Grade 2 skin rash	Investigator discretion	May resume pomalidomide at previous dose
CRS (see Section 6.1.4.1) or ICANS (see Section 6.1.4.2)	Pomalidomide should be suspended at the first sign	CRS (fever, hypoxia, and hypotension) and ICANS must fully resolve before the next administration of pomalidomide
Interstitial lung disease	Interstitial lung disease and related events, including cases of pneumonitis, have been observed with pomalidomide. Careful assessment of participants with an acute onset or unexplained worsening of pulmonary symptoms should be performed to exclude interstitial lung disease. Dosing of pomalidomide should be interrupted pending investigation of these symptoms and if interstitial lung disease is confirmed, appropriate treatment should be initiated. Pomalidomide should only be resumed after a thorough evaluation of the benefits and the risks.	
PML	• Participants should be monitored for any new or worsening neurologic, cognitive, or behavioral signs or symptoms that may be suggestive of PML as part of the differential diagnosis of CNS disorders. • If PML is suspected, withhold administration of pomalidomide; participants should be promptly referred to a specialist and appropriate diagnostic testing should be initiated. Discontinue pomalidomide if PML diagnosis is confirmed. If the diagnosis is excluded, pomalidomide can be restarted at the same dose.	

a. See Section 6.6.2 for additional guidance regarding dose reduction and requirements for subsequent dose escalation.

Bortezomib

Treatment guidelines for bortezomib-related neuropathy are outlined in Table 30. See also Section 6.1.4.6.2 for modification of administration of bortezomib in participants due to injection site reactions.

For other Grade 3 or 4 non-hematologic AEs that are determined by the investigator as may be related to bortezomib, dosing with bortezomib should be interrupted and resumed once the AE has resolved to Grade ≤2. The dose may be reduced to the next lower dose level (see Section 6.6.2.3) or the administration schedule changed to weekly.

Table 30: Treatment Guidelines for Bortezomib-related Neuropathy

Severity (per NCI-CTCAE)	Bortezomib Dosing Interruption or Dose Modification[a]
Grade 1 (asymptomatic; clinical or diagnostic observations only) with no pain or loss of function	No action
Grade 1 with pain or Grade 2 (moderate symptoms; limiting instrumental ADL) **NOTE**: Instrumental ADL refers to preparing meals, shopping for groceries or clothes, using telephone, managing money, etc.	• Withhold bortezomib until the AE resolves or returns to baseline. • Reduce bortezomib dosing by one level (see Section 6.6.2.3) OR Change treatment schedule to 1.3 mg/m^2 once per week (Days 1 and 8 of a 21-day cycle)
Grade 2 with pain or Grade 3 (severe symptoms; limiting self-care ADL) **NOTE**: Self-care ADL refers to bathing; dressing and undressing; feeding self; using the toilet; taking medicinal products; and not bedridden.	Withhold bortezomib until the AE resolves Bortezomib therapy may be reinitiated at a reduced dose level of 0.7 mg/m^2 and change treatment schedule to once per week (Days 1 and 8 of a 21-day cycle)
Grade 4 (life-threatening consequences; urgent treatment indicated) and/or severe autonomic neuropathy	Permanently discontinue bortezomib

a. See Section 6.6.2 for additional guidance regarding dose reduction and requirements for subsequent dose escalation.

6.6.2.4. Dexamethasone Dose Modifications

Table 31 (EPd option of Arm C) and Table 32 (Arm A [Tal-P] and PVd option of Arm C) present dose reductions guidelines for dexamethasone. Any dose/dosage adjustment should be overseen by medically-qualified study site personnel (principal or subinvestigator unless an immediate safety risk appears to be present). If recovery from AEs is prolonged beyond 14 days, then the dose of dexamethasone will be decreased by one dose level when the dose is restarted.

Table 31: Dexamethasone Dose Reduction Guidelines (Arm C [EPd])

Dose Level	Reducing Dexamethasone on Weeks with Elotuzumab		Reducing Dexamethasone on Weeks Without Elotuzumab	
	PO	IV	PO	IV
0	≤75 years old – 28 mg	8 mg	≤75 years old – 40 mg	N/A
	>75 years old – 8 mg		>75 years old – 20 mg	
-1	≤75 years old – 12 mg	8 mg	≤75 years old – 20 mg	N/A
	>75 years old – 0 mg		>75 years old – 12 mg	
-2	≤75 years old – 0 mg	8 mg	≤75 years old – 12 mg	N/A
	>75 years old – 0 mg		>75 years old – 8 mg	
-3	≤75 years old – 0 mg	Contact medical monitor	≤75 years old – 0 mg	N/A
	>75 years old – 0 mg		>75 years old – 0 mg	

Table 32: Dexamethasone Dose Reduction Guidelines (Arm A [Tal-P] and Arm C [PVd])

	Arm A (Tal-P)	**Arm C (PVd)**
	Participants <75 years of age	***Participants ≤75 years of age***
Dose Level	**Weekly Dose**	**Daily Dose**
Starting Dose	40 mg	20 mg
- 1	20 mg	12 mg
- 2	10 mg	8 mg
	Participants <75 years of age with BMI <18.5 or ≥75 years of age	*Participants >75 years of age*
Dose Level	**Weekly Dose**	**Daily Dose**
Starting Dose	20 mg	10 mg
- 1	12 mg	6 mg
- 2	8 mg	4 mg

NOTE: Dexamethasone should be discontinued if the participant is unable to tolerate 8 mg if <75 years of age or 4 mg if <75 years of age with BMI <18.5 or ≥75 years of age.

At investigator discretion, dexamethasone may be tapered prior to complete discontinuation according to institutional practice. Dexamethasone dosing may continue even if talquetamab SC, elotuzumab (as applicable) and/or pomalidomide are interrupted within a cycle.

Participants who discontinue dexamethasone may continue to receive the rest of study treatment.

6.6.2.5. Management of Dexamethasone-related Toxicity

Table 33 summarizes guidelines for dexamethasone-related toxicities.

Table 33: Treatment Guidelines for Dexamethasone-related AEs

Symptom	**Findings**	**Recommended Action**
Cardiovascular	Edema Grade ≥3 (anasarca or limiting function and unresponsive to therapy)	Diuretics as needed, and restart dexamethasone at one dose decrement; if edema persists despite above measures, decrease dose by another dose decrement. Discontinue dexamethasone permanently if symptoms persist despite second reduction.
Gastrointestinal	Dyspepsia, gastric or duodenal ulcer, or gastritis Grade 1 or 2 (requiring medical management)	Continue dexamethasone at same dose and treat with therapeutic doses of H_2 blockers, or proton pump inhibitor. Consider adding sucralfate or other antiulcer treatment as clinically indicated. If symptoms persist despite above measures, decrease dexamethasone dose by one dose decrement (see Table 31 and Table 32).
	Dyspepsia, gastric or duodenal ulcer, or gastritis Grade ≥3 (requiring hospitalization or surgery)	Hold dexamethasone until symptoms return to baseline. Restart dexamethasone at one dose decrement along with concurrent therapy with H_2 blockers, sucralfate, or omeprazole. If symptoms persist despite above measures, discontinue dexamethasone permanently.
	Acute pancreatitis	Discontinue dexamethasone permanently.
General Disorders	Limb edema Grade ≥3 (>30% limb discrepancy in volume; gross deviation from normal anatomic contour; limiting self-care ADL)	Hold dexamethasone until symptoms return to baseline. Diuretics as needed, and restart dexamethasone at one dose decrement; if edema persists despite above measures, decrease dose by another dose decrement. Discontinue dexamethasone permanently if symptoms persist despite second reduction.
Psychiatric Disorders	Confusion or mood alteration Grade ≥2 (interfering with function ± interfering with ADL)	Hold dexamethasone until symptoms return to baseline. Restart dexamethasone at one dose decrement (see Table 31 and Table 32).

Table 33: Treatment Guidelines for Dexamethasone-related AEs

Symptom	Findings	Recommended Action
		If symptoms persist despite above measures, reduce by another dose decrement. If symptoms recur, discontinue dexamethasone.
Musculoskeletal	Muscle weakness Grade ≥2 (symptomatic and interfering with function ± interfering with ADL)	Decrease dexamethasone by one dose decrement (see Table 31 and Table 32). If weakness persists, decrease dose by another dose decrement. Discontinue dexamethasone permanently if symptoms persist.
Metabolism and Nutrition Disorders	Hyperglycemia Grade ≥3 (fasting glucose >250 mg/dL)	Withhold dexamethasone, treat with insulin or other hypoglycemic agents as needed until glucose is Grade ≤2 (<250 mg/dL) then resume dexamethasone. If uncontrolled despite above measures, decrease dose by one dose decrement (see Table 31 and Table 32) until Grade ≤2 (<250 mg/dL).
All Other	Other Grade ≥3 AEs felt related to dexamethasone	Hold dexamethasone dose. Resume at one dose decrement when the AE has resolved to Grade ≤2. If the AE recurs, hold dexamethasone dose until toxicity has resolved to Grade ≤2 and resume dexamethasone dose by another dose decrement. If the AE recurs despite 2 dose decrements, discontinue dexamethasone permanently.

6.6.2.6. Elotuzumab Dose Modification

No dose reduction is allowed for elotuzumab. For guidance on the management of infusion reactions in participants treated with elotuzumab, see Section 6.1.4.6.3.

6.7. Continued Access to Study Treatment After the End of the Study

Participants benefiting from the study treatment can continue receiving study drugs after the end of data collection until the study drugs are commercially available and reimbursable, available from another source (eg, via a dedicated long-term extension study), or until study completion, whichever comes first.

Investigators may recontact the participant to obtain long-term follow-up information regarding the participant's safety or survival status, including endpoint status, as noted in the ICF (refer to Informed Consent in Appendix 2).

6.8. Treatment of Overdose

There are no human or animal data regarding overdose of talquetamab. The highest dose of talquetamab administered in clinical studies to date is 1600 µg/kg every 4 weeks. In case of overdose, best supportive care should be administered.

The highest treatment dose of teclistamab administered in clinical studies to date is 6 mg/kg with various dosing schedules (including monotherapy and combination studies) and 600 mg monotherapy. For any overdose involving study treatment, best supportive care measures should be administered per institutional guidelines and in ongoing consultation with the sponsor.

In the event of an overdose of talquetamab or teclistamab, the investigator or treating physician should:

- Contact the Medical Monitor immediately.
- Closely monitor the participant for AEs/SAEs and laboratory abnormalities for ≥3 weeks.
- Obtain a plasma sample for PK analysis if requested by the sponsor (determined on a case-by-case basis).
- Document the quantity of the excess dose as well as the duration of the overdosing in the eCRF.

Overdoses of the other components of study treatment should be managed in accordance with the applicable prescribing information.

Decisions regarding dosing interruptions or dose reductions will be made by the investigator in consultation with the medical monitor based on the clinical evaluation of the participant.

6.9. Prior and Concomitant Therapy

Throughout the study, investigators may prescribe any concomitant medications or treatments deemed necessary to provide adequate supportive care except for those listed in Section 6.9.3. All medications (including prescriptions and over-the-counter products, and transfusions of blood products) different from the study drugs must be recorded in the appropriate section of the eCRF throughout the study beginning with the signing of the ICF until 30 days after the last dose of study drug(s), or until the start of subsequent systemic antimyeloma treatment, if earlier. This includes any concomitant therapies and any medications used to treat or support AEs, SAEs, or conditions reported as medical history. Recorded information in the eCRF will include a description of the type of the drug, dosing regimen, route of administration, duration of treatment, and its indication.

Modification of an effective pre-existing therapy should not be made for the explicit purpose of entering a participant into the study.

6.9.1. Permitted Therapies

Participants are to receive full supportive care during the study. The following are examples of supportive therapies that may be used:

- Standard supportive care therapies (drugs affecting bone structure and mineralization [see guidance in Section 6.9.2.2], antiemetics; antidiarrheals; anticholinergics; antispasmodics; antipyretics; antihistamines; analgesics; antibiotics and other antimicrobials; H_2 antagonists or proton pump inhibitors; and other medications intended to treat symptoms or signs of disease) as clinically indicated, per institutional guidelines, and as deemed necessary by the investigator.
- Growth factor support, erythropoietin-stimulating agents, and transfusions are permitted to treat symptoms or signs neutropenia, anemia, or thrombocytopenia according to local standards of care. Growth factor support should be avoided during the step-up dosing period and through and the first treatment dose of talquetamab and teclistamab and also during events

of CRS. Also see Section 6.9.3 regarding monitoring for potential thromboembolic events with concomitant use of erythropoietin-stimulating agents with pomalidomide. G-CSF should be considered for Grade 3 neutropenia with infection or fever or any Grade 4 neutropenia. However, the use of growth factors, particularly G-CSF and GM-CSF, should be avoided during CRS (Section 6.1.4.1.1).

- Administration of Ig replacement for hypogammaglobulinemia with IV/SCIG per EMN recommendations (Ludwig 2023) should be followed for participants with hypogammaglobulinemia (IgG <400 mg/dL) (for infection prophylaxis, see Section 6.1.4.4.1). Due to the potential for reactions to immunoglobulin replacement, it is recommended to avoid administration for a minimum of 48 hours after each step-up dose and after the first treatment dose of talquetamab and/or teclistamab, as well as during any event of CRS.
- Prophylaxis for TLS as described in Section 6.1.4.7.
- Documented infectious complications should be treated with PO or IV antibiotics or other anti-infective agents as considered appropriate by the treating investigator for a given infectious condition per institutional guidelines.
- Corticosteroids used as pretreatment medication and as part of the treatment regimen in Arms A and C are permitted as described in the relevant SoA (Section 1.3).
- Best supportive care to prevent or treat toxicities noted in Section 6.1.4.

6.9.2. Recommended Therapies

6.9.2.1. Infection Prophylaxis

See infection prophylaxis guidelines in Section 6.1.4.4.1.

6.9.2.2. Drugs Affecting Bone Structure and Mineralization

Drugs affecting bone structure and mineralization like bisphosphonate therapy or mAbs are strongly recommended for all participants with evidence of lytic destruction of bone or with osteopenia. Therapy is recommended to be continued per treatment guidelines (Dimopoulos 2021). Commercially available IV bisphosphonates (pamidronate and zoledronic acid) are preferred when available, and should be used according to the manufacturer's recommendations, as described in the prescribing information, for participants with osteolytic or osteopenic myeloma associated bone disease. PO bisphosphonates may be used as alternatives if IV bisphosphonates are not available at the study site. It is preferred that investigators use the same route of bisphosphonate therapy for all participants at their sites.

Participants who are using bisphosphonate therapy when they enter the study should continue the same treatment. Participants with evidence of lytic destruction of bone or with osteopenia who are not using a bisphosphonate at the time of randomization should start a bisphosphonate as soon as possible during the first 6 to 8 weeks of study treatment. Investigators should not start bisphosphonate therapy during the study, unless it has been agreed with the sponsor that there is no sign of disease progression.

6.9.2.3. Prevention of Steroid-induced Gastritis

Dexamethasone and other corticosteroids may induce gastritis. Medications to prevent gastritis are permitted per institutional standards (eg, proton pump inhibitors [omeprazole or equivalent], sucralfate, or H_2 blockers [famotidine or equivalent]).

6.9.2.4. Prevention of Deep Vein Thrombosis and Pulmonary Embolism

IMiDs have been associated with increased incidence of deep vein thrombosis and pulmonary embolism (Delforge 2017). Therefore, it is strongly recommended that all participants receiving IMiDs or IMiD-based combinations be treated with prophylactic anticoagulation (see Appendix 20). Risk assessment for anticoagulation and choice of drug should be guided by international guidelines and institutional practice (Palumbo 2008; Farge 2022). Vitamin K antagonists or direct thrombin inhibitors are another option for deep vein thrombosis/pulmonary embolism prophylaxis and treatment; these may be used at the discretion of the investigator. However, the use of warfarin or other vitamin K antagonists during Cycle 1 is prohibited unless no other therapeutic option is available (see Section 6.9.3.1).

6.9.3. Prohibited and Restricted Therapies

The medications in the list below are prohibited during the study. The sponsor must be notified in advance (or as soon as possible thereafter) of any instances in which prohibited therapies are administered.

- Any chemotherapy, anticancer immunotherapy (other than drugs under this study), experimental therapy, or radiotherapy (except as noted below).
- Radiotherapy is generally prohibited for all participants during the Treatment Phase. However, emergency radiotherapy may be permitted under the following conditions:
  - Must consult with and receive approval from the sponsor prior to initiation of radiotherapy (eg, whether study treatment is concurrent, delayed, or discontinued)
  - Measurable soft-tissue plasmacytomas at baseline must not be in the radiation field.
  - Absence of disease progression
  - Localized radiotherapy is permitted only for pain control or for stabilization of an extensive bone lesion at high risk of pathologic fracture or damage to surrounding tissues
  - Radiation must not occur beyond the end of Cycle 2 for any participants. For Arm A and Arm B containing talquetamab, radiation should be avoided until after the period of high risk of CRS (eg, step-up doses and first treatment dose).
  - Approval from the sponsor is needed prior to continuation of systemic study treatment
- Continuation of the study treatment after emergency orthopedic surgery is allowed only in the absence of disease progression and after consultation with and approval by the sponsor.
- Medications used for other indications that have antimyeloma properties (eg, interferon and clarithromycin; Niesvizky 2008; Rossi 2013).

  - As an exception, clarithromycin that is prescribed for a course of ≤14 days to treat an infection for which there is no therapeutic alternative is allowed.

- Systemic corticosteroids should be avoided other than for management of AEs and pretreatment medication and as part of the treatment regimen in Arms A and C as specified in the relevant SoA.
  - Corticosteroids may be administered for the management of AEs if there is not another therapeutic option; the dose regimen is not to exceed a cumulative dose of 140 mg of prednisone or equivalent within 14 days.
- Other immunosuppressant agents should be avoided unless used as pretreatment medications or medication to treat an AE (eg, CRS, ICANS) or unless approved by sponsor.
- Nonsteroidal anti-inflammatory agents should be avoided to minimize the risk of exacerbation of potential subclinical myeloma-related kidney disease.
- The use of IV contrast infusions should be avoided to prevent myeloma-related kidney disease. If administration of IV contrast is necessary, then adequate precautions including hydration are indicated.
- Routine transfusions should not be given on step-up and first treatment dose days.
- The use of transdermal patches at the injection site should be avoided.
- Vaccination with live, attenuated vaccine is prohibited during treatment and for 30 days after the last dose of study treatment unless first discussed with the sponsor. Annual influenza or SARS-CoV-2 vaccines approved or authorized for emergency use are allowed. Discuss with sponsor if plan to administer other inactivated vaccines. Note that antibody responses to vaccines may be suboptimal during study treatment (Ariza-Heredia and Chemaly 2015).

6.9.3.1. Prohibited and Restricted Therapies Specific to Talquetamab and Teclistamab

The following are prohibited and restricted therapies specific to talquetamab/teclistamab:

- CYP450 substrates with narrow therapeutic index should be used with caution from the start of step-up dosing and up to 9 days (talquetamab) or 7 days (teclistamab) after the first treatment dose, as well as during any event of CRS.
- Use of warfarin or other vitamin K antagonists during Cycle 1 is prohibited unless no other therapeutic option is available. For participants who cannot switch to a different anticoagulant and who experience CRS, coagulation parameters should be monitored closely during a CRS event and until CRS symptoms resolve.
- Live attenuated vaccines are not permitted while on treatment and must only be administered ≥30 days after last dose of talquetamab/teclistamab (see Appendix 7).

6.9.3.2. Prohibited and Restricted Therapies Specific to Pomalidomide

- Concomitant use of erythropoietin-stimulating agents with pomalidomide may increase the risk of thrombosis. If required concomitantly during treatment with pomalidomide, these drugs should be used with caution and the participant should be monitored for potential thromboembolic events.

- Concomitant use of estrogen-containing therapies with pomalidomide can increase the risk of thrombosis. Combined PO contraception is not recommended concomitantly during treatment with pomalidomide. Participants of childbearing potential who are using combined PO contraception should switch to an alternative method of highly effective contraception. The risk of venous thromboembolism continues for 4 to 6 weeks after discontinuing combined PO contraception.
- Concomitant use of strong CYP1A2 inhibitors (eg, ciprofloxacin, enoxacin, and fluvoxamine) should be avoided due to inclusion of pomalidomide as part of study treatment. Alternative treatments should be considered. If concomitant use of a strong CYP1A2 inhibitor is unavoidable, the protocol-specified pomalidomide dose should be reduced by 50%.
- Use of CYP1A2 inducers (including cigarette smoking) may reduce pomalidomide exposure. Participants should be advised that these drugs may reduce the efficacy of pomalidomide.

6.9.3.3. Prohibited and Restricted Therapies Specific to Bortezomib (Participants Treated with PVd in Arm C)

Concomitant administration of strong CYP3A4 inducers is prohibited with the use of bortezomib. Administration of strong CYP3A4 inhibitors (eg, ketoconazole, ritonavir) should be avoided and is not recommended in participants receiving bortezomib. If a strong CYP3A4 inhibitor must be given in combination with bortezomib, monitor participants for signs of bortezomib toxicity and consider a bortezomib dose reduction. For an ongoing list of CYP3A inhibitors and inducers, see http://medicine.iupui.edu/flockhart/.

6.10. SST

For participants who discontinue all study treatment for reasons other than PD, it is recommended to document disease progression as per IMWG criteria before starting SST. If an investigator deems it is in the best interest of a participant to start subsequent antimyeloma therapy in the absence of disease progression, the investigator is asked to inform the Sponsor of this decision prior to the start of the desired therapy.

After confirmation of disease progression, subsequent therapy is left to the investigator's discretion. All lines of SST should be recorded in the eCRF.

7. DISCONTINUATION OF STUDY TREATMENT AND PARTICIPANT DISCONTINUATION/WITHDRAWAL

7.1. Discontinuation of Study Treatment

Participants who discontinue treatment with any 1 or more component(s) of study treatment (talquetamab, pomalidomide, teclistamab, elotuzumab, bortezomib, and dexamethasone, depending on treatment arm) may continue to receive treatment with the other components of study treatment, as assigned.

7.1.1. Discontinuation of All Study Treatment

All study treatment must be discontinued for any of the following:

- The participant has confirmed disease progression per IMWG criteria (Appendix 17).
- The participant becomes pregnant.
- The participant receives concurrent (non-protocol) systemic anticancer treatment. If a participant requires radiotherapy for an SPM, consultation with the sponsor is required.
- The participant experiences an SPM that cannot be treated by surgery alone. Participants who require radiation therapy for treatment of SPM must have study treatment discontinued unless, upon consultation with the sponsor and review of data, continuation is agreed upon.
- The participant has an intercurrent illness that prevents further administration of treatment.
- The participant refuses further treatment with the study treatment.
- Noncompliance with study drug administration or procedure requirements.
- The investigator believes that for safety reasons or tolerability reasons (eg, AE) it is in the best interest of the participant to discontinue all study treatment.

If a participant discontinues study treatment for any reason before the end of the Treatment Phase, then the participant will complete the EOT visit and enter the posttreatment Follow-up Phase (see relevant SoA, Section 1.3). Follow-up assessments will continue until the EOS. If the reason for withdrawal from the study is withdrawal of consent, then no additional assessments are allowed. The primary reason for discontinuation of study treatment is to be recorded in the eCRF. Study treatment assigned to the participant who discontinued study treatment may not be assigned to another participant. Additional participants will not be entered.

7.1.2. Discontinuation of Talquetamab SC (Arms A and B Only)

Talquetamab SC must be discontinued for any of the following:

- First event of Grade 3 CRS not resolved within ≥48 hours.
- Second event of Grade 3 CRS or any event Grade 4 CRS.
- Second event of Grade 3 ICANS or any event Grade 4 ICANS.
- Second event of Grade 3 sARR or any event Grade 4 sARR associated with administration of talquetamab SC.
- Grade ≥3 injection-site reaction associated with administration of talquetamab.
- The participant experiences a dose delay for talquetamab SC ≥36 days (talquetamab Q2W dosing schedule) or ≥43 days (talquetamab Q4W dosing schedule), unless continuation is agreed in consultation with the sponsor after review of safety and efficacy.
- Maximum duration of 26 cycles of therapy is reached.

7.1.3. Discontinuation of Teclistamab SC (Arm B Only)

Teclistamab must be discontinued for any of the following:

- First event of Grade 3 CRS not resolved within 48 hours.
- Second event of Grade 3 CRS or any event of Grade 4 CRS.
- Second event of Grade 3 ICANS or any event of Grade 4 ICANS.
- Second event of Grade 3 sARR or any event of Grade 4 sARR associated with the administration of teclistamab.
- Grade ≥3 injection-site reaction associated with administration of teclistamab.
- The participant experiences a dose delay for teclistamab SC ≥43 days (teclistamab Q4W dosing schedule), unless continuation is agreed in consultation with the sponsor after review of safety and efficacy.
- Maximum duration of 26 cycles of therapy is reached.

7.1.4. Discontinuation of Pomalidomide

Pomalidomide must be discontinued for any of the following:

- Angioedema associated with pomalidomide.
- Grade 4 rash or exfoliative or bullous rash associated with pomalidomide.
- Stevens-Johnson syndrome/toxic epidermal necrolysis associated with pomalidomide.
- If toxicities related to pomalidomide occur after dose reduction of pomalidomide to 1 mg (see Table 29).
- The participant experiences a dose delay for pomalidomide for >3 cycles (unless continuation is agreed in consultation with the sponsor after review of safety and efficacy).

Note: If pomalidomide is discontinued, bortezomib, elotuzumab, talquetamab, and dexamethasone (as applicable depending on treatment arm) can be continued.

7.1.5. Discontinuation of Elotuzumab (Arm C [EPd] Only)

Elotuzumab must be discontinued for any of the following:

- Grade 4 infusion reaction.

Note: If elotuzumab is discontinued, pomalidomide and dexamethasone can be continued.

7.1.6. Discontinuation of Bortezomib (Arm C [PVd] Only)

Bortezomib must be discontinued for any of the following:

- Grade 4 (life-threatening consequences; urgent treatment indicated) and/or severe autonomic neuropathy associated with administration of bortezomib.
- If toxicities related to bortezomib occur after dose reduction of bortezomib to 0.7 mg/m^2 (see Section 6.6.2.3).

Note: If bortezomib is discontinued, pomalidomide and dexamethasone can be continued.

7.1.7. Discontinuation of Dexamethasone (Arms A and C Only)

Dexamethasone should be discontinued if toxicities related to dexamethasone occur after 2 dose level reductions of dexamethasone (unless continuation is agreed in consultation with the sponsor after review of safety and efficacy). See Table 31 (EPd option of Arm C) and Table 32 (Arm A and PVd option of Arm C) for guidance regarding dose reduction of dexamethasone.

7.2. Participant Discontinuation/Withdrawal From the Study

A participant will be withdrawn from the study for any of the following reasons:

- lost to follow-up
- withdrawal of consent

When a participant withdraws before study completion, the reason for withdrawal is to be documented in the eCRF and in the source document. If the reason for withdrawal from the study is withdrawal of consent then no additional assessments are allowed.

Withdrawal of Consent

A participant declining to return for scheduled visits does not necessarily constitute withdrawal of consent. Alternate follow-up mechanisms that the participant agreed to when signing the consent form apply (eg, consult with family members, contacting the participant's other physicians, medical records, database searches, use of locator agencies at study completion) as local regulations permit.

7.2.1. Withdrawal From the Use of Study Samples

The participant may withdraw consent for use of study samples including optional study samples, for research not required by the protocol. In such a case, samples will be destroyed after they are no longer needed for the clinical study. Details for local requirements can be found in the respective local ICF. Details of the sample retention are presented in the ICF.

7.3. Lost to Follow-up

To reduce the chances of a participant being deemed lost to follow-up, prior to randomization attempts should be made to obtain contact information from each participant, eg, home, work, and mobile telephone numbers and email addresses for both the participant as well as appropriate family members.

A participant will be considered lost to follow-up if the participant repeatedly fails to return for scheduled visits and is unable to be contacted by the study site. A participant cannot be deemed lost to follow-up until all reasonable efforts made by the study site personnel to contact the participant are deemed futile. The following actions must be taken if a participant fails to return to the study site for a required study visit:

- The study site personnel must attempt to contact the participant to reschedule the missed visit as soon as possible, to counsel the participant on the importance of maintaining the assigned visit schedule, to ascertain whether the participant wishes to or should continue in the study.
- Before a participant is deemed lost to follow-up, the investigator or designee must make every reasonable effort to regain contact with the participant (where possible, 3 telephone calls, e-mails, fax, and, if necessary, a certified letter to the participant's last known mailing address, or local equivalent methods. Locator agencies may also be used as local regulations permit. These contact attempts should be documented in the participant's medical records.
- Should the participant continue to be unreachable, they will be considered to have withdrawn from the study.
- Site personnel, will attempt to collect the vital status of the participant within legal and ethical boundaries for all participants randomized, including those who did not get study treatment. The site may engage a third party to search public sources may be searched for vital status information. If vital status is determined as deceased, this will be documented and the participant will not be considered lost to follow-up. Sponsor personnel will not be involved in any attempts to collect vital status information.

Should a study site close, eg, for operational, financial, or other reasons, and the investigator cannot reach the participant to inform them, their contact information may be transferred to another study site.

8. STUDY ASSESSMENTS AND PROCEDURES

8.1. Administrative and General/Baseline Procedures

8.1.1. Overview

The SoAs (Section 1.3) summarize the frequency and timing of efficacy, PK, immunogenicity, biomarker, PROs, MRU, and safety measurements applicable to this study.

PRO assessments should be completed before any tests, procedures, or other consultations to prevent influencing participant responses. Refer to the PRO completion guidelines for instructions on the administration of PROs.

If multiple assessments are scheduled for the same timepoint, it is recommended that procedures be performed in the following sequence: PROs, ECGs, vital signs, and any type of blood draw last. Urine and blood collections for PK and pharmacodynamic assessments should be kept as close to the specified time as possible. Other measurements may be done earlier than specified timepoints if needed. Actual dates and times of assessments will be recorded in the source documentation and/or the eCRF.

Additional urine pregnancy tests may be performed, as determined necessary by the investigator or required by local regulation or global PPP/REMS programs for participants in Arms A and C treated with pomalidomide (see Appendix 4), to establish the absence of pregnancy at any time during the participation in the study.

MRU data will be collected. Refer to Section 8.9 for details.

8.1.1.1. Screening Phase

All participants must sign an ICF prior to the conduct of any study-related procedures. The Screening Phase begins when the first screening assessment is performed, and all screening tests should be performed within 28 days thereafter except as specified in the relevant SoA.

See Section 5 and Section 6.1.1 for details about changes to eligibility during screening (ie, before randomization) and before the start of administration of study treatment.

8.1.1.1.1. Medical History and Disease Characteristics

At Screening, the investigator or designee will collect a complete medical and surgical history, including but not limited to all ongoing relevant and clinically significant comorbidities according to NCI-CTCAE Version 5.0, including severity grade. Medical history will include information on the participants' concurrent medical conditions. Record all findings on the medical history eCRF. Cardiovascular risk factors which include family history of cardiovascular disease and smoking history will also be required.

In addition to the medical history above, multiple myeloma history must date back to the original diagnosis and include date of diagnosis and prior antimyeloma therapies. For participants who are being referred to the research site, critical referral information will constitute multiple myeloma information from source notes.

8.1.1.2. Treatment Phase

The Treatment Phase begins with the start of administration of study treatment (which must occur within 7 days of randomization) and continues until the completion of the EOT Visit. AEs (related to any of the study drugs as well as certain unrelated AEs) will be managed as described throughout the protocol, including safety monitoring as described. The Treatment Phase will extend from C1D1 until confirmed disease progression, death, intolerable toxicity, withdrawal of consent, or discontinuation of all study treatment (whichever occurs first; see discontinuation criteria in Section 7.1). Upon discontinuation of study treatment, the participant will complete an EOT Visit.

8.1.1.3. EOT Visit

The EOT Visit is required for all participants, including those discontinuing treatment for any reason except being lost to follow-up, death, or withdrawal of consent for study participation. This visit will be completed up to 30 days (+7 days) after the last dose of study treatment or before the start of a new antimyeloma therapy, whichever comes first.

If a participant is unable to return to the site for the EOT Visit, or if the EOT Visit occurs before Day 30 after the last dose of study treatment, the participant should be contacted to collect AEs that occur up to 30 days after the last dose of study treatment or until the start of a SST, whichever comes first. The primary reason for treatment discontinuation will be documented in the eCRF.

8.1.1.4. Posttreatment Follow-up Phase

The Follow-up Phase starts after the EOT Visit and will continue until death, withdrawal of consent, loss to follow-up, or end of the study, whichever occurs first. During follow-up,

participants will be evaluated for survival, subsequent antimyeloma therapies, and second primary malignancies prior to disease progression, and following disease progression as outlined in the Schedule of Activities (ie, every 16 weeks following confirmed disease progression or the EOT Visit, whichever occurs last, until the end of study, unless the subject has died, is lost to follow-up, or has withdrawn consent). If the participant has died, the date and cause of death will be collected and documented on the eCRF, if or when available. AEs will be followed for 30 days after the last dose of study treatment or until the start of SST, if earlier (see follow-up for SAEs in Section 8.4.3); see also Section 8.3.5.3 for follow-up related to the risk of HBV reactivation with study treatment and continued monitoring required for 6 months after last dose of study treatment.

If study treatment is discontinued prior to the onset of disease progression per IMWG criteria (Appendix 17), disease evaluation should continue to be performed until confirmed disease progression, death, start of SST (unless disease progression is not confirmed prior to start of such therapy), withdrawal of consent, or end of the study, whichever occurs first. Although SST should not start until after disease progression, if PD is not confirmed before the start of a new antimyeloma regimen, disease evaluation should continue for up to 30 days after the start of SST for the purpose of confirmation of progression only as indicated in the relevant SoA.

SST and response to treatment including date of subsequent progression (PFS2) will be recorded and survival status will be obtained (see Appendix 10 for definition of a line of therapy).

If the information is obtained via telephone contact, written documentation of the communication must be available for review in the source documents. Where allowed by local law, public records may be used to document death to obtain survival status.

8.1.2. Home Health Care and Telemedicine Visits

In exceptional circumstances, home health care and telemedicine visits may be implemented by or with approval from the sponsor and per the clinical judgment of the investigator, where feasible and permissible by local policy.

Participants for whom there is no safety concern may have home health care and telemedicine (conducted via phone or video conference) visits.

Study procedures such as participant reconsenting, PRO collection, ECOG assessment, AE and concomitant medication reporting, review of body systems, and collection of information on the participant's current health status may be performed with home health care and telemedicine visits. Protocol-specified laboratory assessments (Section 1.3) for efficacy and safety may be collected during home health care visits. All assessments should be followed with in-person examination, as applicable.

If local laboratories are used, it is important to ensure appropriate documentation of laboratory reference ranges. Source documentation and, if applicable, the appropriate eCRFs should be completed and detail how each assessment was collected (eg, remote versus on-site, central versus local laboratory, vital signs taken at home by delegated in-home nursing).

8.1.3. Sample Collection and Handling

The actual dates of sample collection must be recorded in the eCRF or laboratory requisition form.

Refer to the SoA for the timing and frequency of all sample collections.

Instructions for the collection, handling, storage, and shipment of samples are found in the Laboratory Manual that will be provided. Collection, handling, storage, and shipment of samples must be under the specified, and where applicable, controlled temperature conditions as indicated in the Laboratory Manual.

8.1.4. Study-Specific Materials

The investigator will be provided with the following supplies:

- study protocol and amendments if applicable
- IB for talquetamab and teclistamab
- prescribing information for elotuzumab, pomalidomide, bortezomib, and dexamethasone
- IPPI for talquetamab and teclistamab
- SIPPM
- Laboratory Manual
- IWRS manual
- eCRF completion guidelines
- ICF
- diary card for recording of daily temperature as noted in Section 6.1.3.1
- wallet card
- PRO instruments and completion guidelines
- ancillary supplies for study treatment preparation and administration as required

8.2. Efficacy Assessments

Disease evaluations must be performed by the central laboratory on Day 1 of every cycle (±7 days, window not applicable to C1D1), or every 28 days ±7 days if treatment is delayed, during the Treatment Phase for all treatment regimens, as specified in the relevant SoA. Disease evaluations must be performed when all study treatments in that arm are completed. Local laboratory assessments may be used under specified circumstances (Section 8.2.6). If both central and local assessments are performed on the same day, central laboratory results will be prioritized. Response assessments will be based on the IMWG response criteria (Appendix 17).

Responses or progression will be evaluated by investigators, use of a computerized algorithm, and by an IRC. The validated computerized algorithm is a standard program used for the majority of the multiple myeloma protocols. For data analysis and reporting, the sponsor's validated computerized algorithm has been shown to provide consistent review of the data necessary to

determine disease progression and response according to IMWG criteria. The composition, roles, and responsibilities of the IRC will be described in a separate charter. See Section 8.1.1.4 for details regarding disease evaluations during the Follow-up Phase.

The investigational sites are requested to notify the sponsor if a participant is diagnosed with disease progression (which must be confirmed with a consecutive assessment ≥1 calendar day apart if based on M-protein/serum FLC levels) and provide documentation of disease progression (eg, by completing a disease progression form or by contacting the IWRS). The sponsor's medical monitor must review the data provided to confirm that the IMWG criteria for PD have been met. If disease progression is confirmed by sponsor, then the participant will discontinue study treatment, complete the EOT Visit, and enter the Posttreatment Follow-up Phase. If disease progression has not occurred at the time of the EOT Visit, disease evaluations must continue until disease progression is confirmed. SST will not be started until after disease progression is confirmed by the sponsor.

The onset of new and/or increased size of soft tissue plasmacytoma or lytic bone lesions observed during Cycle 1 will not be considered disease progression.

8.2.1. M-protein Measurements in Serum and Urine

Blood and 24-hour urine samples for M-protein measurements will be sent to and analyzed by a central laboratory. If results from both central and local laboratory for the same date are obtained, the former values should be used for the assessment of response.

Only one serum and one 24-hour urine sample per timepoint are required to perform the following tests:

- serum β2-microglobulin
- serum quantitative Ig
- SPEP
- serum FLC assay (note that timing differs by whether disease is measurable by light chain or degree of response)
- SIFE/UIFE
- 24-hour UPEP

Blood and 24-hour urine samples will be collected as specified in the relevant SoA until the development of confirmed disease progression (see Section 8.1.1.4 for guidance regarding disease evaluations in the event that SST is begun prior to confirmation of disease progression). Disease progression based on one of the laboratory tests alone must be confirmed by ≥1 repeat investigation. Disease evaluations will continue beyond relapse from CR until disease progression is confirmed.

8.2.1.1. Potential for Elotuzumab Interference with Disease Evaluations

Elotuzumab is a human IgG1κ mAb that may be detected on SPEP and SIFE assays used for monitoring disease monoclonal Ig (M-protein). This can lead to false positive SPEP and SIFE assay results for participants with IgG kappa myeloma protein and affect assessments of responses.

Therefore, an elotuzumab-specific immunofixation electrophoresis will be performed when elotuzumab interference is suspected based on SPEP and SIFE results. The elotuzumab-specific immunofixation electrophoresis will be performed at a central laboratory to confirm a VGPR or better in participants with IgGκ myeloma when elotuzumab interference is suspected based on SPEP and SIFE results.

8.2.2. Bone Marrow Examination

Bone marrow aspirate will be performed for clinical assessments and biomarker evaluations (see relevant SoA and Table 34). Clinical staging (morphology, IHC, immunofluorescence, or flow cytometry) should be done by a local laboratory; cytogenetics will be done at a central laboratory. Bone marrow aspirate will also be performed to confirm CR and sCR, as well as at PD.

For biomarker evaluations, including FISH, MRD, immunophenotyping, and CyTOF, bone marrow samples will be evaluated at a central laboratory.

The order in which bone marrow aspirate samples are collected is important. Samples for MRD evaluation should be collected from the first pull of bone marrow aspirate. Additional information will be provided in the laboratory manual.

See Section 8.2.3 for additional information on obtaining baseline material for MRD testing.

Table 34: Bone Marrow Testing

Timepoint	Notes	Local Testing	Central Testing
Screening	• IHC/IF (both require kappa/lambda ratio from analysis of >100 PCs) or 2- to 4-color flow cytometry are acceptable methods to evaluate PC clonality. • Disease characterization (morphology and either IHC or flow cytometry performed locally): if a biopsy was done within 42 days before randomization, no need to repeat for morphology; however, fresh aspirate samples need to be obtained and sent to central laboratory for MRD, cytogenetics, immunophenotype[c] and molecular markers.[d]	Evaluate BMPC% Evaluate clonality of BMPCs by flow cytometry or IHC/IF	FISH analysis[a] MRD[b] Immunophenotyping[c], CyTOF, and molecular markers[d]
At time of suspected CR/sCR	• If these timepoints occur within 1 month of another bone marrow aspiration, a repeat bone marrow aspiration will not be requested. • IHC/IF (both require kappa/lambda ratio from analysis of >100 PCs) or 2- to 4-color flow cytometry are acceptable methods to evaluate PC clonality.	Evaluate BMPC% in the bone marrow to confirm CR Evaluate clonality of BMPCs by flow cytometry or IHC/IF in the bone marrow to confirm sCR	MRD[b]
For participants with confirmed CR or sCR, additional samples will be collected 12 months post C1D1 (±3 months) and 12 months (±3 months) after time of confirmed CR or sCR, then every 12 months (±3 months) until PD.		If sCR criteria are not already met, repeat evaluation of clonal BMPCs (ie, flow cytometry or IHC/IF) with subsequent bone marrow testing	MRD[b]
Disease progression		Evaluate BMPC% Evaluate clonality of BMPCs by flow cytometry or IHC/IF	Immunophenotyping[c], CyTOF, and molecular markers[d]

NOTE: Samples collected outside of the specified window will be retained for potential analysis.
a. If central FISH analysis fails, results of local FISH analysis can be provided, if available.
b. For participants enrolling in China, MRD will be tested by NGF or NGS at central laboratory.
c. Immunophenotyping (flow cytometry) testing will be performed in approximately 50% of the participants. For Arm C (EPd/PVd), only screening samples will be tested for immunophenotyping.
d. CyTOF/molecular marker assessments will not be performed in China.

8.2.3. MRD Evaluations

Collection of bone marrow aspirates is required to monitor MRD. Samples will be collected as specified in the relevant SoA and Table 34. Only participants with a confirmed CR or sCR will be evaluated for MRD negativity.

Samples for MRD evaluation should be collected from the first pull of bone marrow aspirate. Additional information will be provided in the laboratory manual.

MRD will be evaluated using an NGS assay (clonoSEQ® assay, Adaptive Biotechnologies™) with a sensitivity of 10^{-5} at a central laboratory. For participants enrolling in China, MRD will be evaluated by an NGS or NGF assay (EuroFlow™ assay, Cytognos S.L.) with a sensitivity of 10^{-5} at a central laboratory. Bone marrow aspirates should be sent to the central laboratory for MRD evaluation.

For MRD NGS evaluations, in the event that fresh bone marrow aspirate sample is not sufficient or evaluable at screening, or does not yield a usable clone ID, any one of the following archived diagnostic bone marrow samples (eg, collected prior to randomization at a time of bone marrow disease) may be used:

- Frozen cell pellets collected from at least 1 million (ideally 5 million) viable bone marrow nuclear cells.
- Frozen bone marrow aspirate (at least 1 mL) collected in an ethylenediaminetetraacetic acid tube and stored at a temperature of -80°C.
- Bone marrow aspirate slides (smear or touch prep): 5 to 10 slides. Unstained slides without cover slips preferred.
- FFPE clot block: 5 to 10 slides of 5 to 8 µm thickness, non-decalcified. Slides without coverslips preferred.
- FFPE scrolls: 5 to 10 scrolls cut from block, 5 to 8 µm thickness, non-decalcified. Please note: decalcified bone marrow core biopsies will not be accepted.
- FFPE slides: 5 to 10 slides (FFPE tissue cut from block, 5 to 8 µm thickness). Unstained slides without cover slips preferred.
- Genomic DNA ≥1 µg (minimum concentration of 15 ng/µL). DNA isolated from fresh bone marrow aspirate preferred.
- Alternatively, if MRD calibration has previously been determined locally using the NGS clonoSEQ® assay (Adaptive Biotechnologies™, Seattle WA), this deidentified data may be provided for the screening MRD assessment.
- Other: as approved by the sponsor.

If after all efforts a calibration clone for MRD assessment cannot be identified based on central or local material, the sponsor will notify the investigator and the participant will not need to continue with bone marrow aspirate collections specifically aimed at MRD determination. Also, if a bone marrow aspirate collected during treatment to evaluate MRD-negativity is deemed of low quality and/or yields no results, another fresh bone marrow aspirate may be requested.

8.2.4. Imaging for Disease Evaluation

Assessment of bone lesions and plasmacytomas must be performed at screening and thereafter as clinically indicated. Imaging will be interpreted locally. Any of the below imaging methods are acceptable for the specific conditions, as indicated:

Imaging Modality	Bone Lesions	Extramedullary Plasmacytoma
Skeletal survey	Yes	No
Low-dose whole-body CT	Yes	No
Whole-body DWI-MRI	Yes	Yes
PET/CT with diagnostic CT component	Yes	Yes

Assessment of bone lesions at screening must include imaging of skull, entire vertebral column, pelvis, chest, humeri, femora, and any other anatomical regions for which the investigator suspects involvement by disease.

Note that if a skeletal survey is used as the imaging modality to assess bone lesions, a whole-body physical exam must also be performed to assess soft-tissue plasmacytomas. The modality used for screening should be maintained for any subsequent imaging procedures and, if indicated, to evaluate for possible disease progression. If imaging was performed within 42 days before randomization, it does not need to be repeated at screening.

During the Treatment Phase, imaging should be performed locally, whenever clinically indicated based on symptoms, to document response or disease progression. If a participant presents with disease progression manifested by symptoms of pain due to bone changes, then disease progression may be documented by skeletal survey or other radiographs, depending on the symptoms that the participant experiences. If the diagnosis of disease progression is obvious by radiographic investigations, then no repeat confirmatory imaging is necessary. If changes are equivocal, then a repeat X-ray is needed in 1 to 3 weeks.

The modality used for imaging must be indicated on the eCRF. If cross-sectional imaging was obtained, the report must be made available to sponsor for review upon request.

8.2.5. Documentation of Soft-Tissue Plasmacytomas

There are 2 types of soft-tissue plasmacytomas in multiple myeloma: 1) extramedullary plasmacytomas from metastatic or hematogenous spread involving only soft-tissues and 2) paraskeletal plasmacytomas with tumor masses arising from skeletal lesions (Rosiñol 2021). Instructions in this section apply to both extramedullary and paraskeletal soft-tissue plasmacytomas.

Extramedullary plasmacytomas must be assessed by PET/CT (with diagnostic CT component). Whole body DWI-MRI is an acceptable alternative if the plasmacytoma is better characterized by this imaging technique in specific participants. All whole-body imaging should include the vertex of the skull and the knees. If institutional protocols are available, the lower extremities should be shown in full. A PET scan (without CT), low-dose whole-body CT scan, or ultrasound are not acceptable methods. At the investigator discretion, organ focused MRI can be performed to follow-up on plasmacytomas.

For as long as a plasmacytoma remains measurable on physical examination, it will be evaluated (presence and size) Q28d (±7 days) from C1D1 and continue until development of confirmed CR or confirmed disease progression or start of SST. For plasmacytomas only detectable by radiologic imaging, then the evaluation of such plasmacytomas should be performed Q12W (±7 days) from C1D1 until confirmed CR or confirmed disease progression or start of SST. The methodology used for evaluation of each plasmacytoma should be consistent across all visits. Irradiated or excised lesions will be considered "not measurable" and will be monitored only for disease progression.

If biopsy of a soft-tissue plasmacytoma is performed for any reason, a sample should be sent to the central laboratory. See Appendix 17 for additional information regarding soft-tissue plasmacytomas and disease response.

8.2.6. Local Laboratory Assessments

All efforts should be made to collect efficacy data centrally. However, local laboratory data may be collected if central laboratory data are not available at a particular timepoint. See Section 5.1 for details of disease assessment using local laboratory at screening. Documentation of the local laboratory data should be sent to the principal investigator and filed in the medical record. It is the principal investigator's responsibility to ascertain that these results are reviewed and entered into the participant's medical record and the eCRF.

8.2.7. PROs

PRO data will be captured to assess the changes in symptoms, functioning, and overall HRQoL with Tal-P, Tal-Tec, EPd, and PVd treatment regimens using the following standard PRO instruments:

- MySIm-Q
- EORTC-QLQ-C30
- EQ-5D-5L
- PGI-S
- Epstein Taste Survey

During the Treatment Phase, standard PRO instruments should be completed by the participant within 2 days prior to dosing before any clinical tests, procedures, or other consultations that would influence his or her perceptions of their current health state. Additionally:

- The PRO instrument will be provided in the local language in accordance with local guidelines. If a participant requires assistance completing the PRO instruments, a study coordinator may assist by reading the questions and response options verbatim, and should not prompt the participant in selecting their response. At completion, the study coordinator should verify that the questionnaires are completed in full or document the reason for missing information. Only study coordinators that have completed the PRO completion training will be permitted to assist participants in completing the PROs.

- The PRO instrument will be available for regulators and for IRB/IEC submissions and will be provided separately in a companion manual with the instruments that will be submitted with the protocol.
- PRO and AE data will not be reconciled with one another.

During the Follow-up Phase, standard PROs (except PGI-S and Epstein Taste Survey) should be collected Q16W (±14 days) after the initial indication of PD or the EOT Visit, whichever occurs first. PRO instruments should continue to be administered even if SST starts. For PRO assessments conducted beyond disease progression or SST, if no site visits are scheduled for additional disease evaluations, participants may complete the PRO assessments via telephone.

Additionally, participants from selected countries will have the option of participating in a single qualitative interview, as described in Section 8.2.7.6.

8.2.7.1. MySIm-Q

The MySIm-Q is a disease-specific PRO assessment with established content validity for patients with relapsed and refractory multiple myeloma and newly diagnosed multiple myeloma. It is included to be complementary to the EORTC-QLQ-C30. It includes 17 items resulting in a symptom subscale and an impact subscale. The recall period is the "past 7 days", and responses are reported on a 5-point verbal rating scale.

8.2.7.2. EORTC-QLQ-C30

The EORTC-QLQ-C30 Version 3 is a self-administered questionnaire that measures the HRQoL of participants with cancer. It includes 30 items that make up 5 functional scales (physical, role, emotional, cognitive, and social), one global health status scale, 3 symptom scales (pain, fatigue, and nausea/vomiting), and 5 single symptom items (dyspnea, insomnia, appetite loss, constipation, and diarrhea) and a single impact item (financial difficulties). The recall period is 7 days ("past week") for most items, and responses are reported using a verbal and numeric rating scales. The item and scale scores are transformed to a 0 to 100 scale. A high score for a functional scale represents a high/healthy level of functioning and a high score for the global health status represents high HRQoL, but a high score for a symptom scale/item represents a high level of symptomatology/problems. The EORTC-QLQ-C30 has been widely used in patients with multiple myeloma. Reliability, validity, and clinically meaningful change has been demonstrated for participants with multiple myeloma (Wisløff 1996; Wisløff 1997).

8.2.7.3. EQ-5D-5L

The EQ-5D-5L is a generic measure of health status. For purposes of this study, the EQ-5D-5L will be used to generate utility scores for use in cost-effectiveness analyses. The EQ-5D-5L is a 5-item questionnaire that assesses 5 domains including mobility, self-care, usual activities, pain/discomfort and anxiety/depression. The EQ-5D-5L uses a 5-point Likert response scale ranging from "No problems" to "Extreme problems". The EQ-5D health states defined by the EQ-5D descriptive system can be converted into a single index value using country-specific value sets ranging from zero (0.0) to 1 (1.0) representing the general health status of the individual (values less than <0 are possible when using the UK scoring algorithm). The index value facilitates

the calculation of quality-adjusted life years that are used to inform economic evaluations of health care interventions. The EQ-5D-5L also includes a visual analog scale rating "health today" with anchors ranging from 0 (worst imaginable health state) to 100 (best imaginable health state).

8.2.7.4. Patient Global Impression – Severity (PGI-S)

Interpreting meaningful change in scores on PRO instruments is an important step in evaluating results. The methods for interpreting meaningful change have evolved over time and various approaches exist. Anchor-based methods are often preferred. Anchor-based methods link scores on the PRO to an external criterion that identifies participants who have experienced an important change in their condition. The PGI-S will be used as an anchor, external criterion, to determine meaningful change in scores for the MySIm-Q and EORTC-QLQ-C30 in this population. The PGI-S contains 2 questions on how the participant would currently rate severity of symptoms and impacts with a 7-day recall period. The response options are presented as a 5-point verbal rating scale from "none" to "very severe."

8.2.7.5. Epstein Taste Survey

The Epstein Taste Survey consists of 17 items from the full 71 item PRO instrument, specific to taste changes. Developed for use in patients with head and neck cancer as a composite of the Vanderbilt Head and Neck Symptom Survey, the concepts included in the taste items measure treatment-related symptoms associated with talquetamab (Ridner 2018, Epstein 2020).

8.2.7.6. Qualitative Interview

Participants from selected countries will have the option of participating in a single interview to be conducted approximately 12 months (±3 months) after start of study treatment. The interviews are estimated to last approximately 45 to 60 minutes and will be audio recorded, transcribed, and analyzed.

Post-baseline interviews have been used in clinical trials to inform exploratory insight into participant experiences. These interviews will be used to understand the participant experience with drug regimens included in the study, not limited to treatment benefits, meaningful change in symptoms and functions that are important to patients, burden and management of side effects, and tolerability. The interviews are semi-structured and are designed to allow participants to provide their own impressions of the study treatment and not be restricted to items included in standard questionnaires. Interviewers will be trained on the interview guide and provided with note-taking pages to capture participant responses. Interviews will take place in the local language and will be audio recorded with approval of the participant for transcription and content analysis. The content analysis will present the qualitative themes that emerge from the narrative data.

The data from these interviews will not be reconciled with the safety database of the study. Evaluation of potential AEs will be conducted at the time of the interview. The trained interviewer, from the vendor, will record any potential AEs per the pharmacovigilance process and report them to the site within 24 hours of the completed interview. Sites will then evaluate the reported event(s) for AEs/SAEs based on procedures described in Section 8.4.

8.3. Safety Assessments

Details regarding the IDMC are provided in Committees Structure in Appendix 2.

AEs will be reported and followed by the investigator as specified in Section 8.4 and Appendix 3.

Any clinically relevant changes occurring during the study must be recorded on the AE section of the eCRF.

Any clinically significant abnormalities persisting at the end of the study/early withdrawal will be followed by the investigator until resolution or until a clinically stable condition is reached.

The study will include the following evaluations of safety and tolerability according to the timepoints provided in the SoA (Section 1.3).

8.3.1. Physical Examinations

A complete physical examination will be conducted at screening. Thereafter, symptom- and disease-directed physical examinations will be conducted once per cycle. Clinically significant physical examination findings identified prior to signing the ICF should be reported as part of medical history, not as AEs.

Body weight will be measured at screening and the beginning of each cycle. Height will be measured at screening only. Weight will be used to calculate the actual dose of talquetamab and teclistamab for administration as detailed in Table 4 and Table 5.

8.3.2. Vital Signs

Temperature, pulse/heart rate, respiratory rate, blood pressure, and oxygen saturation will be assessed.

Blood pressure and pulse/heart rate measurements will be assessed sitting with a completely automated device. Manual techniques will be used only if an automated device is not available.

Blood pressure and pulse/heart rate measurements should be preceded by at least 5 minutes of rest in a quiet setting without distractions (eg, television, cell phones).

Vital signs will only be reported in the eCRF during Cycle 1 (including the step-up doses) and thereafter at any repeat step-up dose(s), and any additional vital signs assessments supporting the start and end dates of an AE (eg, fever, hypertension) should be reported in the eCRF.

8.3.3. ECGs

During the collection of 12-lead ECGs, participants should be in a quiet setting without distractions (eg, television, cell phones). Participants should rest in a supine position for at least 5 minutes before ECG collection and should refrain from talking or moving arms or legs. If blood sampling or vital sign measurement is scheduled for the same timepoint as ECG recording, the procedures should be performed in the following order: ECG(s), vital signs, blood draw.

8.3.4. Clinical Safety Laboratory Assessments

Blood samples for serum chemistry and hematology (including coagulation) will be collected as noted in Appendix 1. The investigator must review the laboratory results, document this review, and record any clinically relevant changes occurring during the study in the AE section of the eCRF. The laboratory reports must be filed with the source documents.

To assess steroid-induced diabetes, hemoglobin A1c will be monitored while participants are on study treatment.

Any additional chemistry and hematologic laboratory assessment supporting the start and end dates of an AE should be reported in the eCRF.

8.3.5. Hepatitis Testing

8.3.5.1. HBV Testing at Screening

HBV serology is a required screening test. The test is not required to be repeated at screening if it was performed as part of standard of care within 3 months prior to the start of administration of study treatment. The HBV screening guide in Table 35 will be used to determine participant eligibility for the study.

Table 35: Eligibility Related to Hepatitis B Test Results

Action	Hepatitis B Test Result			
	HbsAg	anti-HBs	anti-HBc	HBV-DNA[a]
Exclude	Participants who are HbsAg positive or HBV-DNA positive are excluded from the study regardless of the status of anti-HBs and anti-HBc.			
Include	Negative	Negative	Negative	Not required
	Negative	Positive	Positive	Negative
	Negative	Negative	Positive	Negative
	Negative	Positive	Negative	Negative[b]

a. In participants with negative HbsAg test, an HBV-DNA quantification test is required in the following participants to determine eligibility:
   i. Participants who are anti-HBs positive and without history of vaccination.
   ii. Participants with positive anti-HBc and either positive or negative anti-HBs.

b. Participants with serologic findings suggestive of HBV vaccination (ie, anti-HBs positivity as the only serologic marker) and a known history of prior HBV vaccination do not need to be tested for HBV-DNA by PCR.

8.3.5.2. HCV Testing at Screening

HCV antibody is a required screening test. If HCV antibody testing was performed as part of standard of care within 3 months prior to C1D1 repeat testing is not required at screening. Participants with a history of HCV antibody positivity must undergo HCV-RNA testing.

8.3.5.3. Ongoing HBV Testing and HCV Testing

During and following study treatment, participants who have history of HBV infection (eg, antihepatitis B core-positive irrespective of anti-hepatitis B surface status, antihepatitis B surfacepositive and unknown HBV vaccination history, or known history of prior HBV infection

irrespective of hepatitis B serology findings) will be closely monitored for clinical and laboratory signs (including DNA PCR) of reactivation of HBV, by PCR testing, Q12W(±4 weeks) up to 6 months after the last dose of study treatment and as clinically indicated. Testing at Cycle 1 Day 1 is only required if at screening a sample was used from prior to screening. Where required by local law, the results of HBV testing may be reported to the local health authorities. For HBV reactivation, see Section 6.1.4.4.4.

Participants with history of HCV antibody positivity will be monitored for HCV-RNA testing Q12W (±4 weeks) from C1D1 up to 6 months after the last dose of study treatment. For HCV reactivation, see Section 6.1.4.4.5.

8.3.6. HIV Testing

HIV testing as specified in Table 2 is a required screening test for participants with HIV or history of HIV antibody positivity. If HIV testing by PCR and CD4 count was performed as part of standard of care within 3 months prior to C1D1, repeat testing by PCR and CD4 count on C1D1 is not required.

The criteria in Section 5.2 will be used to determine participant eligibility for the study. Participants with a history of HIV antibody positivity who otherwise meet eligibility criteria must undergo HIV-RNA and CD4 count testing Q12W (±4 weeks) from C1D1 up to 6 months after the last treatment dose and as clinically indicated. For HIV reactivation see Section 6.1.4.4.5.

8.3.7. Pregnancy Testing

See relevant SoA for timing of pregnancy testing. After screening, additional serum or urine pregnancy tests may be performed, as determined necessary by the investigator or required by local regulation, to establish the absence of pregnancy at any time during the participation in the study.

Investigators should follow the local label for guidance on participant education/counseling and ensure that all participants receiving pomalidomide adhere to the applicable PPP. When pomalidomide is provided centrally by the sponsor, investigators should comply with the global PPP/REMS. Otherwise, the investigator should apply the local PPP/REMS program, as described in Appendix 4.

8.3.8. Neurologic Examination

For participants in Arm A (Tal-P) and Arm B (Tal-Tec) only, a neurologic examination including the ICE tool evaluation (Appendix 14) will be performed as described in Table 3 and during the Treatment Phase if ICANS is suspected. Repeat ICE evaluation at least once per day, and with any change in clinical status until fully resolved. Table 21 details the ICE score(s) associated with each severity grade of ICANS and includes additional assessments that should be performed to assess severity per the ASTCT grading scale. Refer to Section 6.1.4.2 on ICANS and other neurotoxicity.

8.3.9. ECOG Performance Status

The ECOG performance status scale will be used to grade changes in the participant's ADL (see Appendix 15). When scheduled, ECOG performance status assessments should be obtained prior to any other study procedures planned for the same day whenever possible.

8.4. Adverse Events, Serious Adverse Events, and Other Safety Reporting

Timely, accurate, and complete reporting and analysis of safety information, including AEs, serious AEs, and PQCs, from clinical studies are crucial for the protection of participants, investigators, and the sponsor, and are mandated by regulatory agencies worldwide. The sponsor has established Standard Operating Procedures in conformity with regulatory requirements worldwide to ensure appropriate reporting of safety information; all clinical studies conducted by the sponsor or its affiliates will be conducted in accordance with those procedures.

AEs will be reported by the participant (or, when appropriate, by a caregiver, surrogate, or the participant's legally designated representative [unless prohibited by local regulations]) for the duration of the study.

Further details on AEs, SAEs, and PQCs can be found in Appendix 3.

8.4.1. Time Period and Frequency for Collecting AE and SAE Information

All AEs except CRS and ICANS will be graded per NCI-CTCAE Version 5.0. CRS and ICANS will be graded based on the ASTCT guidelines (Lee 2019; see Table 20 and Appendix 18, respectively). Symptoms of CRS and ICANS will be graded according to NCI-CTCAE Version 5.0 and also reported in eCRF. See also supplementary information for grading of sARRs in Table 23.

All AEs

All AEs (with the exception of progression of multiple myeloma) and special reporting situations, whether serious or non-serious, will be reported from the time a signed and dated ICF is obtained until 30 days after the last dose of study treatment or until the start of SST, if earlier, which may include contact for follow-up of safety. Any AEs occurring more than 30 days after the last dose of study treatment considered to be related to study treatment must be reported to the sponsor. AEs will be followed by the investigator as specified in Appendix 3.

Participants with Grade 3 or higher AEs or unresolved AEs that lead to study drug(s) discontinuation will continue to be assessed until recovery to Grade ≤1 or baseline, the event is deemed irreversible, the participant discontinues the study, or a maximum of 6 months, whichever comes first.

SAEs

All SAEs occurring during the study must be reported to the appropriate sponsor contact person by study site personnel immediately but no later than 24 hours of their knowledge of the event.

SAEs, including those spontaneously reported to the investigator within 30 days after the last dose of study treatment, must be reported. The sponsor will evaluate any safety information that is spontaneously reported by an investigator beyond the time frame specified in the protocol.

Information regarding SAEs (initial and follow-up) will be transmitted to the sponsor/CRO immediately, but no later than 24 hours of their knowledge of the event using the study specific SAE Form, with the complete (eg, causality, narrative) information available in the medical records that has been already assessed by a study site physician, and transmitted via secure transmission method selected by the site from among the alternatives proposed by the SM.

8.4.2. Method of Detecting AEs and SAEs

Care will be taken not to introduce bias when detecting AEs or SAEs. Open-ended and nonleading verbal questioning of the participant is the preferred method to inquire about AE occurrence.

Solicited AEs

Solicited AEs are predefined local and systemic events for which the participant is specifically questioned.

Unsolicited AEs

Unsolicited AEs are all AEs for which the participant is not specifically questioned.

8.4.3. Follow-up of AEs and SAEs

The investigator is obligated to perform or arrange for the conduct of supplemental measurements and evaluations as medically indicated to elucidate the nature and causality of the AE or SAE as fully as possible. This may include additional laboratory tests or investigations, histopathologic examinations, or consultation with other health care professionals.

AEs and the special reporting situation of pregnancy will be followed by the investigator as specified in Appendix 3.

Anticipated events are discussed in Appendix 8.

8.4.4. Regulatory Reporting Requirements for SAEs

The sponsor assumes responsibility for appropriate reporting of the Safety Information to the regulatory authorities/IECs/IRBs in each respective country/territory, as applicable.

An anticipated event is an AE that commonly occurs in the study population independent of exposure to the drug under investigation. For the purposes of this study, anticipated events will be periodically analyzed as specified in Appendix 8.

Where a clinical trial is performed in at least one Member State of the EEA, the company will report qualifying SUSARs electronically to Eudravigilance as per Regulation (EU) No 536/2014 Article 42. SAEs are assessed for reportability and those assessed as meeting SUSAR criteria are unblinded (if applicable) and submitted electronically to Eudravigilance.

8.4.5. Pregnancy

All initial reports of pregnancy in participants or their partners through sperm of participant/from sexual intercourse must be reported to the sponsor by the study site personnel within 24 hours of their knowledge of the event using the appropriate pregnancy notification form. Abnormal pregnancy outcomes (eg, spontaneous abortion, fetal death, stillbirth, congenital anomalies, ectopic pregnancy) are considered SAEs and must be reported using the SAE report form.

Any participant who becomes pregnant during the study must discontinue further study treatment.

Follow-up information regarding the outcome of the pregnancy for participants who become pregnant, or where the pregnancy was the result of participant and their partner, and any postnatal sequelae in the infant will be required.

Investigators should follow the local label for guidance on participant education/counseling and ensure that all participants in Arm A and Arm C treated with pomalidomide adhere to the applicable PPP. When pomalidomide is provided centrally by the sponsor, investigators should comply with the global PPP/REMS. Otherwise, investigators should apply the local PPP/REMS program as described in Appendix 4.

8.4.6. Disease-related Events and Disease-related Outcomes Not Qualifying as AEs or SAEs

All events that meet the definition of an SAE will be reported as SAEs, regardless of whether they are protocol-specific assessments.

Expected progression of disease, which is part of the natural course of the disease under study, would not be considered or reported as an adverse event (or serious adverse event).

Death or hospitalization that is attributed by the investigator to progression of disease should not be considered nor reported as an adverse event (or serious adverse event).

Of note, worsening of disease (and associated hospitalization or death), determined by the investigator to be caused by the study treatment should be reported per the usual reporting requirements (refer to Adverse Event Definitions and Classifications in Appendix 10.3.1).Progression of disease and death due to disease progression should be documented on the appropriate eCRF forms (eg, the Disease Progression form and the Death form).

Signs or symptoms of disease progression that are of clinical significance, such as spinal cord compression, vena cava superior syndrome, major vessel rupture, efflux obstruction or organ failure, should be documented on the appropriate eCRF forms (eg, the Symptomatic Progression Form).

8.4.7. AESIs

In this study, Grade ≥2 CRS and Grade ≥2 ICANS are considered AESIs. Therefore, as part of standard safety monitoring activities by the sponsor, any occurrence of Grade ≥2 CRS (including symptoms of CRS) or Grade ≥2 ICANS must be followed until resolution or until there is no

further improvement. All AESIs will be reported to the sponsor within 24 hours of awareness of the event irrespective or seriousness (ie, serious and non-serious AEs). Serious AESIs will be reported via standard SAE reporting while non-serious AESIs will be reported solely via the eCRF to facilitate sponsor follow-up, if needed. For all other AEs, standard safety data collection and reporting processes will be followed.

8.5. Pharmacokinetics

Serum samples will be used to evaluate the PK of talquetamab and teclistamab (Arm A and Arm B only) and ADAs according to the SoA (Section 1.3). Additional PK/immunogenicity samples will be collected as follows:

- Suspected sARR Grade ≥2 (as soon as sARR is detected, if feasible).
- Suspected CRS or ICANS event Grade ≥2 (as soon as CRS or neurotoxicity is detected, if feasible).

On days where both PK and immunogenicity of either drug (talquetamab or teclistamab) are specified, serum aliquots from the same blood collection will be utilized. The exact dates and times of blood sampling must be recorded on the laboratory requisition form.

Serum collected for PK/immunogenicity may additionally be used to evaluate safety or efficacy aspects that address concerns arising during or after the study period. Genomic analyses will not be performed on these serum samples. Participant confidentiality will be maintained.

8.5.1. Analytical Procedures

PK

Serum samples will be analyzed to determine concentrations of talquetamab (Arms A and B) and teclistamab (Arm B) using a validated, specific, and sensitive immunoassay method by or under the supervision of the sponsor.

Immunogenicity

Refer to Section 8.8.

8.5.2. PK Parameters and Evaluations

Parameters

Sparse blood samples will be collected from all participants in Arms A and B to measure serum concentrations of talquetamab and teclistamab concentrations according to the appropriate SoA (Table 6). The PK parameters of talquetamab and teclistamab will be evaluated using a population PK approach if feasible, which may include data from other studies, and the results may be reported separately. Descriptive summaries for talquetamab and teclistamab trough concentrations at each timepoint will be presented.

PK/Pharmacodynamic Evaluations

Concentration data and pharmacodynamic data of interest may be pooled in a population PK/pharmacodynamics modeling and the results of this analysis may be reported separately.

8.6. Genetics

Sample collection and testing will comply with local regulations.

Genetic tests will include FISH and NGS in BMPCs to evaluate cytogenetic risk and MRD, respectively. Additionally, genome and gene expression profiling may be performed for exploratory studies. However, information pertaining to genetic risk for other diseases or hereditary traits will not be obtained. Thus, no conclusions around incidental findings will be provided.

8.7. Biomarkers

Sample collection and testing will comply with local regulations.

Peripheral blood and bone marrow aspirate will be collected at baseline and during treatment per the relevant SoA and Table 34. Additional biomarker samples may be collected to help understand unexplained AEs.

8.7.1. Pharmacodynamic and Exploratory Biomarkers

Biomarker studies are designed to: 1) identify differences in pharmacologic activity between study treatments, including pharmacodynamic biomarkers of antimyeloma and immune activity; 2) evaluate the effect of study treatments on MRD negativity and its relationship with clinical responses; 3) identify participant subgroups that respond differently to treatments by evaluation of prognostic and disease markers; 4) help understand relationships between treatment intervention, PK, safety, and efficacy responses; and 5) identify predictive biomarkers of response and resistance to Tal-P and Tal-Tec.

Biomarker assessments may involve: A) evaluation of markers in serum/plasma including, but not limited to, circulating markers of antimyeloma (eg, M-protein), immune (eg, immune cells) and pro-inflammatory (eg, CRP and cytokines) activity by standard immunoassay and mass spectrometry-based methods, among others; and B) evaluation of markers in whole blood and bone marrow including, but not limited to, phenotypic (eg, GPRC5D+ and BCMA+ PCs, CD3+CD8+ T cells) and molecular markers (eg, gene expression, mutations and deletions, including GPRC5D and BCMA) in tumor and immune cell populations by flow cytometry, CyTOF, and/or RNA sequencing methods, among others.

8.7.2. MRD

Bone marrow aspirates will be collected as specified in the relevant SoA and Table 34.

MRD negativity is considered as reasonably likely to predict clinical benefit for longer outcomes such as PFS and OS (Kumar 2016). The relationship between MRD negativity, pharmacologic and

clinical activity, including DOR and depth of response to talquetamab and teclistamab, will be explored.

MRD will be evaluated by an NGS assay (clonoSEQ® assay, Adaptive Biotechnologies™) with a sensitivity of 10^{-5} at a central laboratory. For participants enrolling in China, MRD will be evaluated by an NGS or NGF assay (EuroFlow™ assay, Cytognos S.L.) with a sensitivity of 10^{-5} at a central laboratory.

See Section 8.2.3 for additional information on sample requirements for MRD testing.

8.7.3. Additional Collections

If it is determined at any time before study completion that additional material is needed from a FFPE sample for the successful completion of the protocol-specified analyses, the sponsor may request that additional material be retrieved from existing samples. Also, based on emerging scientific evidence, the sponsor may request additional material from previously collected samples during or after study completion for a retrospective analysis. In this case, such analyses would be specific to research related to the study treatment(s) or diseases being investigated.

8.8. Immunogenicity Assessments

Antibodies to talquetamab and teclistamab will be evaluated in serum samples collected from all participants in Arm A and Arm B according to the SoA. Additionally, serum samples should also be collected as detailed in Section 8.5 and at the final visit from participants who discontinued study treatment or were withdrawn from the study. These samples will be tested by the sponsor or sponsor's designee.

Serum samples will be screened for antibodies binding to talquetamab and teclistamab and the titer of confirmed positive samples will be reported. Positive samples for binding antibodies will be tested for neutralizing antibodies to talquetamab and teclistamab. Immunogenicity bioanalysis may be conducted on PK samples collected at other timepoints, if deemed necessary. Other analyses may be performed to verify the stability of antibodies and to further characterize the immunogenicity of talquetamab and teclistamab.

Samples collected for immunogenicity analyses may additionally be used to evaluate safety or efficacy aspects that address concerns arising during or after the study period. Genomic analyses will not be performed on these serum samples. Participant confidentiality will be maintained.

8.9. MRU

MRU data, including the management of AEs associated with the study and medical encounters related to multiple myeloma, will be collected in the eCRF by the investigator and study site personnel for all participants from start of administration of study treatment until PD while on treatment or 30 days after the last dose of study drug(s) or at the start of SST, whichever is earlier. Protocol-mandated procedures, tests, and encounters are excluded. The data collected may be used to conduct exploratory economic analyses and will include, but is not limited to:

- Number of outpatient clinic visits.
- Duration of hospitalization (excluding protocol-mandated hospitalization) and emergency room visits.

8.10. Real-world Data Collection of Participant Medical Information During and After the Study (US Sites Only)

For participants in the US who have provided consent, medical data (electronic health records, claims, and laboratory data from other care settings) obtained after study entry may be accessed utilizing tokenization and matching procedures (ie, the generation of anonymous identifiers or "tokens" [hashed and encrypted combinations of identifying elements] to allow linking of participant data from different sources without compromising the participant's confidentiality). These data may be used for exploratory analyses to enhance understanding of progression to advanced disease stages and subsequent treatment decisions, and to explore the potential for using this method of data collection versus traditional follow-up data collection. The analyses will be described in detail in a dedicated analysis plan.

8.11. Symptomatic Progression

From 30 days prior, to until 30 days after confirmed PD (or the start of SST, whichever is earlier), progression-associated symptoms and concomitant medications will be documented in the eCRF.

9. STATISTICAL CONSIDERATIONS

Statistical analysis will be done by the sponsor or under the authority of the sponsor. A general description of the statistical methods to be used to analyze the efficacy and safety data is outlined below. Specific details will be provided in the SAP.

9.1. Statistical Hypotheses

See Section 3.

9.2. Sample Size Determination

A target of approximately 795 participants will be randomized in the study. Since the study is testing more than one hypothesis, the graphical approach will be used (see Figure 5). Only hypotheses on the first 2 rows will have a pre-assigned alpha: ORR for each comparison will be assigned a 1-sided alpha of 0.001 and PFS for each comparison will be assigned a 1-sided alpha of 0.0115.

The median PFS is expected to be approximately 11 months for the control arm (Dimopoulos 2018, Richardson 2019, Rodriguez-Otero 2023) and it is expected that the treatment with Tal-P or Tal-Tec in comparison with EPd/PVd will lead to a 33% reduction in the risk of progression or death (ie, an expected HR of 0.67), which corresponds to an increase in median PFS from approximately 11 months to approximately 16.4 months. There are 2 primary comparisons in this study: Tal-P versus EPd/PVd and Tal-Tec versus EPd/PVd. With 272 PFS events for each comparison, there is 85% power at a 1-sided alpha of 0.0125 after splitting the 1-sided alpha of 0.025 for the 2 comparisons with one interim analysis and the final analysis. The

calculation assumes a comparison of PFS with the log-rank test with 1:1:1 randomization ratio, and with one interim analysis using O'Brien-Fleming (OF) spending function to define the efficacy boundary when observing approximately 75% PFS events.

Assuming a total of 795 participants will be randomized at a uniform enrollment rate of 40 participants per month, enrollment will continue for approximately 20 months. It is estimated that the targeted 272 PFS events for each comparison (approximately 408 PFS events expected across the 3 arms) will be observed at approximately 25 months assuming an annual dropout rate of 5%.

All the pre-assigned alpha for ORR will be allocated to the ORR interim analysis, which will be conducted when the first 495 participants (approximately 62% information fraction; approximately 330 participants for each comparison) have been followed for at least 6 months or when enrollment is complete, whichever is later. Based on historical studies (Dimopoulous 2018; Richardson 2019), ORR in the control arm is expected to be approximately 65%. With approximately 330 participants for each comparison, there is 87% power for detecting an ORR difference of 20% (85% vs 65%) at a 1-sided alpha of 0.001.

Long-term survival follow-up will continue until approximately 311 deaths for each comparison are observed (approximately 467 deaths expected across the 3 arms), which will be observed at approximately 5 years, assuming an annual dropout rate of 2.5%. With approximately 311 deaths for each comparison, there is 80% power to detect an OS HR of 0.72 (30 versus 42 months; Dimopoulous 2023) with the log-rank test at 1-sided nominal alpha of 0.025. Interim analysis will be conducted at the time of PFS primary analysis and every ~12 month afterwards using Kim-DeMets spending function with parameter=2, and it is estimated that ~43%, ~68% and ~87% of OS events will be observed at the interim analyses.

9.3. Analysis Sets

For purposes of analysis, the following analysis sets are defined:

Analysis Set	Description
ITT	All participants who were randomized in the study.
ORR	The first 495 participants who were randomized in the study.
Safety	All participants who have received ≥1 dose of study treatment.
Talquetamab PK-evaluable	All participants in Arm A (Tal-P) and Arm B (Tal-Tec) who have received ≥1 dose of talquetamab and have ≥1 posttreatment PK sample.
Teclistamab PK-evaluable	All participants in Arm B (Tal-Tec) who have received ≥1 dose of teclistamab and have ≥1 posttreatment PK sample.
Talquetamab Immunogenicity-evaluable	All participants in Arm A (Tal-P) and Arm B (Tal-Tec) who have received ≥1 dose of talquetamab and have appropriate serum samples for detection of ADAs to talquetamab (≥1 sample after the start of the first dose of talquetamab).
Teclistamab Immunogenicity-evaluable	All participants in Arm B (Tal-Tec) who have received ≥1 dose of teclistamab and have appropriate serum samples for detection of ADAs to teclistamab (≥1 sample after the start of the first dose of teclistamab).

9.4. Statistical Analyses

The SAP will be finalized prior to database lock and it will include a more technical and detailed description of the statistical analyses described in this section. This section is a summary of the planned statistical analyses of the most important endpoints including primary and key secondary endpoints.

9.4.1. General Considerations

Continuous variables will be summarized using the number of observations: mean, standard deviation, coefficient of variation, median, and range as appropriate. Categorical variables will be summarized using number of observations and percentages as appropriate. For time-to-event variables, the Kaplan-Meier method will be used for descriptive summaries.

Response assessments will be based on IMWG criteria, and response to treatment will be determined by an IRC. Data determined by investigators and by computer algorithm will be used to assess the robustness of the results.

Data will be regularly monitored to ensure complete and accurate data capture. In case where missing data occurs for efficacy, safety, PK and PRO assessments, no data imputation will be applied. Sensitivity analyses may be conducted to evaluate potential impact of missing data.

9.4.2. Primary Endpoint

Definition

PFS is defined as duration from the date of randomization to either PD or death, whichever comes first. For participants who have not progressed and are alive, data will be censored at the last disease evaluation before the start of any SST.

Primary Estimand

The primary estimand, the main clinical quantity of interest to be estimated in the study, is defined by the following 5 components:

- Population: participants who have received 1 to 4 prior lines of therapy including an anti-CD38 mAb and lenalidomide.
- Variable: PFS as assessed by the IRC.
- Study treatment: Tal-P combination; Tal-Tec combination; EPd combination; PVd combination.
- Intercurrent event: study treatment switching to SST. Hypothetical strategy: censor PFS event occurred after SST, as if the participants had continued treatment as planned and had not switched to SST.
- Summary: HRs of Tal-P versus EPd/PVd and Tal-Tec versus EPd/PVd.

The primary analysis will consist of a stratified log-rank test for the comparison of the PFS distribution between the treatment arms. The determination of PD and response to study treatment

will be determined by an IRC. The Kaplan-Meier method will be used to estimate the distribution of overall PFS for each treatment arm. The treatment effect expressed as a HR and its 2-sided 95% CIs will be estimated using a stratified Cox proportional hazard model with treatment as the sole explanatory variable. Stratification factors used for the analysis include number of prior lines of therapy (1 versus ≥2), ISS stage at screening (I versus II/III), and investigator's intended choice of therapy (EPd versus PVd).

The ITT Analysis Set will be used for the primary analysis for PFS. The distribution (median and Kaplan-Meier curves) of PFS will be provided using Kaplan-Meier estimates.

9.4.3. Secondary Endpoints

The key secondary endpoints are overall response, CR or better, MRD-negative CR, and OS. Other secondary endpoints include VGPR or better, PFS2, TTNT, and PROs. The summary statistics are defined below:

ORR: defined as the proportion of participants with best overall response of PR or better.

CR or better rate: defined as the proportion of participants with best overall response of CR or better.

VGPR or better rate: defined as the proportion of participants with best overall response of VGPR or better.

PFS2: defined as time from randomization to progression on the next line of therapy or death, whichever comes first. Those who are alive and for whom a second disease progression has not been observed are censored at the last date of follow-up.

OS: defined as the time from randomization to death. If the participant is alive or the vital status is unknown, then the participant's data will be censored at the date the participant was last known to be alive.

MRD-negative CR rate: defined as the proportion of participants who achieve both CR or better and MRD negativity at a threshold of 10^{-5} at any timepoint after the date of randomization and before disease progression or start of SST.

PROs: see Section 9.4.11.

TTNT: defined as the time from randomization to the start of SST. Death due to PD without start of subsequent therapy will be considered as an event. Participants who withdrew consent to study, lost to follow-up, or died due to causes other than disease progression will be censored at the date of death or the last date known to be alive.

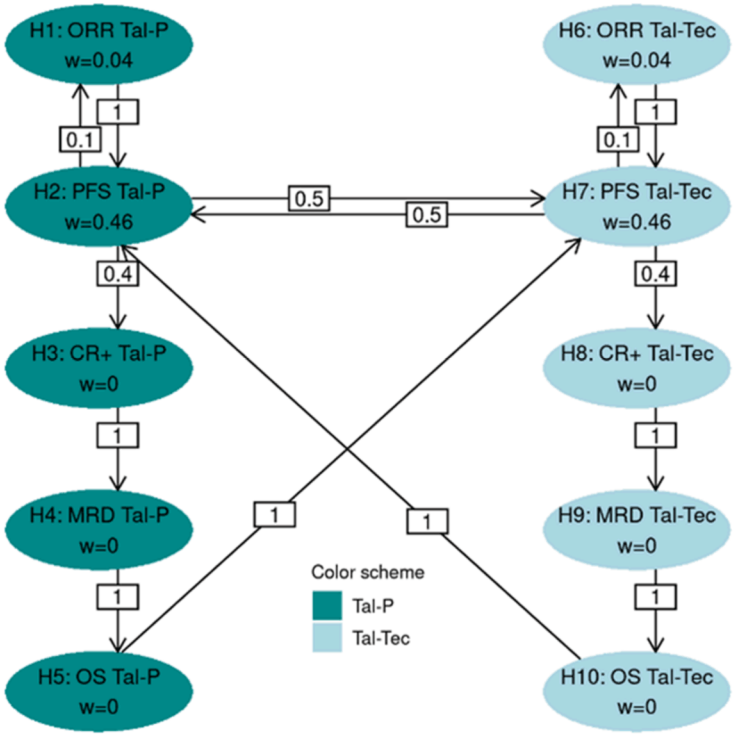
The analysis methods for binary endpoints (ie, overall response, CR or better, VGPR or better, MRD-negative CR) will be conducted using a stratified CMH test. The CMH estimate of odds ratio and its corresponding 95% CI and p-value for testing treatment difference will be reported.

For time-to-event endpoints (ie, PFS2, OS, TTNT), similar statistical methods will be applied as for PFS.

9.4.4. Multiplicity

The overall type I familywise error rate for testing the primary hypotheses and key secondary hypotheses will be controlled at a 1-sided 0.025 level using a graphical testing strategy, as presented in Figure 5. Details of the testing strategy and the key secondary endpoints will be specified in the SAP. The graphic testing procedure will assign half of the overall alpha initially to each of the treatment comparisons of Tal-P versus EPd/PVd and Tal-Tec versus EPd/PVd. If a hypothesis is rejected, the graph of the testing strategy (including the alphas for the remaining hypotheses and propagation fractions) will be updated (Maurer and Bretz 2013). For example, if PFS is rejected, the key secondary endpoints CR or better, MRD-negative CR, and OS will be tested sequentially.

Figure 5: Multiplicity Strategy



9.4.5. Exploratory Endpoints

Analysis of exploratory endpoints will be described in the SAP.

9.4.6. Safety Analyses

All safety analyses will be made on the Safety Analysis Set.

AEs

The verbatim terms used in the eCRF by investigators to identify AEs will be coded using MedDRA. Any AE occurring at or after the initial administration of study treatment through the day of last dose plus 30 days is considered to be treatment emergent. All reported TEAEs will be included in the analysis. For each AE, the percentage of participants who experience at least one occurrence of the given event will be summarized by treatment arm.

Summaries, listings, datasets, or participant narratives may be provided, as appropriate, for those participants who die, who discontinue study treatment due to an AE, or who experience a severe or a SAE.

Parameters with predefined NCI-CTCAE severity grades will be summarized. ASTCT severity grading will be utilized for CRS and ICANS (see Section 8.4.1).

Clinical Laboratory Tests

Laboratory data will be summarized by type of laboratory test. Descriptive statistics will be calculated for each laboratory analyte at baseline and for observed values and changes from baseline at each scheduled timepoint. Shift tables from baseline to worst toxicity grade during the posttreatment period will be provided for selected laboratory analytes. Worst AE grade during treatment will be presented, according to NCI-CTCAE Version 5.0.

ECG

ECG data at baseline will be summarized by treatment arm.

Vital Signs

Vital signs including temperature, pulse/heart rate, respiratory rate, and blood pressure (systolic and diastolic) (supine and standing) will be summarized over time, using descriptive statistics or graphically. The percentage of participants with values beyond clinically important limits will be summarized.

Physical Examinations

Post-baseline physical examination findings will be captured as AEs, and therefore will not be summarized separately.

9.4.7. PK Analyses

PK analyses will be performed on the PK-evaluable Analysis Sets.

All serum concentrations below the lowest quantifiable concentration or missing data will be labeled as such in the concentration data presentations or Statistical Analysis System dataset. Concentrations below the lower quantifiable concentration will be treated as zero in the summary statistics. All participants and samples excluded from the analysis will be clearly documented in the CSR.

Descriptive statistics will be used to summarize serum trough concentrations at each sampling timepoint.

If sufficient data are available, population PK analysis of serum concentration-time data might be performed using nonlinear mixed effects modeling and may include data from other clinical studies. If the population PK analysis is conducted, details will be given in a population PK analysis plan and the results of the analysis will be reported separately.

9.4.8. Pharmacodynamic and Exploratory Biomarkers Analyses

Pharmacodynamic biomarker studies are aimed to track markers of antimyeloma and immune activity. Pharmacodynamic measures will be listed, tabulated, and where appropriate, plotted. As this is an open-label study with an active-control treatment, statistical analyses will be performed to aid in the understanding of the results.

Exploratory biomarker studies are designed to identify markers predictive of response (or resistance) to Tal-P and Tal-Tec. Analyses will be stratified by clinical covariates or molecular subgroups using the appropriate statistical methods (eg, parametric or non-parametric, univariate or multivariate, analysis of variance, or survival analysis, depending on the endpoint). Correlation of baseline expression levels or changes in expression levels with response or time-to-event endpoints will identify responsive (or resistant) subgroups in addition to genes and pathways attenuated following treatment with Tal-P and Tal-Tec.

Results of pharmacodynamic and exploratory biomarker analyses may be presented in a separate report. Planned analyses are based on the availability of clinically valid assays and may be deferred if emerging study data show no likelihood of providing useful scientific information.

Stopping Analysis

Biomarker analyses are dependent upon the availability of appropriate biomarker assays and clinical response rates. Biomarker analysis may be deferred or not performed, if during or at the end of the study, it becomes clear that the analysis will not have sufficient scientific value for biomarker evaluation, or if there are not enough samples or responders to allow for adequate biomarker evaluation. In the event the study is terminated early or shows poor clinical efficacy, completion of biomarker assessments is based on justification and intended utility of the data.

9.4.9. Immunogenicity Analyses

The incidence of anti-talquetamab and anti-teclistamab antibodies will be summarized for all participants who receive ≥one dose of talquetamab and teclistamab, respectively, and have appropriate samples for detection of antibodies to talquetamab/teclistamab to (ie, participants with ≥one sample obtained after their first dose of talquetamab/teclistamab).

A listing of participants who are positive for antibodies to talquetamab/teclistamab will be provided. The maximum titers of antibodies to talquetamab/teclistamab will be summarized for participants who are positive for antibodies to talquetamab/teclistamab.

The incidence of neutralizing antibodies to talquetamab and teclistamab will be summarized for participants who are positive for antibodies to talquetamab and teclistamab, respectively.

9.4.10. PK/Pharmacodynamic Analyses

If sufficient data are available, other PK/pharmacodynamics modeling may be performed, including but not limited to, exploring the relationship between serum concentrations of talquetamab and teclistamab, pharmacodynamics, and endpoints of clinical efficacy and safety.

9.4.11. PRO Analyses

The MySIm-Q, EORTC-QLQ-C30, EQ-5D-5L utility and visual analog scale scores, PGI-S, and Epstein Taste Survey will be descriptively summarized at each timepoint. Time to worsening and time to improvement for the PRO scores will be measured as the interval from the date of randomization to the start date of meaningful change. The proportion of participants with meaningful and sustained change in HRQoL will be evaluated using established meaningful change thresholds or changes based on PGI-S. Differences between treatment arms will be assessed by change from baseline using generalized linear models. Specific details will be provided in the SAP.

Item level and total score for the Epstein Taste Survey will be summarized descriptively at each cycle, by treatment arm.

Content analysis will be performed using deidentified interview transcripts. Each interview will be coded based on a coding dictionary where dominant trends will be identified about the participant's experience and then summarized by treatment arm.

9.4.12. MRU Analyses

MRU will be descriptively summarized by treatment arm.

9.4.13. Symptomatic Progression

Time to symptomatic disease progression is defined as the time from randomization to the date of earliest new symptoms or worsening of symptoms. Symptoms are defined as those directly associated with multiple myeloma disease progression, as determined by the investigator according to the IMWG consensus guidelines, and require at least one of the following:

- Need for new multiple myeloma treatment, ie, initiation of a new systemic anti-multiple myeloma treatment such as change to an alternative therapy, and/or
- Clinical interventions to treat symptoms, ie, initiation or intensification of palliative/supportive therapies such as systemic pain medication, hospitalization, radiotherapy, bone surgery, kyphoplasty, plasmapheresis, dialysis, and transfusions (platelets and red blood cells).

9.5. Interim Analyses

The planned interim analyses are summarized in Table 36.

Table 36: Summary of Planned Interim Analyses

Analysis	Timing	Purpose
IA1	First 495 participants enrolled and followed for at least 6 months	• ORR efficacy interim
	Approximately 75% PFS events	• PFS interim • CR or better efficacy interim • MRD-negative CR efficacy interim
IA2	100% PFS events	• PFS primary • ORR primary • CR or better primary • MRD-negative CR primary • OS efficacy interim
IA3 to OS final	Approximately every 12 months from IA2	• OS efficacy interim and final

An interim analysis (IA1) is planned for ORR when the first 495 participants (approximately 330 participants for each comparison) have been followed for at least 6 months. A PFS interim analysis is planned when approximately 306 PFS events across the 3 arms (approximately 75% information fraction and 204 PFS events in 2 arms) have been accumulated. Efficacy boundary for PFS will be calculated using the OF spending function based on the actual observed number of events. The interim analysis for ORR and PFS may be conducted either jointly or separately, depending on the timing of the analysis.

If PFS is significant at IA1, CR or better will be tested based on the ITT population; if significant, MRD-negative CR will be tested based on the ITT population. For testing of CR or better and MRD-negative CR, the Bonferroni approach will be used to split the total alpha available equally at IA1 and IA2. For example, if the total 1-sided alpha recycled from PFS is 0.0046 for CR or better, a 1-sided alpha of 0.0023 will be used at IA1 and IA2.

As the expected OS information fraction at IA1 is ~30%, OS will not be tested at this time, but an administrative 1-sided alpha of 0.00005 will be allocated for OS.

A PFS primary analysis (IA2) is planned when approximately 272 PFS events are observed for each comparison.

IA2 is also the primary analysis for ORR. If PFS is significant at IA1 or IA2 and ORR is not significant at IA1, ORR will be tested again based on the ITT population at IA2. With alpha recycling from PFS, the actual alpha for testing ORR at IA2 will be re-calculated based on total alpha available for ORR.

If PFS is significant at IA2, CR or better and MRD-negative CR will be tested with a similar approach as at IA1.

At IA2, it is estimated that approximately 43% of planned total number of OS events will be accumulated. Additional OS interim analyses are planned approximately every 12 months after the PFS primary analysis. At each OS interim analysis, efficacy boundary will be calculated based on the actual observed number of OS events using the Kim-DeMets spending function with parameter=2. The final OS analysis will be performed when approximately 311 deaths have been observed for each comparison (see Section 9.2).

Since there are 2 comparisons (each treatment arm compared with control) in the study, the number of events observed for the 2 comparisons may be different at any given analysis. Therefore, for both PFS and OS, if the total number of events for each comparison is different from the expected number of events, the boundaries for PFS primary and OS final analyses will be adjusted according to the actual alpha spent at the interim analysis and actual number of events observed at the interim analyses and the final analysis.

Further details of interim analyses for the primary and key secondary endpoints will be provided in the SAP.

An IDMC, consisting of 2 clinicians in the relevant therapeutic area and one statistician, will be established to review efficacy and safety results at the planned interim analyses. After the interim review, they will make recommendations regarding the continuation of the study. In addition to review efficacy interim data, the IDMC will review cumulative safety data (eg, SAEs) after the first 20 participants have been treated for ≥2 cycles or discontinued, and thereafter approximately every 6 months.

10. SUPPORTING DOCUMENTATION AND OPERATIONAL CONSIDERATIONS

10.1. Appendix 1: Clinical Laboratory Tests

The following tests will be performed according to the SoA by the local laboratory:

The actual date of assessment and, if required, the actual time of the assessment of laboratory samples will be recorded in the source documentation and in the eCRF or laboratory requisition form.

Protocol-Required Laboratory Assessments

Laboratory Assessments	Parameters	
Hematology	• Platelet count • Hemoglobin • White blood cell count • ANC • Absolute lymphocyte count	
Coagulation	• PT/INR • Activated partial thromboplastin time • Fibrinogen	
Clinical Chemistry	• Sodium • Potassium • Creatinine	• Total bilirubin (direct if congenital nonhemolytic hyperbilirubinemia) • Alkaline phosphatase

Laboratory Assessments	Parameters	
	• Glucose (fasting, or nonfasting) • AST • ALT	• Lactic acid dehydrogenase • Albumin • Lipase and/or amylase
TLS	• Uric acid • Phosphate • Potassium • Calcium	
Pregnancy[a]	• β-hCG pregnancy tests must have a minimum sensitivity of 50 mIU/mL. • Suggested urine dipstick evaluation; however, perform according to locally available test equipment.	

Serology	• Hepatitis B[b] (see Table 35)
	  ○ HBsAg, Anti-HBs and anti-HBc
	  ○ HBV-DNA if needed (see Table 35)
	• Hepatitis C[b]:
	  ○ Anti-HCV
	  ○ HCV-RNA if anti-HCV-positive
	• HIV (for participants with known HIV)
	  ○ HIV PCR testing
	  ○ CD4 count	

NOTE: eGFR will be calculated per Appendix 11.

a. Highly sensitive urine or serum pregnancy test is required for screening. After screening, urine dipstick is suggested but assessment of pregnancy may be performed by other locally available testing (including serum pregnancy test).

b. See Section 5.2 and Section 8.3.5 to determine eligibility. See Section 8.3.5.3 for routine HBV and HCV monitoring.

10.2. Appendix 2: Regulatory, Ethical, and Study Oversight Considerations

10.2.1. Regulatory and Ethical Considerations

Investigator Responsibilities

The investigator is responsible for ensuring that the study is performed in accordance with the protocol, current ICH guidelines on GCP, and applicable regulatory and country- or territory-specific requirements.

GCP is an international ethical and scientific quality standard for designing, conducting, recording, and reporting studies that involve the participation of human participants. Compliance with this standard provides public assurance that the rights, safety, and well-being of study participants are protected, consistent with the principles that originated in the Declaration of Helsinki, and that the study data are credible.

Protocol Clarification Communications

If text within a final approved protocol requires clarification (eg, current wording is unclear or ambiguous) that does not change any aspect of the current study conduct, a PCC may be prepared. The PCC Document will be communicated to the Investigational Site, Site Monitors, Local Trial Managers, Clinical Trial Managers, and/or Contract Research Organizations who will ensure that the PCC explanations are followed by the investigators.

The PCC Document may be shared by the sites with IECs/IRBs per local regulations.

The PCC Documents must NOT be used in place of protocol amendments, but the content of the PCC Document must be included in any future protocol amendments.

Protocol Amendments

Neither the investigator nor the sponsor will modify this protocol without a formal amendment by the sponsor. All protocol amendments must be issued by the sponsor and signed and dated by the investigator. Protocol amendments must not be implemented without prior IEC/IRB approval, or when the relevant competent authority has raised any grounds for non-acceptance, except when necessary to eliminate immediate hazards to the participants, in which case the amendment must be promptly submitted to the IEC/IRB and relevant competent authority. Documentation of amendment approval by the investigator and IEC/IRB must be provided to the sponsor. When the change(s) involves only logistic or administrative aspects of the study, the IEC/IRB (where required) only needs to be notified.

In situations where a departure from the protocol is unavoidable during the study, the investigator or other physician in attendance will contact the appropriate sponsor representative listed in the Contact Information page(s), which will be provided as a separate document. Except in emergency situations, this contact must be made before implementing any departure from the protocol. In all cases, contact with the sponsor must be made as soon as possible to discuss the situation and agree on an appropriate course of action. The data recorded in the eCRF and source documents will

reflect any departure from the protocol, and the source documents will describe this departure and the circumstances requiring it.

Regulatory Approval/Notification

This protocol and any amendment(s) must be submitted to the appropriate regulatory authorities in each respective country/territory, if applicable. A study may not be initiated until all local regulatory requirements are met.

Required Prestudy Documentation

The following documents must be provided to the sponsor before shipment of study treatment to the study site:

- Protocol and amendment(s), if any, signed and dated by the principal investigator
- A copy of the dated and signed (or sealed, where appropriate per local regulations), written IEC/IRB approval of the protocol, amendments, ICF, any recruiting materials, and if applicable, participant compensation programs. This approval must clearly identify the specific protocol by title and number and must be signed (or sealed, where appropriate per local regulations) by the chairman or authorized designee.
- Name and address of the IEC/IRB, including a current list of the IEC/IRB members and their function, with a statement that it is organized and operates according to GCP and the applicable laws and regulations. If accompanied by a letter of explanation, or equivalent, from the IEC/IRB, a general statement may be substituted for this list. If an investigator or a member of the study site personnel is a member of the IEC/IRB, documentation must be obtained to state that this person did not participate in the deliberations or in the vote/opinion of the study.
- Regulatory authority approval or notification, if applicable
- Signed and dated statement of investigator (eg, Form FDA 1572), if applicable
- Documentation of investigator qualifications (eg, curriculum vitae)
- Completed investigator financial disclosure form from the principal investigator, where required
- Signed and dated clinical trial agreement, which includes the financial agreement
- Any other documentation required by local regulations

The following documents must be provided to the sponsor before enrollment of the first participant:

- Completed investigator financial disclosure forms from all subinvestigators
- Documentation of subinvestigator qualifications (eg, curriculum vitae)
- Name and address of any local laboratory conducting tests for the study, and a dated copy of current laboratory normal ranges for these tests, if applicable
- Local laboratory documentation demonstrating competence and test reliability (eg, accreditation/license), if applicable

IEC or IRB

Before the start of the study, the investigator (or sponsor where required) will provide the IEC/IRB with current and complete copies of the following documents (as required by local regulations):

- Final protocol and, if applicable, amendments
- Sponsor-approved ICF (and any other written materials to be provided to the participants)
- IB (or equivalent information) and amendments/addenda
- Sponsor-approved participant recruiting materials
- Information on compensation for study-related injuries or payment to participants for participation in the study, if applicable
- Investigator's curriculum vitae or equivalent information (unless not required, as documented by the IEC/IRB)
- Information regarding funding, name of the sponsor, institutional affiliations, other potential conflicts of interest, and incentives for participants
- Any other documents that the IEC/IRB requests to fulfill its obligation

This study will be undertaken only after the IEC/IRB has given full approval of the final protocol, amendments (if any, excluding the ones that are purely administrative, with no consequences for participants, data or study conduct, unless required locally), the ICF, applicable recruiting materials, and participant compensation programs, and the sponsor has received a copy of this approval. This approval letter must be dated and must clearly identify the IEC/IRB and the documents being approved.

During the study the investigator (or sponsor where required) will send the following documents and updates to the IEC/IRB for their review and approval, where appropriate:

- Protocol amendments (excluding the ones that are purely administrative, with no consequences for participants, data or study conduct)
- Revision(s) to ICF and any other written materials to be provided to participants
- If applicable, new or revised participant recruiting materials approved by the sponsor
- Revisions to compensation for study-related injuries or payment to participants for participation in the study, if applicable
- New edition(s) of the IB and amendments/addenda
- Summaries of the status of the study at intervals stipulated in guidelines of the IEC/IRB (at least annually)
- Reports of AEs that are serious, unlisted/unexpected, and associated with the study treatment
- New information that may adversely affect the safety of the participants or the conduct of the study
- Deviations from or changes to the protocol to eliminate immediate hazards to the participants

- Report of deaths of participants under the investigator's care
- Notification if a new investigator is responsible for the study at the site
- Development Safety Update Report and Line Listings, where applicable
- Any other requirements of the IEC/IRB

For all protocol amendments (excluding the ones that are purely administrative, with no consequences for participants, data or study conduct), the amendment and applicable ICF revisions must be submitted promptly to the IEC/IRB for review and approval before implementation of the change(s).

At least once a year, the IEC/IRB will be asked to review and reapprove this study, where required.

At the end of the study, the investigator (or sponsor where required) will notify the IEC/IRB about the study completion (if applicable, the notification will be submitted through the head of investigational institution).

Country/Territory Selection

This study will only be conducted in those countries/territories where the intent is to launch or otherwise help ensure access to the developed product if the need for the product persists, unless explicitly addressed as a specific ethical consideration in Section 4.2.1.

Other Ethical Considerations

For study-specific ethical design considerations, refer to Section 4.2.1.

10.2.2. Financial Disclosure

Investigators and subinvestigators will provide the sponsor with sufficient, accurate financial information in accordance with local regulations to allow the sponsor to submit complete and accurate financial certification or disclosure statements to the appropriate regulatory authorities. Investigators are responsible for providing information on financial interests during the study and for 1 year after completion of the study.

Refer to Required Prestudy Documentation (above) for details on financial disclosure.

10.2.3. Informed Consent Process

Each participant must give consent according to local requirements after the nature of the study has been fully explained. The ICF(s) must be signed before performance of any study-related activity. The ICF(s) that is/are used must be approved by both the sponsor and by the reviewing IEC/IRB and be in a language that the participant can read and understand. The informed consent must be in accordance with principles that originated in the Declaration of Helsinki, current ICH and GCP guidelines, applicable regulatory requirements, and sponsor policy.

Informed consent may be obtained remotely. Refer to Monitoring Guideline.

Before enrollment in the study, the investigator or an authorized member of the study site personnel must explain to potential participants the aims, methods, reasonably anticipated benefits, and potential hazards of the study, and any discomfort participation in the study may entail. Participants will be informed that their participation is voluntary and that they may withdraw consent to participate at any time. They will be informed that choosing not to participate will not affect the care the participant will receive for the treatment of his or her disease. Participants will be told that alternative treatments are available if they refuse to take part and that such refusal will not prejudice future treatment. Finally, they will be told that the investigator will maintain a participant identification register for the purposes of long-term follow-up if needed and that their records may be accessed by health authorities and authorized sponsor personnel without violating the confidentiality of the participant, to the extent permitted by the applicable law(s) or regulations. By signing the ICF the participant is authorizing such access, which includes permission to obtain information about his or her survival status. It also denotes that the participant agrees to allow his or her study physician to recontact the participant for the purpose of obtaining consent for additional safety evaluations, and subsequent disease-related treatments, if needed. The physician may also recontact the participant for the purpose of obtaining consent to collect information about his or her survival status.

The participant will be given sufficient time to read the ICF and the opportunity to ask questions. After this explanation and before entry into the study, consent must be appropriately recorded by means of the participant's personally dated signature. After having obtained the consent, a copy of the ICF must be given to the participant.

Participants who are rescreened are required to sign a new ICF.

When prior consent of the participant is not possible and the participant's legally designated representative (unless prohibited by local regulations) is not available, enrollment procedures must be described in the protocol with documented approval/favorable opinion by the IEC/IRB to protect the rights, safety, and well-being of the participant and to ensure compliance with applicable regulatory requirements. The participant or legally designated representative (unless prohibited by local regulations) must be informed about the study as soon as possible and give consent to continue.

10.2.4. Recruitment Strategy

The first participant screened is considered the first act of recruitment and it becomes the study start date.

The Sponsor has provided the study sites with optional patient recruitment material templates, to be used in accordance with local regulations and policies.

10.2.5. Data Protection

Privacy of Personal Data

The collection and processing of personal data from participants enrolled in this study will be limited to those data that are necessary to fulfill the objectives of the study.

These data must be collected and processed with adequate precautions to ensure confidentiality and compliance with applicable data privacy protection laws and regulations. Appropriate technical and organizational measures to protect the personal data against unauthorized disclosures or access, accidental or unlawful destruction, or accidental loss or alteration must be put in place. Sponsor personnel whose responsibilities require access to personal data agree to keep the identity of participants confidential.

The informed consent obtained from the participant includes information about, and where required per applicable regulations, explicit consent for the processing of personal data and for the investigator/institution to allow direct access to his or her original medical records (source data/documents) for study-related monitoring, audit, IEC/IRB review, and regulatory inspection. The informed consent also provides information to address the lawful transfer of the data to other entities and to other countries/territories.

The participant has the right to request through the investigator access to his or her personal data and the right to request rectification of any data that are not correct or complete, or make requests concerning his or her personal data in accordance with applicable data protection law. Reasonable steps will be taken to respond to such a request, taking into consideration the nature of the request, the conditions of the study, and the applicable laws and regulations.

In the event of a data security breach, the sponsor will apply measures to adequately manage and mitigate possible adverse effects taking into consideration the nature of the data security breach as necessary to address other obligations such as notifying appropriate authorities in accordance with applicable data protection law.

Exploratory pharmacodynamic and biomarker research is not conducted under standards appropriate for the return of data to participants. In addition, the sponsor cannot make decisions as to the significance of any findings resulting from exploratory research. Therefore, exploratory research data will not be returned to participants or investigators, unless required by law or local regulations. Privacy and confidentiality of data generated in the future on stored samples will be protected by the same standards applicable to all other clinical data.

10.2.6. Storage, Use, Transfer, and Retention of Data and Samples

Study samples will be coded or anonymized at all times in accordance with the informed consent and will not be labeled with personal identifiers.

Investigator and study site will only store, use, transfer and retain data and study samples, including optional study samples, in accordance with the informed consent and applicable laws, and in accordance with any separate written agreement with sponsor. Other than what is specified in a separate written agreement with sponsor, study site and investigator shall not conduct or facilitate any research by a third party not required by the protocol (i) on participants if such research interferes with the conduct of the study or (ii) on samples collected from study participants during the study, including optional samples, if the research relates to JNJ 64407564 and JNJ 64007957 or (iii) on data collected from study participants during the study if the research relates to JNJ-64407564 and JNJ-64007957.

Sponsor may store, use, transfer or retain the data and study samples, including optional study samples, for uses not specified by the protocol, including compatible research,

10.2.7. Committees Structure

IDMC

An IDMC will be established to monitor data on an ongoing basis and to review interim data. Committee membership responsibilities, authorities, and procedures will be documented in its charter. The committee will meet periodically as noted in Section 9.5 to review interim data. After the review, the IDMC will make recommendations regarding the continuation of the study.

IRC

Participant responses or progression will be evaluated by an IRC. The composition, roles, and responsibilities of the IRC will be described in a separate charter.

10.2.8. Use of Information and Publication

All information, including but not limited to information regarding JNJ-64407564 and JNJ-64007957, supplied by the sponsor to the study site or the investigator and not previously published, and any data, including exploratory biomarker research data, generated as a result of this study, are considered confidential and remain the sole property of the sponsor. The study site and investigator shall not use this information except in the performance of this study and shall not disclose this information to anyone except to persons involved in the study that need such information to assist in conducting the study, and then only on like terms of confidentiality and non-use.

Study site and investigator shall not publish study results except as required by law or as specified in a separate, written agreement between the sponsor and the study site or investigator.

The Sponsor will register the study and publish the study results in compliance with applicable law and may register the study or publish study results when not required.

Authorship of any peer-reviewed publications will be determined by mutual agreement in line with International Committee of Medical Journal Editors authorship guidelines.

In accordance with standard editorial and ethical practice, the sponsor will generally support publication of multicenter studies only in their entirety and not as individual site data. In this case, a coordinating investigator will be designated by mutual agreement.

Registration of Clinical Studies and Disclosure of Results

The sponsor will register and disclose the interim results of clinical studies as required by law.

The summary of the results from the interim analysis as described in Section 9.4, may be submitted to the EU database within one year of the intermediate data analysis date.

The disclosure of the study results will be performed after the end of study.

10.2.9. Data Quality Assurance

Data Quality Assurance/Quality Control

Steps to be taken to ensure the accuracy and reliability of data include the selection of qualified investigators and appropriate study sites, review of protocol procedures with the investigator and study site personnel before the study, periodic monitoring visits by the sponsor, and direct transmission of clinical laboratory data from a central laboratory into the sponsor's data base. Written instructions will be provided for collection, handling, storage, and shipment of samples.

Guidelines for eCRF completion will be provided and reviewed with study site personnel before the start of the study.

The sponsor may review the eCRF for accuracy and completeness during on-site monitoring visits and after transmission to the sponsor; any discrepancies will be resolved with the investigator or designee, as appropriate. After upload of the data into the study database they will be verified for accuracy and consistency with the data sources.

10.2.10. eCRF Completion

Electronic case report forms are prepared and provided by the sponsor for each participant in electronic format. All data relating to the study must be recorded in the eCRF. All eCRF entries, corrections, and alterations must be made by the investigator or authorized study site personnel. The investigator must verify that all data entries in the eCRF are accurate and correct.

The study data will be transcribed by study site personnel from the source documents onto an electronic eCRF, if applicable. Study-specific data will be transmitted in a secure manner to the sponsor.

Worksheets may be used for the capture of some data to facilitate completion of the eCRF. Any such worksheets will become part of the participant's source documents. Data must be entered into the eCRF in English. The eCRF must be completed as soon as possible after a participant visit and the forms must be available for review at the next scheduled monitoring visit.

All participative measurements (eg, pain scale information or other questionnaires) will be completed by the same individual who made the initial baseline determinations whenever possible.

If necessary, queries will be generated in the eDC tool. If corrections to an eCRF are needed after the initial entry into the eCRF, this can be done in either of the following ways:

- Investigator and study site personnel can make corrections in the eDC tool at their own initiative or as a response to an auto query (generated by the eDC tool).
- Sponsor or sponsor delegate can generate a query for resolution by the investigator and study site personnel.

10.2.11. Source Documents

At a minimum, source documents consistent in the type and level of detail with that commonly recorded at the study site as a basis for standard medical care must be available for the following: participant identification, eligibility, and study identification; study discussion and date of signed informed consent; dates of visits; results of safety and efficacy parameters as required by the protocol; record of the assessment by the investigator of all AEs and follow-up of AEs; concomitant medication; treatment receipt/dispensing/return records; study treatment administration information; and date of study completion and reason for early discontinuation of study treatment or withdrawal from the study, if applicable.

The author of an entry in the source documents should be identifiable. Given that PROs are reports of a participant's health condition that come directly from the participant, without interpretation by a clinician or anyone else, the responses to PRO measures entered by trial participants into source records cannot be overridden by site staff or investigators.

Specific details required as source data for the study and source data collection methods will be reviewed with the investigator before the study and will be described in the monitoring guidelines (or other equivalent document).

The minimum source documentation requirements for Section 5.1 (inclusion criteria) and Section 5.2 (exclusion criteria) that specify a need for documented medical history are as follows:

- Referral letter from treating physician or
- Discharge summaries

Inclusion and exclusion criteria not requiring documented medical history must be verified at a minimum by participant interview or other protocol-required assessment (eg, physical examination, laboratory assessment) and documented in the source documents.

An eSource system may be utilized, which contains data traditionally maintained in a hospital or clinic record to document medical care (eg, electronic source documents) as well as the clinical study-specific data fields as determined by the protocol. This data is electronically extracted for use by the sponsor. If eSource is utilized, references made to the eCRF in the protocol include the eSource system, but information collected through eSource may not be limited to that found in the eCRF.

10.2.12. Monitoring

The sponsor will use a combination of monitoring techniques central, remote, or on-site monitoring to monitor this study.

The sponsor will perform on-site monitoring visits as frequently as necessary. The monitor will record dates of the visits in a study site visit log that will be kept at the study site. The first post-initiation visit will be made as soon as possible after enrollment has begun. At these visits, the monitor may compare the data entered into the eCRF with the source documents (eg, hospital/clinic/physician's office medical records). At these visits, the monitor will compare

data entered into the eCRF with the source documents (eg, hospital/clinic/physician's office medical records); a sample may be reviewed. The nature and location of all source documents will be identified to ensure that all sources of original data required to complete the eCRF are known to the sponsor and study site personnel and are accessible for verification by the sponsor study site contact. If electronic records are maintained at the study site, the method of verification must be discussed with the study site personnel.

Direct access to source documents (medical records) must be allowed for the purpose of verifying that the recorded data are consistent with the original source data. Findings from this review will be discussed with the study site personnel. The sponsor expects that, during monitoring visits, the relevant study site personnel will be available, the source documents will be accessible, and a suitable environment will be provided for review of study-related documents. The monitor will meet with the investigator on a regular basis during the study to provide feedback on the study conduct.

In addition to on-site monitoring visits, remote contacts can occur. It is expected that during these remote contacts, study site personnel will be available to provide an update on the progress of the study at the site.

Central monitoring will take place for data identified by the sponsor as requiring central review.

10.2.13. On-site Audits

Representatives of the sponsor's clinical quality assurance department may visit the study site at any time during or after completion of the study to conduct an audit of the study in compliance with regulatory guidelines and company policy. These audits will require access to all study records, including source documents, for inspection. Participant privacy must, however, be respected. The investigator and study site personnel are responsible for being present and available for consultation during routinely scheduled study site audit visits conducted by the sponsor or its designees.

Similar auditing procedures may also be conducted by agents of any regulatory body, either as part of a national GCP compliance program or to review the results of this study in support of a regulatory submission. The investigator must immediately notify the sponsor if he or she has been contacted by a regulatory agency concerning an upcoming inspection.

10.2.14. Record Retention

In compliance with the ICH/GCP guidelines, the investigator/institution will maintain all eCRF and all source documents that support the data collected from each participant, as well as all study documents as specified in ICH/GCP Section 8., Essential Documents for the Conduct of a Clinical Trial, and all study documents as specified by the applicable regulatory requirement(s). The investigator/institution will take measures to prevent accidental or premature destruction of these documents.

Essential documents must be retained until at least 2 years after the last approval of a marketing application in an ICH region and until there are no pending or contemplated marketing applications in an ICH region or until at least 2 years have elapsed since the formal discontinuation of clinical development of the investigational product. These documents will be retained for a longer period if required by the applicable regulatory requirements or by an agreement with the sponsor. It is the responsibility of the sponsor to inform the investigator/institution as to when these documents no longer need to be retained. For trials performed under Regulation [EU] No. 536/2014, the sponsor and the investigator shall archive the content of the clinical trial master file for at least 25 years after the end of the clinical trial.

If the responsible investigator retires, relocates, or for other reasons withdraws from the responsibility of keeping the study records, custody must be transferred to a person who will accept the responsibility. The sponsor must be notified in writing of the name and address of the new custodian. Under no circumstance shall the investigator relocate or dispose of any study documents before having obtained written approval from the sponsor.

If it becomes necessary for the sponsor or the appropriate regulatory authority to review any documentation relating to this study, the investigator/institution must permit access to such reports.

10.2.15. Study and Site Start and Closure

First Act of Recruitment

The first participant screened is considered the first act of recruitment and it becomes the study start date.

Study/Site Termination

The sponsor reserves the right to close the study site or terminate the study at any time for any reason at the sole discretion of the sponsor. Study sites will be closed upon study completion. A study site is considered closed when all required documents and study supplies have been collected and a study-site closure visit has been performed.

The investigator may initiate study-site closure at any time, provided there is reasonable cause and sufficient notice is given in advance of the intended termination.

Reasons for the early closure of a study site by the sponsor or investigator may include but are not limited to:

- Failure of the investigator to comply with the protocol, the requirements of the IEC/IRB or local health authorities, the sponsor's procedures, or GCP guidelines
- Inadequate recruitment of participants by the investigator
- Discontinuation of further study treatment development

10.3. Appendix 3: AEs, SAEs, PQCs, and Other Safety Reporting: Definitions and Procedures for Recording, Evaluating, Follow-up, and Reporting

10.3.1. AE Definitions and Classifications

AE

An AE is any untoward medical occurrence in a clinical study participant administered a pharmaceutical (investigational or non-investigational) product. An AE does not necessarily have a causal relationship with the treatment. An AE can therefore be any unfavorable and unintended sign (including an abnormal finding), symptom, or disease temporally associated with the use of a medicinal (investigational or non-investigational) product, whether or not related to that medicinal (investigational or non-investigational) product. (definition per ICH)

This includes any occurrence that is new in onset or aggravated in severity or frequency from the baseline condition, or abnormal results of diagnostic procedures, including laboratory test abnormalities.

Note: The sponsor collects AEs starting with the signing of the ICF (refer to All AEs under Section 8.4.1 for time of last AE recording).

SAE

An SAE based on ICH and EU Guidelines on Pharmacovigilance for Medicinal Products for Human Use is any untoward medical occurrence that at any dose:

- results in death
- is life-threatening
  (The participant was at risk of death at the time of the event. It does not refer to an event that hypothetically might have caused death if it was more severe.)
- requires inpatient hospitalization or prolongation of existing hospitalization
- results in persistent or significant disability/incapacity
- is a congenital anomaly/birth defect
- is a suspected transmission of any infectious agent via a medicinal product
- is Medically Important*

*Medical and scientific judgment must be exercised in deciding whether expedited reporting is also appropriate in other situations, such as important medical events that may not be immediately life-threatening or result in death or hospitalization but may jeopardize the participant or may require intervention to prevent one of the other outcomes listed in the definition above. These should usually be considered serious.

If a serious and unexpected AE occurs for which there is evidence suggesting a causal relationship between the study treatment and the event (eg, death from anaphylaxis), the event must be reported

as a serious and unexpected suspected adverse reaction even if it is a component of the study endpoint (eg, all-cause mortality).

Unlisted (Unexpected) AE/Reference Safety Information

An AE is considered unlisted if the nature or severity is not consistent with the applicable product reference safety information. For JNJ-64407564 and JNJ-64007957, the expectedness of an AE will be determined by whether or not it is listed in the IB and Addenda. For pomalidomide, elotuzumab, bortezomib, and dexamethasone with a marketing authorization, the expectedness of an AE will be determined by whether or not it is listed in the prescribing information.

10.3.2. Attribution Definitions

Assessment of Causality

The causal relationship to study treatment is assessed by the investigator and documented in the Medical Records.

The assessment of causality must consider the following factors:

- Temporal relationship
- Clinical characteristics of event
- Pharmacological plausibility
- Confounding risk factors:
  - Concomitant medication
  - Underlying/concurrent disease
  - Family/social history
- Challenge:
  - De-challenge: Did the reaction improve when the investigational product was withdrawn, in the absence of any other treatment?
  - Re-challenge: What happens if participant is re-challenged with investigational product?
- Other considerations: Participant characteristics and past medical history, and quality of information

The following selection must be used to assess all AEs.

Related

There is a reasonable causal relationship between study treatment administration and the AE.

Not Related

There is not a reasonable causal relationship between study treatment administration and the AE.

10.3.3. Severity Criteria

AE severity is a clinical determination of the intensity of an AE. The severity assessment for an AE or serious AE should be completed by the investigator using the NCI-CTCAE, Version 5.0, except as noted for CRS (see Section 6.1.4.1) and ICANS (see Section 6.1.4.2). Any AEs or SAEs not listed in the NCI-CTCAE Version 5.0 or evaluable by ASTCT criteria should be evaluated for severity/intensity by using the standard grades as follows:

- **Grade 1** Mild; asymptomatic or mild symptoms; clinical or diagnostic observations only; intervention not indicated.
- **Grade 2** Moderate; minimal, local or noninvasive intervention indicated; limiting age-appropriate instrumental ADL.[a]
- **Grade 3** Severe or medically significant but not immediately life-threatening; hospitalization or prolongation of hospitalization indicated; disabling; limiting self-care ADL.[b]
- **Grade 4** Life-threatening consequences; urgent intervention indicated.
- **Grade 5** Death related to AE.

ADL=activities of daily living

a. Instrumental ADL refers to preparing meals, shopping for groceries or clothes, using the telephone, managing money, etc
b. Self-care ADL refers to bathing; dressing and undressing; feeding self; using the toilet; taking medications; and not bedridden.

The investigator should use clinical judgment in assessing the severity of events not directly experienced by the participant (eg, laboratory abnormalities).

10.3.4. Special Reporting Situations

Safety events of interest on a sponsor study treatment in an interventional study that may require expedited reporting or safety evaluation include, but are not limited to:

- Overdose of a sponsor study treatment.
- Suspected abuse/misuse of a sponsor study treatment.
- Accidental or occupational exposure to a sponsor study treatment.
- Any failure of expected pharmacologic action (ie, lack of effect if used according to the local label) of a sponsor study treatment (to be reported as a PQC for marketed products).
- Unexpected therapeutic or clinical benefit from use of a sponsor study treatment.
- Medication error, intercepted medication error or potential medication error involving a Johnson & Johnson medicinal product (with or without patient exposure to the Johnson & Johnson medicinal product, eg, product name confusion, product label confusion, intercepted prescribing or dispensing errors).
- Exposure to a sponsor study treatment from breastfeeding.
- Reporting of participant pregnancy or participant partner(s) pregnancy.

Participant-specific special reporting situations must be recorded in the eCRF. Any special reporting situation that meets the criteria of an SAE must be recorded on the SAE page of the eCRF.

10.3.5. Procedures

All AEs

All AEs, regardless of seriousness, severity, or presumed relationship to study treatment, must be recorded using medical terminology in the source document and the eCRF. Whenever possible, diagnoses should be given when signs and symptoms are due to a common etiology (eg, cough, runny nose, sneezing, sore throat, and head congestion should be reported as "upper respiratory infection"). Investigators must record in the eCRF their opinion concerning the relationship of the AE to study therapy. All measures required for AE management must be recorded in the source document and reported according to sponsor instructions.

For all studies with an outpatient phase, including open-label studies, the participant must be provided with a "wallet (study) card" and instructed to carry this card with them for the duration of the study indicating the following:

- study number
- statement, in the local language(s), that the participant is participating in a clinical study
- investigator's name and 24-hour contact telephone number
- local sponsor's name and 24-hour contact telephone number (for medical personnel only)
- site number
- participant number
- any other information that is required to do an emergency breaking of the blind

SAEs

All SAEs that have not resolved by the end of the study, or that have not resolved upon the participant's discontinuation from the study, must be followed until any of the following occurs:

- The event resolves.
- The event stabilizes.
- The event returns to baseline, if a baseline value/status is available.
- The event can be attributed to agents other than the study treatment or to factors unrelated to study conduct.
- It becomes unlikely that any additional information can be obtained (participant or health care practitioner refusal to provide additional information, lost to follow-up after demonstration of due diligence with follow-up efforts).

Any event requiring hospitalization (or prolongation of hospitalization) that occurs during participation in the study must be reported as an SAE, except hospitalizations for the following:

- Hospitalizations not intended to treat an acute illness or AE (eg, social reasons such as pending placement in long-term care facility).
- Surgery or procedure planned before entry into the study (must be documented in the eCRF). Note: Hospitalizations that were planned before the signing of the ICF, and where the underlying condition for which the hospitalization was planned has not worsened, will not be considered SAEs. Any AE that results in a prolongation of the originally planned hospitalization is to be reported as a new SAE.
- The administration of blood or platelet transfusion unless there is prolonged hospitalization for a complication of such transfusion remains a reportable SAE.
- Should additional monitoring of participants be required following resolution of clinical symptoms of CRS, extended hospitalization for participant observation will not be considered an SAE.

Disease progression should not be recorded as an AE or SAE term; instead, signs and symptoms of clinical sequelae resulting from disease progression/lack of efficacy will be reported if they fulfill the SAE definition (refer to AE Definitions and Classifications in Appendix 3).

Information regarding SAEs (initial and any follow-up) will be transmitted to the sponsor/CRO immediately, but no later than 24 hours of their knowledge of the event, using the study specific SAE reporting form with the complete (eg, causality, narrative) information available in the medical records, that has been already assessed by a study site physician, and transmitted via secure transmission method selected by the site from among the alternatives proposed by the SM.

10.3.6. PQC Handling

Definition

A PQC is defined as any suspicion of a product defect related to manufacturing, labeling, or packaging, ie, any dissatisfaction relative to the identity, quality, durability, reliability or performance of a distributed product, including its labeling, drug delivery system, or package integrity. A PQC may have an impact on the safety and efficacy of the product. In addition, it includes any technical complaints, defined as any complaint that indicates a potential quality issue during manufacturing, packaging, release testing, stability monitoring, dose preparation, storage or distribution of the product or the drug delivery system.

Procedures

All initial PQCs must be reported to the sponsor by the study site personnel within 24 hours after being made aware of the event.

A sample of the suspected product should be maintained under the correct storage conditions until a shipment request is received from the sponsor.

10.3.7. Contacting Sponsor Regarding Safety, Including Product Quality

The names (and corresponding telephone numbers) of the individuals who must be contacted regarding safety issues, PQC, or questions regarding the study are listed in the Contact Information page(s), which will be provided as a separate document.

10.4. Appendix 4: Contraceptive and Barrier Guidance

Participants must follow contraceptive measures as outlined in Section 5.1. Pregnancy information will be collected and reported as noted in Section 8.4.5 and Appendix 3.

Definitions

POCBP

A participant is considered fertile following menarche and until becoming postmenopausal unless permanently sterile (see below).

Participant Not of Childbearing Potential

- **premenarchal**
  A premenarchal state is one in which menarche has not yet occurred.
- **postmenopausal**
  A postmenopausal state is defined as no menses for 24 months without an alternative medical cause. A high follicle stimulating hormone level (>40 IU/L or mIU/mL) in the postmenopausal range may be used to confirm a postmenopausal state in participants not using hormonal contraception or HRT, however in the absence of 24 months of amenorrhea, a single follicle stimulating hormone measurement is insufficient. If there is a question about menopausal status in participants on HRT, the participant will be required to use one of the non-estrogen-containing hormonal highly effective contraceptive methods if they wish to continue HRT during the study.
- **permanent absence of reproductive potential (for the purpose of this study)**
  - Has undergone a procedure that precludes reproductive potential.
  - Has a congenital abnormality that precludes reproductive potential.

Note: If the childbearing potential changes after start of the study (eg, a premenarchal participant experiences menarche) or the risk of pregnancy changes (eg, a participant who is not heterosexually active becomes active), a participant must begin a highly effective method of contraception, as described throughout the inclusion criteria.

If reproductive status is questionable, additional evaluation should be considered.

Contraceptive (birth control) use by participants must be consistent with local regulations regarding the acceptable methods of contraception for those participating in clinical studies.

Typical use failure rates may differ from those when used consistently and correctly. Use must be consistent with local regulations regarding the use of contraceptive methods for participants in clinical studies.

Due to the embryo-fetal risk associated with pomalidomide, all participants must be subject to the applicable PPP/REMS. Where pomalidomide is provided centrally by the sponsor, investigators should comply with the pomalidomide global PPP. Otherwise, investigators should apply the local

PPP/REMS program, as implemented in the postmarketing setting, and ensure that all participants adhere to the applicable program.

Examples of Contraceptives

EXAMPLES OF CONTRACEPTIVES[a] ALLOWED DURING THE STUDY INCLUDE:
USER INDEPENDENT **Highly Effective Methods That Are User Independent** *Failure rate of <1% per year when used consistently and correctly.*
•Implantable progestogen-only hormone contraception associated with inhibition of ovulation[b]
•Intrauterine device (IUD)
•Levonorgestrel-releasing intrauterine hormone-releasing system (IUS)
•Tubal closure (eg, bilateral tubal occlusion, bilateral tubal ligation)
•Azoospermic partner (vasectomized or due to medical cause) *(Vasectomized partner is a highly effective contraceptive method provided that the vasectomized partner is the sole sexual partner of the POCBP and the absence of sperm in the vasectomized partner has been confirmed. If not, additional highly effective method of contraception must be used. Spermatogenesis cycle is approximately 74 days.)*
USER DEPENDENT **Highly Effective Methods That Are User Dependent** *Failure rate of <1% per year when used consistently and correctly.*
•Combined (estrogen- and progestogen-containing) hormonal contraception associated with inhibition of ovulation[b,c] – PO[c] – intravaginal – transdermal
•Progestogen-only hormone contraception associated with inhibition of ovulation[b] – PO (eg, desogestrel) – injectable
•Sexual abstinence *(Sexual abstinence is considered a highly effective method only if defined as refraining from heterosexual intercourse during the entire period of risk associated with the study treatment. The reliability of sexual abstinence needs to be evaluated in relation to the duration of the study and the preferred and usual lifestyle of the participant.)*
ADDITIONAL EFFECTIVE METHODS *Failure rate of >1% per year when used consistently and correctly.*
•Latex or synthetic condom with or without spermicide[d]
•Diaphragm
•Cervical cap
UNACCEPTABLE CONTRACEPTIVE METHODS
•Progestogen-only PO hormonal contraception where inhibition of ovulation is not the primary mode of action.
•Condoms
•Natural family planning (periodic abstinence, calendar, symptothermal, post-ovulation methods, and rhythm method)
•Withdrawal (coitus-interruptus)
•Spermicides alone

•Lactational amenorrhea method (LAM)/breastfeeding
•Cervical shield – a cervical shield should not be confused with a cervical cap which is an effective secondary form of contraception.
•Combined oral contraception NOTE: Because of the increased risk of venous thromboembolism in patients with multiple myeloma taking pomalidomide and dexamethasone, combined oral contraceptive pills are not recommended. If a participant is currently using combined oral contraception the participant should switch to one of the recommended contraception methods listed above. The risk of venous thromboembolism continues for 4 to 6 weeks after discontinuing combined oral contraception. The efficacy of contraceptive steroids may be reduced during cotreatment with dexamethasone. NOTE: Examples of contraceptives may differ based on region. Investigators should comply with the requirements of the applicable PPP/REMS region.
a) Typical use failure rates may differ from those when used consistently and correctly. Use must be consistent with local regulations regarding the use of contraceptive methods for participants in clinical studies. b) Hormonal contraception may be susceptible to interaction with the study treatment, which may reduce the efficacy of the contraceptive method. In addition, consider if the hormonal contraception may interact with the study treatment. c) Concomitant use of estrogen-containing therapies with pomalidomide can increase the risk of thrombosis. Combined oral contraception is not recommended concomitantly during treatment with pomalidomide. Participants of childbearing potential who are using combined oral contraception should switch to an alternative method of highly effective contraception. The risk of venous thromboembolism continues for 4 to 6 weeks after discontinuing combined oral contraception. The efficacy of contraceptive steroids may be reduced during co-treatment with dexamethasone. d) Multiple types of condoms must not be used together (due to risk of failure with friction).

10.5. Appendix 5: Liver Safety: Suggested Actions and Follow-up Assessments

Please follow guidance provided in applicable Product Information.

10.6. Appendix 6: AEs, Adverse Device Effects, SAEs, Serious Adverse Device Effects, Unanticipated Serious Adverse Device Effects, and Device Deficiencies: Definition and Procedures for Recording, Evaluating, Follow-up, and Reporting in Medical Device Studies

Not applicable.

10.7. Appendix 7: Study Conduct During COVID-19 Pandemic or Other Natural Disaster/Major Disruption

It is recognized that COVID-19 or other natural disaster/major disruption(s) may have an impact on the conduct of this clinical study due to, for example, self-isolation/quarantine by participants and study site personnel; travel restrictions/limited access to public places, including hospitals; study site personnel being reassigned to critical tasks.

The sponsor is providing options for study-related participant management in the event of disruption to the conduct of the study. This guidance does not supersede any local or government requirements or the clinical judgment of the investigator to protect the health and well-being of participants and site staff. If, at any time, a participant's safety is considered to be at risk, study treatment will be discontinued, and study follow-up will be conducted. If it becomes necessary to discontinue participation in the study, the procedures outlined in the protocol for discontinuing study treatment will be followed.

If, as a result of the COVID-19 pandemic or other natural disaster/major disruption, scheduled visits cannot be conducted in person at the study site, they will be performed to the extent possible remotely/virtually or delayed until such time that on-site visits can be resumed. At each contact, participants will be interviewed to collect safety data. Key efficacy endpoint assessments should be performed if required and as feasible. Participants will also be questioned regarding general health status to fulfill any physical examination requirement.

Every effort should be made to adhere to protocol-specified assessments for participants on study treatment, including follow-up. Modifications to protocol-required assessments may be permitted after consultation with the participant, investigator, and the sponsor. Missed assessments/visits will be captured in the clinical trial management system for protocol deviations. Discontinuation of study treatments and withdrawal from the study should be documented with the dedicated prefix in the eCRF.

The sponsor will continue to monitor the conduct and progress of the clinical study, and any changes will be communicated to the sites and to the health authorities according to local guidance. If a participant has tested positive for SARS-CoV-2, the investigator should contact the sponsor's responsible medical officer to discuss plans for study treatment and follow-up. Modifications made to the study conduct as a result of major disruption(s) should be summarized in the CSR.

<u>Exposure to COVID-19</u>

- Any prior COVID-19 infection is to be recorded on the Medical History page.
- In the event a participant requires treatment for a coronavirus infection, refer to the protocol to determine any requirements for dose hold, dose modifications, or other prohibitions and considerations. Report the COVID-19 infection as an AE and any concomitant medications used to treat it to the sponsor following the usual concomitant medication and AE reporting requirements. Ensure any AEs of COVID-19 include the exact term "COVID-19" and any SAE document confirmed infection with coronavirus.

- It is important to follow local health authority guidance specific to your region of practice, such as social distancing, masking in public places, and vaccinations, including boosters.
- In the event a participant is diagnosed with a COVID-19 infection and requires treatment or is eligible for prophylactic treatment after contact with an individual infected with COVID-19, ensure that the participant receives all necessary treatments for COVID-19 that are approved (including those available under Emergency Use Authorization) in the country including, but not limited to antiviral drugs (eg. paxlovid), mAbs, and dexamethasone.
- The use of IV Ig to correct hypogammaglobulinemia is recommended.

Recommendations for COVID-19 Vaccination

Per Section 6.9.1, administration of non-live vaccines approved or authorized for emergency use (eg, COVID-19) by local health authorities are allowed before or during this study.

It is recommended that participants receive prophylactic COVID-19 vaccination when locally available, at the discretion of investigator judgment or institutional practice, and in compliance with the study protocol and local labels for the vaccine. Below is general guidance for consideration.

Many vaccines against COVID-19 are being developed with different technologies and platforms and may have safety and efficacy profiles that are not fully characterized even after preliminary health authority approval. However, the benefit-risk ratio of receiving a COVID-19 vaccine among patients with multiple myeloma participating in talquetamab studies is considered to be positive and should be considered for administration while in compliance with the study protocol and when not otherwise contraindicated for use in the vaccine label.

Per protocol, live attenuated vaccines must be completed ≥4 weeks prior to randomization (see Section 5.2) or initiated ≥30 days after last dose of talquetamab or teclistamab (see Section 6.9). There are no specific timing restrictions for inactivated vaccines, which include vaccines which use alternative technology like mRNA or replication-incompetent viral vectors, per protocol. Enrollment into an interventional clinical trial for an experimental vaccine is prohibited during study. Any vaccination, including COVID-19 vaccinations, must be recorded on the Concomitant Medication page of the eCRF. AEs and SAEs associated COVID-19 vaccine must be collected and reported in accordance with the pharmacovigilance practice.

No data are currently available to suggest that COVID-19 vaccines pose specific or additional safety risk beyond other vaccines for cancer patients undergoing treatment. Theoretically, a diminished immune response may occur in immunocompromised patients, and therefore these patients may have reduced vaccine effectiveness.

For guidance on vaccination, please refer to:

National Comprehensive Cancer Network. Preliminary recommendations of the NCCN COVID-19 Vaccination Advisory Committee* Version 1.0 1/22/2021. NCCN https://www.nccn.org/covid-19/pdf/COVID-19_Vaccination_Guidance_V1.0.pdf (2021),

Garassino, M. C. et al. The ESMO call to action on COVID-19 vaccinations and patients with cancer: Vaccinate. Monitor. Educate. Ann. Oncol. https://doi.org/10.1016/j.annonc.2021.01.068 (2021), and

Desai et al. COVID-19 vaccine guidance for patients with cancer participating in oncology clinical trials Nature Reviews Vol18; 313 https://doi.org/10.1038/s41571-021-00487-z. For additional information for reference or guidance, several organizations and journals have published recommendations for COVID-19 vaccine administration in cancer patients, including the following:

- European Society for Blood and Marrow Transplantation: https://www.ebmt.org/covid-19-and-bmt
- ASTCT: https://www.hematology.org/covid-19/ash-astct-covid-19-vaccination-for-hct-and-car-t-cell-recipients
- Centers for Disease Control and Prevention: https://www.cdc.gov/vaccines/covid-19/info-by-product/clinical-considerations.html

10.8. Appendix 8: Anticipated Events

Purpose

This appendix only applies to the reporting of anticipated events by the sponsor to the US FDA, and US-based investigators and IECs/IRBs, in accordance with the FDA's guidance. The intent is to minimize the submission of a multitude of uninformative IND safety reports to these recipients.

Definition of an Anticipated Event

An anticipated event is an AE (serious or non-serious) that commonly occurs, independent of exposure to study treatment, as a consequence of (a) the underlying disease or condition under investigation, (b) characteristics of the study population (eg, age), or (c) the background treatment regimen.

Background

The FDA acknowledges that certain SAEs can be anticipated to occur commonly in the study population regardless of drug exposure. Although these anticipated SAEs may meet the definition of unexpected (ie, SUSARs), because they are not listed in the IB, they do not warrant expedited reporting as individual cases, or even in aggregate if the incidence is consistent with the expected background rates in the study population.

Reporting of Anticipated Events

All AEs will be recorded by the investigator in the eCRF regardless of whether considered to be anticipated events and will be reported to the sponsor as described in Section 8.4. Any anticipated event that meets serious criteria will be reported to the sponsor as described in Section 8.4.1.

To meet US safety reporting, the sponsor's SAC will periodically perform aggregate analysis of anticipated events per the Anticipated Events Safety Monitoring Plan (see below). If an anticipated event is determined to occur more frequently in the experimental arm(s) of the study and there is a reasonable possibility that the anticipated event could be drug-related, the sponsor will prepare an aggregate safety report for reporting of these events to FDA and US-based IRBs/ECs and investigators.

Sponsor's SAC

The sponsor's SAC is an established safety committee, independent of the study team, that performs reviews of prespecified anticipated events at an aggregate level. The SAC will meet to aid in the recommendation to the sponsor's study team as to whether there is a reasonable possibility that an anticipated event is related to the study treatment. The SAC will consider in its analysis all relevant drug development data, in addition to the clinical study data.

Statistical Analysis

Details of statistical analysis of anticipated events, including the frequency of review and threshold to trigger an aggregate analysis of anticipated events will be provided in a separate Anticipated Events Safety Monitoring Plan.

Anticipated Events for the Study

The below list includes AEs that are commonly anticipated for the disease stage, study population and background treatment, for which the sponsor does not plan to report individual cases, regardless of the assessment of causality. This list is limited to those events for which an individual occurrence, or even an aggregate incidence consistent with the background rate in the study population, is inconsequential in the developing safety profile of the study treatment.

For the purposes of this study, the following events will be considered anticipated events:

- bleeding
- bone diseases
- hyperuricemia
- hyperviscosity syndrome
- renal failure or insufficiency
- hypercalcemia
- infection

10.9. Appendix 9: IMWG Diagnostic Criteria

Diagnostic criteria for myeloma must be met when the participant was diagnosed. Multiple myeloma is defined as clonal BMPCs ≥10% or biopsy-proven bony or extramedullary plasmacytoma[a] and any 1 or more of the following myeloma defining events:

- Myeloma defining events:
  - Evidence of end organ damage that can be attributed to the underlying PC proliferative disorder, specifically:
    - **C:** Hypercalcemia: serum calcium >0.25 mmol/L (>1 mg/dL) higher than the ULN or >2.75 mmol/L (>11 mg/dL)
    - **R:** Renal insufficiency: creatinine clearance <40 mL per min[b] or serum creatinine >177 µmol/L (>2 mg/dL)
    - **A:** Anemia: hemoglobin value of >20 g/L below the lower limit of normal, or a hemoglobin value <100 g/L
    - **B:** Bone lesions: 1 or more osteolytic lesions on skeletal radiography, CT, or PET-CT[c,d]
- Any 1 or more of the following biomarkers of malignancy:
  - Clonal BMPC percentage[a] ≥60%
  - Involved:uninvolved serum FLC ratio[e] ≥100
  - >1 focal lesions on MRI studies[f]

[a] Clonality should be established by showing κ/λ-light-chain restriction on flow cytometry, IHC, or immunofluorescence. BMPC percentage should preferably be estimated from a core biopsy specimen; in case of a disparity between the aspirate and core biopsy, the highest value should be used.
[b] Measured or estimated by validated equations.
[c] If bone marrow has less than 10% clonal PCs, more than one bone lesion is required to distinguish from solitary plasmacytoma with minimal marrow involvement.
[d] PET-CT=^{18}F- fluorodeoxyglucose positron emission tomography with CT.
[e] These values are based on the serum Freelite assay (The Binding Site Group, Birmingham, UK). The involved FLC must be ≥100 mg/L.
[f] Each focal lesion must be 5 mm or more in size.

Source: Rajkumar 2011

10.10. Appendix 10: Prior Cancer Therapy for Multiple Myeloma

A line of therapy is defined as 1 or more cycles of a planned treatment regimen. This may consist of 1 or more planned cycles of single-agent therapy or combination therapy, as well as a sequence of treatments administered in a planned manner. For example, a planned treatment approach of induction therapy followed by autologous stem cell transplantation, followed by maintenance is considered one line of therapy. A single short course of corticosteroids (no more than the equivalent of dexamethasone 40 mg/day for 4 days) would not be considered a prior line of therapy. A new line of therapy starts when a planned course of therapy is modified to include other treatment agents (alone or in combination) as a result of disease progression, relapse, or toxicity (Rajkumar 2011). A new line of therapy also starts when a planned period of observation off therapy is interrupted by a need for additional treatment for the disease.

Source: Rajkumar 2011.

10.11. Appendix 11: Formulas for Estimating Glomerular Filtration Rate Using Modified Diet in Renal Disease Formula (in mL/min)

For standardized serum creatinine (S_{Cr}) in **mg/dL**, the eGFR in **mL/min** is:

$$eGFR = 175\ x\ S_{Cr}\ (mg/dL)^{-1.154}\ x\ age\ (years)^{-0.203}\ x\ 1.212_{if\ black}\ x\ 0.742_{if\ female}\ x\ \frac{BSA\ (m^2)}{1.73}$$

BSA is estimated based on height and weight (Mosteller 1987):

$$BSA\ (m^2) = \sqrt{\frac{height\ (cm)\ x\ weight\ (kg)}{3600}}$$

If creatinine levels are given in µmol/L, the equation above can be used after converting the creatinine level to mg/dL:

$$creatinine\ (mg/dL) = \frac{creatinine\ (\mu mol/L)}{88.4}$$

Alternatively, for standardized serum creatinine (S_{Cr}) in **µmol/L**, the eGFR in **mL/min** is:

$$eGFR = 30849\ x\ S_{Cr}\ (\mu mol/L)^{-1.154}\ x\ age\ (years)^{-0.203}\ x\ 1.212_{if\ black}\ x\ 0.742_{if\ female}\ x\ \frac{BSA\ (m^2)}{1.73}$$

NOTES:

- Renal clearance of a drug is proportional to individual GFR (expressed in **mL/min**) and not BSA-standardized GFR (mL/min/1.73m^2).
- The formulas above for the calculation of eGFR are based on Modified Diet in Renal Disease (Levey 2006) but have been adapted to estimate GFR in **mL/min** (FDA CDER. Guidance for Industry. Pharmacokinetics in Patients with Impaired Renal Function – Study Design, Data Analysis, and Impact on Dosing. 2020).
- Serum creatinine values must be standardized: creatinine assay calibration traceable to an isotope dilution mass spectrometry reference measurement procedure (this is the case for most assays)

10.12. Appendix 12: Serum Calcium Corrected for Albumin

If calcium is expressed in mg/dL and albumin is expressed in g/dL:
Corrected calcium (mg/dL) =
serum calcium (mg/dL) + 0.8•(4 - serum albumin [g/dL])

If calcium is expressed in mmol/L and albumin is expressed in g/L:
Corrected calcium (mmol/L) =
serum calcium (mmol/L) + 0.02•(40 - serum albumin [g/L])

Source: Burtis 1999.

10.13. Appendix 13: Conversion Table for Glucocorticoid Dose

Steroid	Approximate Equivalent Dose to Dexamethasone 8 mg	Approximate Equivalent Dose to Dexamethasone 16 mg	Approximate Equivalent Dose to Dexamethasone 20 mg	Approximate Equivalent Dose to Dexamethasone 40 mg
Methylprednisolone	45 mg	90 mg	110 mg	220 mg
Prednisone/Prednisolone	55 mg	110 mg	140 mg	280 mg

10.14. Appendix 14: ICE Tool

Category	Points
Orientation: orientation to year, month, city, hospital	4 points
Naming: ability to name 3 objects (eg, point to clock, pen, button)	3 points
Following commands: ability to follow simple commands (eg, “Show me 2 fingers” or “Close your eyes and stick out your tongue”)	1 point
Writing: ability to write a standard sentence (eg, “Our national bird is the bald eagle.”)	1 point
Attention: ability to count backward from 100 by 10	1 point

Source: Lee 2019

10.15. Appendix 15: ECOG Performance Status Score

Grade	ECOG Performance Status
0	Fully active, able to carry on all pre-disease performance without restriction.
1	Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature, eg, light housework, office work.
2	Ambulatory and capable of all self-care but unable to carry out any work activities. Up and about more than 50% of waking hours.
3	Capable of only limited self-care, confined to bed or chair more than 50% of waking hours.
4	Completely disabled. Cannot carry on any self-care. Totally confined to bed or chair.
5	Dead.

Source: Oken 1982.

10.16. Appendix 16: Considerations for Outpatient Talquetamab and Teclistamab Administration Through the First Treatment Dose

When evaluating the suitability for outpatient administration, if allowed by local regulations and institutional guidance, investigators should assess the participant's clinical status and the healthcare facility capability to safely manage outpatient logistics and discuss with sponsor. General recommendations for each of these considerations are provided below.

Clinical Consideration

General guidance for clinical considerations for a participant who is suitable for outpatient administration and follow-up includes the following:

- No packed red blood cell or platelet transfusions within the last 7 days prior to dosing.
- No fever or active infection (bacterial, fungal, viral) since study enrollment.
- No Grade ≥3 hematologic AEs or Grade ≥3, clinically significant non-hematologic AEs.
- No clinically significant coagulopathy that would increase the risk of bleeding in the setting of cytopenia.
- No high tumor burden, as defined as in Section 6.1.4.7.
- No risk factors for developing clinically significant TLS and requiring management with increased hydration, allopurinol, or rasburicase. Participants who are receiving prophylactic treatment for TLS are eligible for outpatient administration, if deemed stable by the investigator.
- No rapidly progressing disease per investigator assessment.
- No deterioration in neurologic status, including mental status changes such as confusion or increased somnolence. The only exception is confusion or somnolence that has resolved and must be attributed to diphenhydramine premedication for talquetamab/teclistamab.
- The following laboratory parameters:
  - eGFR ≥40 mL/min
  - AST and ALT ≤3×ULN.

Logistical Consideration for Qualified Healthcare Facility

Outpatient administration and post-talquetamab/teclistamab injection follow-up must take place at a qualified healthcare facility. The following should be considered for outpatient administration and follow-up:

- Participant is required to stay within 30 minutes of transportation to the site and remain in the company of a competent adult at all times from after Step-up Dose 1 until 48 hours after administration of treatment dose (see Section 6.1.3.1).
- Site must prepare a documented plan to ensure timely management of CRS/ICANS if these occur in an outpatient setting, including a mechanism to notify the hematology team and treat with tocilizumab or alternative treatment if clinically indicated.

- Participant and accompanying competent adult must be made aware of the presenting signs and symptoms of talquetamab- or teclistamab-associated toxicities, including but not limited, to CRS, ICANS, infections, etc.
- Site must provide participant with educational material and wallet card, including but not limited to, notification of enrollment in a clinical trial, emergency contact information, and signs and symptoms of CRS/ICANS.
- Participant should carry the wallet card for study participant identification at all times.
- Participant will receive daily phone call follow-ups from the hospital site staff during typical business hours and participant will be able to contact a clinical team member after hours until 48 hours after the first treatment dose.
- Participant must comply with all the protocol requirement procedures, including measuring and recording of body temperature twice daily (≥8 hours apart) in a participant diary and coming to the site for safety assessments according to Table 3 (Arm A and Arm B).
- Admission to the hospital is required at any time in the event of any presenting signs and symptoms of CRS and/or ICANS in association with step-up doses and first treatment dose. Participants will be promptly treated upon presentation to the hospital and will remain hospitalized for at least 48 hours. Additionally, if CRS/ICANS occurs at step-up dose 1, then step-up dose 2 must occur inpatient. If CRS/ICANS occurs at step-up dose 2, then the first treatment dose must occur inpatient. If CRS/ICANS occurs at the first treatment dose or beyond, specifications in Table 19 must be followed.

10.17. Appendix 17: IMWG Response Criteria (2016)

Response	Response Criteria
sCR	• CR as defined below, *plus* • Normal FLC ratio, *and* • Absence of clonal PCs by IHC (κ/λ ratio ≤4:1 or ≥1:2 for κ and λ participants, respectively, after counting ≥100 PCs) or negative 2-4 color flow cytometry
CR[a]	• Negative immunofixation of serum and urine, *and* • Disappearance of any soft-tissue plasmacytomas, *and* • <5% PCs in bone marrow • No evidence of initial M-protein isotype(s) on immunofixation of the serum and urine[b] • For FLC only patients: normal FLC ratio of 0.26 to 1.65 (or reference range in testing laboratory) in addition to CR criteria listed above
VGPR[a]	• Serum and urine M-component detectable by immunofixation but not on electrophoresis, *or* • ≥90% reduction in serum M-component plus urine M-component <100 mg/24 hours • In addition to the above criteria, if present at baseline >90% reduction in the SPD compared with baseline for soft-tissue plasmacytoma • For FLC only patients: ≥90% decrease in the difference between involved and uninvolved FLC levels
PR	• ≥50% reduction of serum M-protein and reduction in 24-hour urinary M-protein by ≥90% or to < 200 mg/24 hours • If serum and urine M-protein are not measurable, a decrease ≥50% in the difference between involved and uninvolved FLC levels is required in place of the M-protein criteria • In addition to the above criteria, if present at baseline, ≥50% reduction in the size (SPD) of soft-tissue plasmacytomas is also required
Minimal response	• ≥25% but ≤49% reduction of serum M-protein *and* reduction in 24-hour urine M-protein by 50% to 89% • In addition to the above criteria, if present at baseline, ≥50% reduction in the size (SPD) of soft-tissue plasmacytomas is also required
Stable disease	• Not meeting criteria for sCR, CR, VGPR, PR, MR, or PD
PD[c]	Any 1 or more of the following criteria: • Increase of 25% from lowest response value in any of the following: – Serum M-component (absolute increase must be ≥0.5 g/dL), *and/or* – Urine M-component (absolute increase must be ≥200 mg/24 hours), *and/or* – Only in participants without measurable serum and urine M-protein levels: the difference between involved and uninvolved FLC levels (absolute increase must be >10 mg/dL) – Only in participants without measurable serum and urine M-protein levels and without measurable disease by FLC levels, bone marrow plasma cell percentage (the absolute percentage must be ≥10%) • Soft-tissue plasmacytoma progressive disease criteria: – Appearance of a new soft tissue plasmacytoma(s) – ≥50% increase from nadir in the SPD of all measurable soft-tissue plasmacytomas existing at baseline, or – ≥50% increase from nadir in the longest diameter of any existing soft-tissue plasmacytoma that is >1 cm on the short axis (if multiple soft-tissue plasmacytomas are present at baseline, only one needs to show the increase from nadir in the longest diameter) • Definite development of new bone lesions or definite increase in the size of existing bone lesions

	• ≥50% increase in circulating PCs (minimum of 200 cells/µL) if this is the only measure of disease

NOTE: All response categories (CR, sCR, VGPR, PR, MR, and PD) require 2 consecutive assessments made at any time before the institution of any new therapy; CR, sCR, VGPR, PR, MR, and stable disease categories also require no known evidence of progressive or new bone lesions if radiographic studies were performed. VGPR and CR categories require serum and urine studies regardless of whether disease at baseline was measurable on serum, urine, both, or neither. Bone marrow assessments need not be confirmed.

NOTE: To qualify for VGPR or PR/minimal response, the sum of products of the perpendicular diameters of the existing extramedullary plasmacytomas must have decreased by >90% or ≥50%, respectively, and new plasmacytomas must not have developed. To qualify for disease progression, either the sum of products of the perpendicular diameters of the existing extramedullary plasmacytomas must have increased by ≥50%, or the longest diameter of previous lesion >1 cm in short axis must have increased ≥50%, or a new plasmacytoma must have developed. When not all existing extramedullary plasmacytomas are reported, but the sum of products of the perpendicular diameters of the reported plasmacytomas have increased by ≥50%, then the criterion for disease progression is met.

Radiographic studies are not required to satisfy these response requirements. For PD, serum M-component increases of ≥1 g/dL are sufficient to define relapse if lowest M-component is ≥5 g/dL.

a. Clarifications to the criteria for coding CR and VGPR in participants in whom the only measurable disease is by serum FLC levels: CR in such participants indicates a normal FLC ratio of 0.26 to 1.65 (or reference range in testing laboratory) in addition to CR criteria listed above. VGPR in such participants requires a ≥90% decrease in the difference between involved and uninvolved FLC levels.
b. In some cases, it is possible that the original M-protein light chain isotype is still detected on immunofixation but the accompanying heavy-chain component has disappeared; this would not be considered as a CR even though the heavy-chain component is not detectable, since it is possible that the clone evolved to one that secreted only light chains. Thus, if a participant has IgA lambda myeloma, then to qualify as CR there should be no IgA detectable on serum or urine immunofixation; if free lambda is detected without IgA, then it must be accompanied by a different heavy-chain isotype (IgG, IgM, etc).
c. Clarifications to the criteria for coding PD: bone marrow criteria for PD are to be used only in participants without measurable disease by M-protein and by FLC levels; “25% increase” refers to M-protein, and FLC, and does not refer to bone lesions, or soft-tissue plasmacytomas and the “lowest response value” does not need to be a confirmed value.

Source: Adapted from Durie 2015, Rajkumar 2011, and Kumar 2016.

10.18. Appendix 18: Severity Grading for ICANS (ASTCT)

Neurotoxicity Domain	Grade 1	Grade 2	Grade 3	Grade 4
ICE score[a]	7-9	3-6	0-2	0 (participant is unarousable and unable to perform ICE testing)
Depressed level of consciousness[b]	Awakens spontaneously	Awakens to voice	Awakens only to tactile stimulus	Participant is unarousable or requires vigorous or repetitive tactile stimuli to arouse Stupor or coma
Seizures	N/A	N/A	Any clinical seizure focal or general that resolves rapidly; or non-convulsive seizures on electroencephalogram that resolve with intervention	Life-threatening, prolonged seizures (>5 min) or repetitive clinical or electrical seizures without return to baseline in between
Motor findings[c]	N/A	N/A	N/A	Deep focal motor weakness such as hemiparesis or paraparesis
Raised ICP/IOP/cerebral edema	N/A	N/A	Focal/local edema on neuroimaging[d]	Diffuse cerebral edema on neuroimaging; decerebrate or decorticate posturing; or cranial nerve VI palsy; or papilledema; or Cushing's Triad

NOTE: By convention, Grade 5 ICANS is defined as death due to ICANS in which another cause is not the principal factor leading to this outcome.

NOTE: ICANS grade is determined by the most severe event (ICE score, level of consciousness, seizure, motor findings, raised ICP/cerebral edema) not attributable to any other cause. For example, a participant with an ICE score of 3 who has a generalized seizure is classified as having Grade 3 ICANS.

a. A participant with an ICE score of 0 may be classified as having Grade 3 ICANS if the participant is awake with global aphasia. But a participant with an ICE score of 0 may be classified as having Grade 4 ICANS if the participant is unarousable. See Section 8.3.8 for instructions for administering the ICE Tool.

b. Depressed level of consciousness should be attributable to no other cause (eg, no sedating medication).

c. Tremors and myoclonus associated with immune effector cell therapies may be graded according to NCI-CTCAE Version 4.03, but they do not influence ICANS grading.

d. Intracranial hemorrhage with or without associated edema is not considered a neurotoxicity feature and is excluded from ICANS grading. It may be graded according to NCI-CTCAE Version 5.0.

Source: Lee 2019.

10.19. Appendix 19: NYHA Functional Classification

NYHA Class	Symptoms
I	Cardiac disease, but no symptoms and no limitation in ordinary physical activity, eg, shortness of breath when walking, climbing stairs etc.
II	Mild symptoms (mild shortness of breath and/or angina) and slight limitation during ordinary activity.
III	Marked limitation in activity due to symptoms, even during less-than-ordinary activity, eg, walking short distances (20–100 m). Comfortable only at rest.
IV	Severe limitations. Experiences symptoms even while at rest. Mostly bedbound patients.

10.20. Appendix 20: Prophylaxis of Venous Thromboembolism

Risk Assessment Model for the Management of Venous Thromboembolism in Multiple Myeloma Patients Treated With IMiDs

Risk Factors	Actions
Individual risk factors	
Obesity[a]	If no risk factor or any 1 individual/myeloma-related risk factor is present • Aspirin 81 to 325 mg once daily If 2 or more individual/myeloma-related risk factors are present: • LMWH (equivalent of enoxaparin 40 mg once daily) • Full-dose warfarin or other vitamin K antagonists (target INR 2-3)
Previous venous thromboembolism	
Central venous catheter or pacemaker	
Associated disease Cardiac disease Chronic renal disease Diabetes Acute infection Immobilization	
Surgery General surgery Any anesthesia Trauma	
Medications Erythropoietin	
Blood clotting disorders	
Myeloma-related risk factors	
Diagnosis	
Hyperviscosity	
Myeloma therapy-related risk factors	
High-dose dexamethasone[b]	All patients who receive high-dose dexamethasone or doxorubicin or multiagent chemotherapy, independent of the presence of additional risk factors: • LMWH (equivalent of enoxaparin 40 mg once daily) • Full-dose warfarin or other vitamin K antagonists (target INR 2-3)
Doxorubicin	
Multiagent chemotherapy	

BMI=body mass index; IMiD=immunomodulatory imide drug; INR=international normalized ratio; LMWH=low molecular weight heparin

NOTE: Per Section 6.9.3.1, the use of warfarin or other vitamin K antagonists during Cycle 1 is prohibited unless no other therapeutic option is available.

a. Obesity defined as BMI ≥30 kg/m^2

b. ≥480 mg per month

Source: Palumbo 2008.

10.21. Appendix 21: Protocol Amendment History

The Protocol Amendment Summary of Changes Table for the current amendment is located directly before the Table of Contents.

Amendment 1 (14 July 2023)

Overall Rationale for the Amendment: The overall reason for this amendment is to comply with company requirements.

The changes made to the clinical protocol 64407564MMY3009 as part of Protocol Amendment 1 are listed below, including the rationale of each change and a list of all applicable sections.

Section Number and Name	Description of Change	Brief Rationale
5.4 Screen Failures	Revised text to specify that the investigator will not generate screening and enrollment logs directly from IWRS, and any deviations from their use require sponsor review and approval.	To meet company requirements.
1.3 SoA Table 3 SoA for Treatment Phase, EOT Visit, and Posttreatment Follow-up Phase – Arm A (Tal-P) and Arm B (Tal-Tec); Table 6 SoA for Treatment Phase, EOT Visit, and Posttreatment Follow-up Phase – Arm C (EPd); Table 9 SoA for Treatment Phase, EOT Visit, and Posttreatment Follow-up Phase – Arm C (PVd) 8.2.7 PROs	Text was revised to clarify and consolidate guidance regarding PRO assessments.	To clarify descriptive information on PRO assessments.
8.2.7.6 Qualitative Interview	Text revised to specify that participants from selected countries will have the option of participating in a single interview that will last approximately 45-60 minutes.	To clarify description of qualitative interview.
Throughout the protocol	Updates for consistency with protocol sections.	Clarification.

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INVESTIGATOR AGREEMENT

I have read this protocol and agree that it contains all necessary details for carrying out this study. I will conduct the study as outlined herein and will complete the study within the time designated.

I will provide copies of the protocol and all pertinent information to all individuals responsible to me who assist in the conduct of this study. I will discuss this material with them to ensure that they are fully informed regarding the study treatment, the conduct of the study, and the obligations of confidentiality.

Coordinating Investigator (where required):

Name (typed or printed): _____

Institution and Address: _____

_____

_____

_____

Signature: _____ Date: _____
(Day Month Year)

Principal (Site) Investigator:

Name (typed or printed): _____

Institution and Address: _____

_____

_____

_____

Telephone Number: _____

Signature: _____ Date: _____
(Day Month Year)

Sponsor's Responsible Medical Officer:

Name (typed or printed): Christoph Heuck, MD

Institution: Janssen Research & Development

Signature: electronic signature appended at the end of the protocol Date: _____
(Day Month Year)

Note: If the address or telephone number of the investigator changes during the study, written notification will be provided by the investigator to the sponsor, and a protocol amendment will not be required.

Signature

User	Date	Reason
Heuck Christoph 1048738	21-Mar-2024 17:16:27 (GMT)	Document Approval